Answer Key & Rationales · Form 9

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.23-0423.

#	Key	Area	Sub-area	P
1	**B**	III	III.B	
2	**C**	II	II.B	
3	**A**	III	III.B	
4	**B**	I	I.C	
5	**C**	I	I.A	**P**
6	**C**	II	II.A	
7	**B**	III	III.A	
8	**B**	I	I.B	
9	**A**	I	I.B	**P**
10	**D**	III	III.B	
11	**D**	II	II.B	
12	**B**	III	III.B	
13	**A**	I	I.A	
14	**B**	I	I.A	
15	**A**	II	II.B	
16	**C**	III	III.C	
17	**C**	III	III.B	
18	**B**	I	I.B	
19	**B**	I	I.C	
20	**C**	II	II.B	
21	**B**	I	I.C	
22	**B**	II	II.A	
23	**D**	III	III.B	
24	**A**	I	I.B	
25	**C**	III	III.A	
26	**C**	III	III.A	
27	**A**	I	I.B	
28	**A**	III	III.D	
29	**C**	III	III.B	
30	**B**	I	I.A	**P**
31	**B**	I	I.C	
32	**A**	III	III.B	
33	**C**	I	I.B	
34	**B**	I	I.A	
35	**B**	III	III.D	
36	**C**	I	I.C	
37	**D**	III	III.B	**P**
38	**A**	II	II.B	**P**
39	**A**	II	II.A	
40	**B**	I	I.A	
41	**B**	II	II.A	
42	**A**	II	II.C	
43	**A**	II	II.A	
44	**D**	II	II.A	
45	**A**	I	I.B	
46	**B**	I	I.A	
47	**B**	III	III.B	
48	**C**	I	I.B	
49	**C**	I	I.A	
50	**B**	III	III.B	**P**
51	**D**	III	III.C	
52	**A**	II	II.C	
53	**B**	I	I.B	
54	**B**	II	II.C	
55	**C**	II	II.A	
56	**D**	II	II.A	
57	**C**	II	II.A	
58	**A**	II	II.B	
59	**A**	II	II.B	
60	**B**	III	III.B	
61	**D**	I	I.B	
62	**B**	III	III.C	
63	**A**	I	I.B	
64	**A**	II	II.A	
65	**C**	II	II.B	
66	**C**	I	I.B	
67	**B**	II	II.A	
68	**C**	II	II.C	
69	**D**	III	III.B	
70	**D**	I	I.C	
71	**B**	I	I.B	
72	**D**	III	III.B	
73	**C**	III	III.B	**P**
74	**C**	III	III.C	
75	**B**	II	II.B	
76	**A**	III	III.A	
77	**A**	III	III.B	
78	**A**	II	II.B	
79	**B**	II	II.B	**P**
80	**A**	I	I.B	
81	**A**	II	II.A	
82	**D**	III	III.B	
83	**C**	I	I.A	
84	**C**	II	II.B	
85	**A**	I	I.A	
86	**B**	II	II.A	
87	**D**	II	II.A	
88	**A**	II	II.B	**P**
89	**C**	I	I.B	
90	**A**	I	I.B	
91	**B**	III	III.B	
92	**B**	III	III.D	
93	**A**	I	I.B	
94	**B**	I	I.B	
95	**D**	III	III.B	
96	**B**	III	III.A	
97	**C**	II	II.B	**P**
98	**D**	II	II.A	
99	**A**	II	II.A	
100	**C**	III	III.B	
101	**A**	III	III.B	**P**
102	**C**	I	I.C	
103	**C**	III	III.B	
104	**A**	II	II.A	
105	**A**	III	III.B	
106	**C**	I	I.C	
107	**B**	III	III.B	
108	**A**	II	II.B	
109	**C**	III	III.A	
110	**C**	III	III.D	
111	**A**	I	I.C	
112	**C**	I	I.C	
113	**A**	I	I.A	
114	**C**	II	II.A	
115	**A**	II	II.C	
116	**A**	III	III.C	
117	**D**	I	I.A	
118	**C**	II	II.B	
119	**A**	I	I.A	
120	**A**	I	I.A	
121	**B**	I	I.A	**P**
122	**B**	I	I.C	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**____ / 40**
I.A Ethical Principles and Responsibilities	13, 14, 34, 40, 46, 49, 83, 85, 113, 117, 119, 120	12	____
I.B Ethical Service Delivery	8, 18, 24, 27, 33, 45, 48, 53, 61, 63, 66, 71, 80, 89, 90, 93, 94	17	____
I.C Diversity and Social Justice	4, 19, 21, 31, 36, 70, 102, 106, 111, 112, 122	11	____
II. Assessment and Planning		**35**	**____ / 35**
II.A Assessment Concepts	6, 22, 39, 41, 43, 44, 55, 56, 57, 64, 67, 81, 86, 87, 98, 99, 104, 114	18	____
II.B Assessment Methods and Techniques	2, 11, 15, 20, 58, 59, 65, 75, 78, 84, 108, 118	12	____
II.C Assessment Practices	42, 52, 54, 68, 115	5	____
III. Intervention and Practice		**35**	**____ / 35**
III.A Practice Concepts	7, 25, 26, 76, 96, 109	6	____
III.B Intervention Methods and Techniques	1, 3, 10, 12, 17, 23, 29, 32, 47, 60, 69, 72, 77, 82, 91, 95, 100, 103, 105, 107	20	____
III.C Practice Evaluation and Research	16, 51, 62, 74, 116	5	____
III.D Supervision and Administration	28, 35, 92, 110	4	____

Pretest items (skip when scoring): 5, 9, 30, 37, 38, 50, 73, 79, 88, 97, 101, 121

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. INT-1635 (rev 1) · Best answer: B

A client on probation attends a court-ordered program. When the client mentions arriving on time for a new job all week, the social worker gives specific praise. When the client jokes about 'beating the system,' the worker calmly offers a different view and moves on. The worker also arrives on time and keeps every promise. Which approach is the social worker **MOST** likely using?

A. Motivational interviewing
B. Prosocial modeling
C. Contingency management

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). MI does use affirmations, so the praise fits partly. But MI responds to a remark like the joke with reflection rather than the worker's own contrasting view, and modeling punctuality is not part of the method.

B **Correct.** Trotter's (2022) prosocial modeling approach for involuntary clients has the worker notice and reward prosocial comments and actions, model reliable and respectful behavior, and briefly challenge procriminal remarks. Specific praise for punctuality, a calm counter to the 'beating the system' joke, and the worker's own punctuality and kept promises are each part of it. It beats A because motivational interviewing meets a provocative remark with reflection rather than the worker's own contrasting view, and it does not center the worker's modeling of reliable behavior (Miller & Rollnick, 2023).

C **Incorrect** (trap: Wrong model or concept). Contingency management gives tangible rewards, such as vouchers, for verified target behaviors on a set schedule. Here the reinforcement is verbal and paired with modeling and gentle challenge, which marks a different approach.

Takeaway: Prosocial modeling rewards clients' prosocial words and actions, models reliability and respect, and briefly challenges procriminal remarks with involuntary clients.

Sources: Trotter, C. (2022). Working with Involuntary Clients: A Guide to Practice (4th ed.). Routledge, Prosocial modelling and reinforcement; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Affirming; responding to discord

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

2. ASM-1678 (rev 1) · Best answer: C

A neurologist asks a hospital social worker to assess how a 34-year-old patient newly diagnosed with multiple sclerosis will manage after discharge. The patient is tearful but calm and plans to return home to a demanding job. Which question is **MOST** useful for this assessment?

A. Over the past 2 weeks, how often have you felt hopeless?
B. Would you like information about local MS support groups?
C. How have you gotten through hard times before, and who helped?

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A mood screening question belongs in a full assessment and should be asked if distress deepens. It measures symptoms rather than the coping strategies and supports that this referral asks about.

B **Incorrect** (trap: Right action, wrong time). Support groups may become part of the plan, but offering a resource before assessing the patient's own coping and supports is premature. The answer to option C would show whether a group fits.

C **Correct.** Capacity to cope is judged largely from the resources a person brings: past ways of handling stress and the people who help (Lazarus & Folkman, 1984). Asking how the patient got through earlier hard times, and who helped, gathers both in one open question and starts from strengths (Hepworth et al.). That is what the neurologist requested. It beats A because a mood question measures current depressive symptoms, which matter, but it does not show how the patient handles stress or who will support them after discharge.

Takeaway: Assess coping capacity by asking how the client has handled past hardships and who helped; symptom screens do not measure coping resources.

Sources: Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, coping resources; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessing strengths and coping
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.5 Assessing resilience and coping capacity · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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3. INT-1638 (rev 3) · Best answer: A

At a senior center, an older adult in problem-solving therapy chose to ask a neighbor for rides to medical visits. At the next session, the client sighs, reports that the neighbor said no, and says, 'This method just doesn't work for me.' What should the social worker do **FIRST**?

A. Acknowledge the client's disappointment about the refusal
B. Arrange agency transportation for the client's visits
C. Revisit the earlier list of options with the client

Why each option is right or wrong

A **Correct.** A setback often brings a negative problem orientation, the sense that problems cannot be solved and effort is pointless, which problem-solving therapy treats as a target in its own right (Nezu, Nezu & D'Zurilla, 2013). Acknowledging the client's disappointment first shows understanding and eases the discouragement that would otherwise undercut the next attempt. It beats C because returning to the alternatives list is the right next step, but going straight there can feel like brushing past the letdown and may confirm the client's view that the method ignores how they feel.

B **Incorrect** (trap: Good but not best). Agency transportation could belong on the client's list of options, but booking it for the client skips the client's own problem solving. The therapy aims to build skills the client can use when the next problem arises.

C **Incorrect** (trap: Right action, wrong time). Returning to the alternatives list and choosing another option is how problem-solving therapy handles a failed solution (D'Zurilla & Goldfried, 1971), and it is the next step here. Jumping to it before acknowledging the client's discouragement risks pressing on while the client is ready to give up on the method.

Takeaway: When a chosen solution fails, acknowledge the client's discouragement first; then return to the alternatives list and try another option.

Sources: D'Zurilla, T. J., & Goldfried, M. R. (1971). Problem solving and behavior modification. Journal of Abnormal Psychology, 78(1), 107-126, whole article; Nezu, A. M., Nezu, C. M., & D'Zurilla, T. J. (2013). Problem-Solving Therapy: A Treatment Manual. Springer Publishing, Planful problem solving: solution implementation and verification

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.16 Structured problem-solving approaches · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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4. ETH-1605 (rev 2) · Best answer: B

A 27-year-old client in private practice says, 'I'm gay, but my faith is everything to me. I want you to help me get rid of these same-sex attractions.' The client reports guilt and growing isolation from both church friends and gay friends. What is the **MOST** appropriate response by the social worker?

A. Agree to work with the client on reducing same-sex attractions

B. Decline change work and offer to explore faith and identity

C. Encourage the client to leave the faith community for now

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Self-determination is a core value, but it does not require providing a practice the profession opposes as ineffective and potentially harmful. Agreeing would also reinforce the client's belief that the orientation itself is the problem.

B **Correct.** NASW opposes sexual orientation change efforts, including referral to them, and an APA task force found no reliable evidence that they work, with reports of harm (NASW, 2015; APA, 2009). Self-determination does not oblige a social worker to provide a harmful practice. The affirming response is to say so honestly and offer help with the real distress: the conflict between faith and identity, guilt and isolation. It beats C because steering the client away from the congregation imposes the worker's values on a client who calls faith central (NASW Code 1.05(c)).

C **Incorrect** (trap: Too directive). Some clients do find new communities, but this client has said faith is central. Urging distance from the congregation puts the worker's values in place of the client's and could deepen isolation.

Takeaway: Decline sexual orientation change efforts even when requested; help the client work through faith-identity conflict without imposing an outcome.

Sources: NASW (2015). Sexual Orientation Change Efforts (SOCE) and Conversion Therapy with Lesbians, Gay Men, Bisexuals, and Transgender Persons (position statement), whole statement; American Psychological Association (2009). Report of the American Psychological Association Task Force on Appropriate Therapeutic Responses to Sexual Orientation, conclusions and recommendations; NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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5. ETH-1087 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

At an adult day program, a client with mild neurocognitive disorder wants to keep paying their own bills. After weighing options and consulting a supervisor, the social worker supports the plan, and the client agrees to a weekly check-in with a trusted friend. The reasoning is documented. What should the social worker do **NEXT**?

A. Request a formal capacity evaluation of the client
B. Treat the question as resolved and close it
C. Set a date to review how the plan is working

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A capacity evaluation fits when there are signs a person cannot understand or weigh a decision. The diagnosis alone does not show that, and the choice was already weighed with consultation.

B **Incorrect** (trap: Good but not best). The decision and its documentation are complete, but neurocognitive disorders can progress. Closing the matter skips the monitoring step that would catch problems early.

C **Correct.** Ethical decision-making does not end with the decision. Reamer's framework closes with monitoring, evaluating and documenting the outcome (Reamer, 2018). A client with a progressive condition may change, so a set review date checks whether the plan still protects both autonomy and finances. C beats A because nothing in the stem suggests the client cannot understand or weigh this decision; an evaluation now would second-guess a choice already analyzed and supported.

Takeaway: After an ethical decision is made and documented, monitor and evaluate its outcome, especially when circumstances may change.

Sources: Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework: monitor and evaluate
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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6. ASM-1650 (rev 1) · Best answer: C

In the fourth session of a community agency's early-recovery group, a member mentions taking buprenorphine. Two longtime members reply that people on 'replacement drugs' are not truly sober, and several others nod. The member goes quiet. Over the past two sessions, members have made similar remarks about anyone who uses medication. What is the social worker's **BEST** response?

A. Talk with the member one-on-one once the group has ended
B. Let the debate continue so members reach their own view
C. Name the pattern and open discussion of different recovery paths
D. Suggest the member move to a recovery group for people on medication

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A private check-in is a reasonable follow-up, but the problem is a norm the whole group is building. Handling it only outside the session signals that the remarks are acceptable in group and leaves the member isolated there.

B **Incorrect** (trap: Misuses the technique). Leaving members to settle it lets the loudest voices set the norm. When a pattern shames a member and spreads misinformation about treatment, the leader steps in rather than waiting for the group to correct itself.

C **Correct.** Group norms are the shared expectations that shape what members may say and do, and leaders have the most influence on them while a group is young (Toseland & Rivas, 2017). Similar remarks across three sessions show a norm forming that shames medication use. Naming the pattern in the room and inviting talk about varied pathways corrects it for everyone; SAMHSA's recovery principles state that recovery occurs through many pathways. It beats A because a private check-in supports one member but leaves the group-level norm in place for the next member who takes medication.

D **Incorrect** (trap: Hands off too soon). Moving the member out treats the member as the problem and teaches the group that other paths are unwelcome. A specialized group could fit later if the member wants one, but it does nothing about the norm forming here.

Takeaway: When a harmful norm emerges in a group, address it with the whole group in session, not only privately with the affected member.

Sources: Toseland, R. W., & Rivas, R. F. (2017). An Introduction to Group Work Practice (8th ed.). Pearson, group dynamics: norms and social control; SAMHSA (2012) Working Definition of Recovery, Guiding principles: recovery occurs via many pathways

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

7. INT-1625 (rev 1) · Best answer: B

A client in outpatient treatment for alcohol use disorder grew up with a parent who withdrew affection after any mistake. In the fourth month, the client admits to hiding a relapse and waits to be dropped from the program. The social worker responds calmly and keeps working with the client. Weeks later, the client says, 'You didn't pull away, and now I can admit mistakes at work.' Which concept **MOST** likely explains this change?

A. Holding environment
B. Corrective emotional experience
C. Transference

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Winnicott's holding environment is the steady, safe frame of the relationship that contains a client's distress. It is part of the background here, but the change follows one specific event: an expected rejection that did not come. That points to a corrective emotional experience.

B **Correct.** Alexander and French (1946) described the corrective emotional experience: a client relives an old relational conflict with the helper, the helper responds differently from the original figure, and the new outcome loosens the old expectation. Here the client expects the parent's withdrawal after confessing a relapse, the social worker stays steady, and the client then risks admitting mistakes elsewhere. It beats A because a holding environment (Winnicott, 1965) describes the reliable, containing frame that makes such work possible, not the specific disconfirmation of an expected response that produced this change.

C **Incorrect** (trap: Wrong model or concept). The client's expectation of being dropped is transference: a response carried over from the parent. Transference explains the fear before the confession, not the change afterward. The shift came from the social worker responding differently than the parent did.

Takeaway: When a helper responds differently than early caregivers did to a replayed conflict, the corrective emotional experience can revise the client's expectations.

Sources: Alexander, F., & French, T. M. (1946). Psychoanalytic Therapy: Principles and Application. Ronald Press, Corrective emotional experience; Winnicott, D. W. (1965). The Maturational Processes and the Facilitating Environment. Hogarth Press, Holding environment

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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8. ETH-1543 (rev 2) · Best answer: B

A client in a group practice has 2 sessions left before insurance benefits run out for the year. The client still has frequent panic attacks and cannot afford to pay out of pocket. An appeal to the insurer was denied. What should the social worker do **FIRST**?

A. End services after the last covered session, with referrals
B. Discuss continuation options, such as a reduced fee
C. Keep meeting weekly and let the unpaid balance build

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Referrals may be the right outcome, and they belong in any ending plan. Deciding to end before discussing alternatives with the client, though, skips the client's preferences and options such as a reduced fee.

B **Correct.** Social workers who anticipate an interruption notify clients promptly and seek transfer, referral or continuation based on the client's needs and preferences (NASW Code 1.17(e)). With 2 covered sessions left and ongoing panic, the first step is a joint conversation about options: a reduced fee, less frequent sessions, a lower-cost program or a planned transfer. It beats A because referrals may well be part of the plan, but choosing termination before reviewing options with the client skips the client's preferences and risks a needless break in care (NASW Code 1.17(b)).

C **Incorrect** (trap: Good but not best). Letting a balance build without a clear agreement creates debt the client cannot pay and may later force an abrupt ending for nonpayment. Fees should be discussed openly and consider the client's ability to pay (NASW Code 1.13(a)).

Takeaway: When coverage ends mid-treatment, first discuss continuation options with the client, such as reduced fees or transfer, before ending services.

Sources: NASW Code of Ethics (2021), 1.17(b), (e); NASW Code of Ethics (2021), 1.13(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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9. ETH-1048 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

At a group home for adults with intellectual developmental disorder, a resident tells the social worker that a night staff member slapped them, and the social worker sees a red mark on the resident's cheek. The social worker informs the house manager, who says the resident 'makes things up' and plans an internal review next week. What should the social worker do **NEXT**?

A. Report the incident to the designated outside agency
B. Wait for the results of the internal review
C. Ask the staff member for an account of the night

Why each option is right or wrong

A **Correct.** Suspected abuse of a dependent adult by facility staff goes to the outside agency that state law designates, and a manager's doubts about the resident do not lower the threshold. The resident's account plus a visible mark give reasonable suspicion. HIPAA permits disclosures about abuse victims that law requires, limited to what the law asks (45 CFR 164.512(c)(1)(i)). A beats B because an internal review does not replace an outside investigation, and next week is too long for a resident still exposed to the staff member.

B **Incorrect** (trap: Misreads law or ethics). Internal reviews help agencies correct practice, but they do not replace the outside investigation that abuse laws call for. Waiting a week leaves the resident exposed and lets the facility judge itself.

C **Incorrect** (trap: Outside the social work role). Questioning the accused staff member is investigative work that belongs to the designated agency. It could alert the staff member, shape the account, and put pressure on the resident before investigators arrive.

Takeaway: Doubts about an alleged victim's credibility do not replace an outside report; suspected staff abuse goes to the designated investigating agency.

Sources: HIPAA Privacy Rule, 45 CFR 164.512(c)(1)(i)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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10. INT-1091 (rev 3) · Best answer: D

A home health social worker is assigned to a 78-year-old whose spouse of 50 years died 6 weeks ago. Before the first visit, an adult child calls, says the client 'cries all the time and talks to the photo every morning,' and asks the worker to arrange antidepressant medication. What should the social worker do **FIRST**?

A. Refer the client to a prescriber for an antidepressant evaluation
B. Begin structured treatment for prolonged grief
C. Help the family see these reactions as expected grief
D. Meet with the client to explore how they are living with the loss

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A medication evaluation fits if assessment shows a major depressive episode, such as pervasive worthlessness, loss of functioning or suicidal thoughts. Arranging it from a relative's phone report skips the client's own account and may treat expected sadness as illness.

B **Incorrect** (trap: Near-miss diagnosis). Structured prolonged grief treatment is for grief that stays intense and impairing at least 12 months after the death in adults (DSM-5-TR). At 6 weeks it is premature, and nothing yet shows impairment.

C **Incorrect** (trap: Right action, wrong time). Daily crying and talking to a photo are common in early grief, and an ongoing inner bond with the person who died is often part of healthy mourning (Klass et al., 1996). Telling the family so is the right message once the worker has met the client; said first, it rests only on a relative's report and could miss depression or risk.

D **Correct.** Assessment comes before intervention, and the person to assess is the client. So far the worker has only the adult child's description. Exploring how the client is experiencing the loss, including mood, self-worth, sleep, daily routines, supports and any thoughts of death, separates expected grief from a major depressive episode (DSM-5-TR) and lets the client's own wishes guide what follows. It beats C because reassuring the family before meeting the client could miss depression or suicide risk; normalizing fits once the assessment supports it.

Takeaway: When family asks for treatment for a grieving relative, first explore the loss with the client directly; normalize or refer only after that.

Sources: Klass, D., Silverman, P. R., & Nickman, S. L. (Eds.). (1996). Continuing Bonds: New Understandings of Grief. Taylor & Francis, continuing bonds as part of normal grief; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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11. ASM-1002 (rev 1) · Best answer: D

During a telehealth session, a 31-year-old client describes 5 days last month of sleeping 3 hours a night without feeling tired. The client talked rapidly, started several work projects and felt 'brilliant.' A coworker commented on the change, but the client kept working and met deadlines. Last year the client was treated for a major depressive episode. What is the **MOST** likely diagnosis?

- **A.** Bipolar I disorder
- **B.** Cyclothymic disorder
- **C.** Major depressive disorder
- **D.** Bipolar II disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Bipolar I requires a manic episode, which causes marked impairment, leads to hospitalization or includes psychosis. A 5-day period in which the client kept working and met deadlines describes hypomania, not mania.

B **Incorrect** (trap: Near-miss diagnosis). Cyclothymic disorder involves at least 2 years of numerous milder highs and lows that never meet full episode criteria. This client had a full hypomanic episode and a major depressive episode, so cyclothymia does not fit.

C **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder applies only when there has never been a manic or hypomanic episode. The recent 5-day high period changes the diagnosis and matters for treatment planning.

D **Correct.** Bipolar II disorder requires at least one hypomanic episode and at least one major depressive episode, with no lifetime manic episode (DSM-5-TR). Five days of reduced need for sleep, rapid speech, increased activity and inflated self-view, noticed by others but without marked impairment, meets hypomania. The past major depressive episode completes the picture. It beats A because bipolar I requires a manic episode: at least 1 week (or any length if hospitalized) with marked impairment, hospitalization or psychosis. This client kept working and met deadlines.

Takeaway: Hypomania (4+ days, noticed by others, no marked impairment) plus a major depressive episode is bipolar II; any manic episode makes it bipolar I.

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar II Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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12. INT-1102 (rev 3) · Best answer: B

A client has taken buprenorphine for 8 months and is stable at work and home. In a telehealth session, the client mentions that people at a mutual-help meeting call medication 'not real recovery' and says, 'Maybe I should just stop it next week.' What should the social worker do **FIRST**?

A. Accept the plan and schedule weekly check-ins during the stop
B. Ask what the client heard and how they now see the medication
C. Suggest a mutual-help group that welcomes members on medication
D. Help the client meet with the prescriber before any change

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Respecting choice matters, but the client has voiced a 'maybe,' not a settled decision. Simply monitoring an abrupt stop leaves out both the client's own reasons and the medical guidance any stop would need; check-ins fit once a change is planned with the prescriber.

B **Correct.** The client's 'maybe' after criticism at a meeting signals ambivalence, and the worker does not yet know what the client heard, believes or wants. Motivational interviewing meets ambivalence by exploring it, so the client can voice their own reasons for and against staying on medication (Miller & Rollnick, 2023). Asking first also shows what misinformation to address and keeps the choice with the client (NASW Code 1.02). It beats D because a prescriber visit is the right step if the client still intends to stop, but arranging it before hearing the client out treats a passing doubt as a decision; with a week to go, there is time to explore now and link to the prescriber next.

C **Incorrect** (trap: Right action, wrong time). A group that accepts medication-assisted recovery may ease the pressure and is a good later step. Suggesting it now assumes the meeting is the whole story before the worker knows what the client heard or what they want.

D **Incorrect** (trap: Right action, wrong time). Medication decisions belong with the prescriber, and stopping buprenorphine abruptly risks withdrawal and overdose once tolerance drops; most people who stop medication return to opioid use (SAMHSA TIP 63, 2021). A prescriber visit is therefore the next step if the client still plans to stop. Arranging it first skips learning what the client thinks and wants.

Takeaway: When a client on MOUD voices doubts about continuing, explore their reasons first; link to the prescriber before any planned change.

Sources: Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Evoking: exploring ambivalence; SAMHSA TIP 63 (2021), Part 1: Introduction to Medications for Opioid Use Disorder Treatment; NASW Code of Ethics (2021), 1.02

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

13. ETH-1085 (rev 3) · Best answer: A

A hospital social worker has one remaining charity-funded home health slot this month, and two patients being discharged qualify. The program's written criteria rank applicants by medical risk at home. A nurse suggests giving the slot to the patient who asked about it first. What is the **BEST** way for the social worker to decide?

A. Use the established risk ranking for both patients
B. Give the slot to the patient who asked first
C. Ask the program director to choose between the patients

Why each option is right or wrong

A **Correct.** When resources cannot meet every need, allocation should be open and fair, nondiscriminatory, and based on principles applied consistently (NASW Code 3.07(b)). The program already has written criteria based on medical risk, so applying them to both patients is the fair and defensible choice. A beats B because first-come order is consistent in itself but is not the program's stated standard, and it rewards knowing to ask rather than need.

B **Incorrect** (trap: Good but not best). First-come order can be a fair rule when a program adopts it in advance. Here the written criteria rank by medical risk, and switching standards case by case makes allocation inconsistent.

C **Incorrect** (trap: Hands off too soon). A director can settle a true tie or a question about the criteria. Here the written risk ranking already supplies the standard the social worker is expected to apply, so passing the decision up avoids a choice the policy makes possible and delays both discharges.

Takeaway: Allocate scarce services by open, consistently applied criteria rather than case-by-case preference.

Sources: NASW Code of Ethics (2021), 3.07(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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14. ETH-0001 (rev 1) · Best answer: B

During a session at a community mental health agency, a client says they have saved a supply of pills and plan to take them tonight. The client then leaves the office and does not answer phone calls. The social worker calls emergency services. What information is **BEST** for the social worker to share with the dispatcher?

A. A general description of the risk without the client's name
B. The client's name, likely location and stated overdose plan
C. The client's diagnosis, treatment history and recent session notes

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Leaving out the client's name protects privacy, but responders cannot find or help a person they cannot identify. When a threat to life is imminent, the Code permits disclosing identifying information to those who can prevent the harm.

B **Correct.** Confidentiality gives way when disclosure is needed to prevent serious, foreseeable and imminent harm, but only the information needed for that purpose may

be shared (NASW Code 1.07(c)). Responders need to know who the client is, where to look and what the danger is, so B is the minimum disclosure that can still protect the client. It beats C because diagnosis, history and session content do not help responders find the client. Sharing them breaches confidentiality with no safety benefit. HIPAA likewise permits a good-faith disclosure to people able to prevent a serious and imminent threat (45 CFR 164.512(j)).

C **Incorrect** (trap: Misreads law or ethics). Diagnosis, treatment history and session notes go beyond what responders need to locate the client and respond. Sharing them breaks the minimum-necessary limit on safety disclosures, even though some disclosure is justified here.

Takeaway: In an emergency disclosure, share only what responders need to prevent the harm: who, where and what danger.

Sources: NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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15. ASM-1127 (rev 1) · Best answer: A

On an inpatient unit, the team plans to diagnose schizophrenia in a Black adult who is guarded, distrusts staff and declines to share personal details. The social worker's interview finds 5 weeks of early waking, slowed movement, poor appetite and intense guilt. The client says past providers 'never listened.' What is the **MOST** appropriate action for the social worker?

A. Press the team for a full evaluation of a depressive episode
B. Explore the client's earlier experiences with mental health providers
C. Support the working diagnosis given the persistent suspicion

Why each option is right or wrong

A **Correct.** A diagnosis must account for all the data. DSM-5-TR warns that schizophrenia is overdiagnosed and mood disorders underrecognized in some racialized groups, including Black Americans, partly because clinicians misread culturally grounded mistrust as paranoia. The client has 5 weeks of classic depressive symptoms, so the team should assess for a major depressive episode, possibly with psychotic features, before settling on schizophrenia. It beats B because understanding past mistreatment helps the alliance but leaves the misdiagnosis risk in place (DSM-5-TR).

B **Incorrect** (trap: Good but not best). Learning how earlier providers treated the client builds trust and may explain the guardedness. It does not, by itself, address the depressive syndrome the team is overlooking, which matters most for the diagnosis and care.

C **Incorrect** (trap: Near-miss diagnosis). Guardedness and distrust after poor past care are not, by themselves, evidence of delusions. Supporting schizophrenia now ignores weeks of depressive symptoms and repeats a documented pattern of misdiagnosis.

Takeaway: Before accepting a psychotic diagnosis, weigh mood symptoms and culturally grounded mistrust, which are often misread as paranoia in racialized clients.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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16. INT-1568 (rev 1) · Best answer: C

A home-based case management program for older adults serves clients with very different aims. One wants to attend a senior center weekly, another to pay bills on time, and a third to keep medical appointments. The director wants one evaluation method that tracks each client's own targets and yields scores that can be compared across clients. Which method is **BEST**?

- **A.** Self-anchored rating scale
- **B.** Geriatric Depression Scale
- **C.** Goal Attainment Scaling
- **D.** Behavioral frequency count

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A self-anchored scale lets each client define the end points of a rating, which suits individual tracking. It does not combine several goals into a standardized score, so comparing clients is difficult.

B **Incorrect** (trap: Wrong model or concept). The Geriatric Depression Scale is a standardized screen for one construct, depression. It does not measure progress on attendance, bill paying or appointments.

C **Correct.** Goal Attainment Scaling sets individualized goals for each client, defines expected outcome levels for each goal from -2 to +2, and converts results to a standard score that can be compared across clients (Kiresuk & Sherman, 1968). It fits a program whose clients pursue very different aims. It beats A because a self-anchored scale is individualized but tracks one dimension at a time and offers no standard way to combine goals or compare clients.

D **Incorrect** (trap: Wrong model or concept). Counting behaviors works well for one observable target, such as visits attended. Counts of different behaviors across clients cannot be combined or compared on a common metric.

Takeaway: Goal Attainment Scaling measures progress on individualized goals and converts it to a standard score comparable across clients.

Sources: Kiresuk, T. J., & Sherman, R. E. (1968). Goal attainment scaling: A general method for evaluating comprehensive community mental health programs. Community Mental Health Journal, 4(6), 443-453, whole article
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 2 of 3

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17. INT-1020 (rev 1) · Best answer: C

A client in a DBT skills group feels intense shame and wants to stay home after stumbling over words in a work presentation. With the social worker, the client has already checked the facts and concluded the shame does not fit, since no personal values were violated. Which skill is **BEST** to teach next?

A. Radical acceptance
B. Interpersonal effectiveness
C. Opposite action
D. Self-soothing

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Radical acceptance means fully accepting a reality that cannot be changed. It belongs to distress tolerance, and here the client faces not an unchangeable reality but an emotion that does not fit the facts, with an urge to avoid.

B **Incorrect** (trap: Wrong model or concept). Interpersonal effectiveness skills help a client ask for something or say no while keeping relationships and self-respect. The client's problem is shame and an urge to withdraw, not a request to make of someone.

C **Correct.** In DBT emotion regulation, when checking the facts shows an emotion does not fit the situation, the next step is opposite action: acting against the emotion's urge (Linehan, 2015). Shame's urge is to hide, so the opposite is to show up, such as returning to work and making eye contact with colleagues. It beats A because radical acceptance is a distress-tolerance skill for facts that cannot be changed; here the problem is an emotion that does not fit the facts and an urge that can be acted against.

D **Incorrect** (trap: Wrong model or concept). Self-soothing uses the senses to get through a painful moment. It may ease distress but leaves the urge to withdraw unchallenged, so avoidance and shame can grow.

Takeaway: When an emotion doesn't fit the facts, DBT's opposite action means acting against its urge; radical acceptance is for unchangeable realities.

Sources: Linehan, M. M. (2015). DBT Skills Training Manual (2nd ed.). Guilford Press, emotion regulation: check the facts and opposite action

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.6 Teaching emotion-regulation skills · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 2 of 3

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18. ETH-1556 (rev 1) · Best answer: B

A supervisee at a youth services agency invites their clinical supervisor to the public commencement where the supervisee will receive an MSW. The invitation also includes a family dinner at the supervisee's home afterward. Supervision will continue for another year. What is the **MOST** appropriate response by the supervisor?

A. Politely decline both the public ceremony and the family dinner
B. Attend the public ceremony and decline the family dinner
C. Attend the public ceremony and the family dinner

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Declining everything looks safe, but it treats a brief, public, professional event as a violation. It can feel rejecting at a milestone and misses the difference between low-risk crossings and harmful dual relationships.

B **Correct.** Supervisors set clear, culturally sensitive boundaries and avoid dual relationships with supervisees that risk exploitation or harm (NASW Code 3.01(b), 3.01(c)). Briefly attending a public commencement is a low-risk boundary crossing that recognizes a professional milestone. A private family dinner moves toward a social relationship that could blur a year of evaluation ahead. It beats A because the Code bars risky dual relationships, not all contact outside the office, and a crossing with a clear purpose need not become a violation (Reamer, 2020).

C **Incorrect** (trap: Boundary slip or collusion). Joining a private family dinner while supervision continues for a year mixes social and evaluative roles. The supervisor's power over evaluations makes a social relationship risky, however sincere the invitation.

Takeaway: Low-risk, public boundary crossings with supervisees can be appropriate; avoid social ties that blur an ongoing evaluative role.

Sources: NASW Code of Ethics (2021), 3.01(b); NASW Code of Ethics (2021), 3.01(c); Reamer (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, Boundary crossings versus boundary violations

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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19. ETH-1625 (rev 1) · Best answer: B

At a drop-in center for youth experiencing homelessness, a 16-year-old who left home after coming out as gay tells the social worker about staying with adults who offer a bed in exchange for sex. The youth says, 'Nobody makes me. It's my choice.' No third party arranges the meetings. Under federal law, how is this situation classified?

A. Survival sex, since no one else arranges or profits

B. Sex trafficking of a minor

C. Trafficking if force or coercion can be shown

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Survival sex describes trading sex to meet basic needs such as food or shelter. It names the youth's motive, not the legal status. Federal law needs no third-party profiteer: an adult who obtains or patronizes a minor for a sex act in exchange for a bed is trafficking.

B **Correct.** Federal law defines a commercial sex act as any sex act exchanged for anything of value. It treats a person under 18 induced into such an act as a victim of a severe form of trafficking, with no need to prove force, fraud or coercion (22 U.S.C. 7102(4), (11)(A)). A bed is something of value, and the youth is 16. It beats A because 'survival sex' describes why the youth agrees, not how the law classifies it. Treating the exchange as a choice can leave a marginalized youth without protection, and the Code calls for action against exploitation (NASW Code 6.04(d)).

C **Incorrect** (trap: Misreads law or ethics). Force, fraud or coercion must be shown when the person in commercial sex is an adult. For anyone under 18, federal law drops that element, so the youth's description of the exchange as a personal choice does not change the classification.

Takeaway: A minor who trades sex for anything of value, including a place to sleep, is a trafficking victim under federal law; no force must be shown.

Sources: Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(4), (11)(A); NASW Code of Ethics (2021), 6.04(d)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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20. ASM-0006 (rev 1) · Best answer: C

A client tells the social worker, 'I know my drinking is hurting my relationship, and part of me wants to cut back. But I'm not sure I'm ready to give up my evening drinks.' Which stage of change is the client **MOST** likely in?

A. Precontemplation
B. Preparation
C. Contemplation

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Precontemplation means the person does not yet see a problem or is not considering change. This client names the harm and wants to cut back, which places them past precontemplation.

B **Incorrect** (trap: Wrong model or concept). Preparation combines an intention to change soon with concrete steps or a plan. The client voices doubt about readiness and describes no plan, so preparation is premature.

C **Correct.** Contemplation is marked by ambivalence: the person sees reasons to change and reasons not to, without a commitment to act soon (Prochaska & DiClemente, 1983; SAMHSA TIP 35). The client recognizes the harm and wants to cut back but is unsure about giving up drinking. It beats B because preparation involves intending to act soon and taking small steps, such as setting a date, and this client has made no commitment or plan. Stage-matched work here means exploring and resolving ambivalence (TIP 35, Ch. 5).

Takeaway: Ambivalence, seeing reasons to change without committing to act, marks contemplation; explore and resolve the ambivalence.

Sources: SAMHSA TIP 35 (2019), Ch. 5: From Contemplation to Preparation; Prochaska, J. O., & DiClemente, C. C. (1983). Stages and processes of self-change of smoking: Toward an integrative model of change. Journal of Consulting and Clinical Psychology, 51(3), 390-395, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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21. ETH-1614 (rev 1) · Best answer: B

A caregiver support group at a nonprofit agency meets in a second-floor room with no elevator. A new member who uses a wheelchair wants to join. The program director suggests that the social worker offer this member individual sessions instead. A ground-floor conference room is free at the group's meeting time. What should the social worker do?

A. Offer the member individual sessions as the director suggests
B. Move the group to the accessible downstairs room
C. Have the member join the group by video from downstairs

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Individual sessions may help some caregivers, but offering them in place of the group gives the member a separate service because of disability. The member wants the group, and an accessible room is free.

B **Correct.** Agencies open to the public must serve people with disabilities in the most integrated setting appropriate to their needs, and offering a separate service does not justify exclusion from the regular program (28 CFR 36.203). An accessible room is free at the same time, so moving the group is simple and lets the member take part fully. It beats C because joining by video from another room keeps the member apart from peers when an equal, in-person option is readily available.

C **Incorrect** (trap: Good but not best). Video can widen access when no accessible space exists. Here it would seat the member apart from the group when an accessible room is free, so it falls short of equal participation.

Takeaway: Serve clients with disabilities in the most integrated setting: fix access to the regular program rather than offering a separate service.

Sources: ADA Title III regulations, 28 CFR 36.203
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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22. ASM-1050 (rev 1) · Best answer: B

An adult with schizophrenia in supportive housing has said for 2 days that 'the devil is twisting my stomach.' The client has voiced similar beliefs before. Today the client is hunched over, has vomited twice, refuses meals and feels warm to the touch. Staff ask the social worker to treat it as a familiar delusion. What should the social worker do **FIRST**?

A. Ask the psychiatrist to review the antipsychotic dose
B. Arrange a same-day medical evaluation of the stomach symptoms
C. Explore what the stomach image means to the client

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A medication review may help if psychotic symptoms are worsening, but it treats the complaint as purely psychiatric. Vomiting, food refusal and warmth are physical signs that need medical evaluation first; the psychiatrist can be updated afterward.

B **Correct.** People with serious mental illness have higher rates of physical illness, and their physical complaints are often wrongly credited to the psychiatric condition, a pattern called diagnostic overshadowing. Vomiting, refusing food and feeling warm are new physical signs that need same-day medical evaluation, whatever the delusional framing. This beats A because a medication review addresses the psychosis but leaves a possible abdominal emergency, such as appendicitis, unexamined (Jones, Howard & Thornicroft, 2008; De Hert et al., 2011).

C **Incorrect** (trap: Ignores risk). Exploring the meaning of a delusional image can support engagement once the client is medically safe. Doing it first leaves possible acute illness unassessed while the client vomits and refuses food.

Takeaway: New physical signs in a client with serious mental illness need medical evaluation, even when the complaint is voiced through a delusion.

Sources: Jones, S., Howard, L., & Thornicroft, G. (2008). 'Diagnostic overshadowing': Worse physical health care for people with mental illness. Acta Psychiatrica Scandinavica, 118(3), 169-171, whole article; De Hert, M., et al. (2011). Physical illness in patients with severe mental disorders. I. Prevalence, impact of medications and disparities in health care. World Psychiatry, 10(1), 52-77, pp. 52-77

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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23. INT-1636 (rev 2) · Best answer: D

In an integrated primary care clinic, a client with moderate depression says, 'My license is suspended, so I can't get to work. That has to come first.' The social worker believes the depression should be treated before anything else. What is the **BEST** way to set the first goal?

A. Make treating the depression the first goal of the contract
B. Ask the client to rank the problems in order of importance
C. Work on the license and the depression as equal goals
D. Start with the steps to reinstate the license

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Putting the social worker's priority first overrides the client's stated view and risks a contract the client does not own. Depression can be monitored and addressed as the work proceeds unless risk requires otherwise.

B **Incorrect** (trap: Right action, wrong time). Ranking problems helps when priorities are unclear. This client has already said what comes first and why, so asking again delays goal setting and may signal that the answer was not heard.

C **Incorrect** (trap: Good but not best). Including both problems sounds thorough, but with moderate depression, splitting effort across two goals can overwhelm. Partializing starts with one manageable, client-chosen target and adds others later.

D **Correct.** Goals should reflect what the client wants to change; starting where the client is builds motivation and commitment to the contract (Hepworth et al.). The client has said why the license matters most, since without it there is no way to get to work, and the problem is concrete and solvable. Early success on a client-chosen goal can also lift mood, and mood can be monitored as the work proceeds. It beats C because

tackling both at once spreads limited energy in a client with depression; partializing means choosing one starting point, and the client has named it.

Takeaway: Set the first goal around the client's own priority unless safety dictates otherwise; client-owned goals build motivation and commitment.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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24. ETH-1074 (rev 1) · Best answer: A

In Medicare billing, what does 'incident-to' billing refer to?

- **A.** Billing staff services under a supervising practitioner's number
- **B.** Billing an emergency service code after a critical incident
- **C.** Billing services under the staff member's own provider number
- **D.** Billing a liability insurer for care after an accident

Why each option is right or wrong

A **Correct.** Medicare's incident-to rules let services furnished by auxiliary personnel be billed under the supervising physician's or practitioner's provider number, but only when specific conditions, including required supervision, are met, and only the supervising practitioner may bill (42 CFR 410.26(b)). The ethics point is that claims must accurately show who provided the service (NASW Code 3.05); billing this way when the conditions are not met is fraud. A beats C because incident-to is defined by billing under the supervisor's number, not the staff member's own.

B **Incorrect** (trap: Wrong model or concept). The word 'incident' here refers to services that are incidental to a practitioner's care, not to critical incidents or crisis events.

C **Incorrect** (trap: Wrong model or concept). Billing under the staff member's own number is ordinary direct billing by an enrolled provider. Incident-to is the opposite arrangement: the service is billed under the supervising practitioner.

D **Incorrect** (trap: Wrong model or concept). Seeking payment from a liability insurer after an accident is a different process. Incident-to concerns who furnished a service versus whose number it is billed under.

Takeaway: Incident-to billing places auxiliary staff services under a supervisor's number only when payer conditions are met; otherwise it misstates the provider.

Sources: Medicare Part B regulations, 42 CFR 410.26(b); NASW Code of Ethics (2021), 3.05

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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25. INT-1555 (rev 3) · Best answer: C

A home hospice patient with end-stage lung disease has said many times that they want to die at home. The patient's partner, the only caregiver, has slept little for 2 weeks. As the social worker arrives for a visit, the partner says quietly, 'I don't know how much longer I can do this.' What should the social worker do **FIRST**?

A. Recommend moving the patient to a nursing facility for the remaining time
B. Arrange a short inpatient respite stay to relieve the partner
C. Explore with the partner what feels unmanageable right now

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Placement may become necessary if home care cannot be sustained, but it gives up the patient's clearly stated wish for a home death before less drastic supports have been tried (NASW Serious Illness Care Standard 1).

B **Incorrect** (trap: Right action, wrong time). Short inpatient respite, up to 5 consecutive days under the Medicare hospice benefit (42 CFR 418.204), may be exactly what this partner needs, and it protects the plan for a home death. Arranging it before hearing the partner out assumes the problem is only lost sleep and skips checking whether guilt, grief or hopelessness also needs attention.

C **Correct.** Assessment comes before intervention (Hepworth et al.). 'I don't know how much longer I can do this' could mean exhaustion from night care, guilt about the patient's wish for a home death, anticipatory grief, or something more serious such as hopelessness. Exploring what the partner means shows which help fits, whether respite, more in-home hours or support around the home-death wish, and whether the caregiver's own safety needs attention. It beats B because respite is the likely next step, but arranging it before understanding the statement could miss part of what the partner is facing.

Takeaway: When a hospice caregiver says they cannot go on, explore what that means before arranging respite or other help.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: exploring and understanding problems and strengths; Medicare hospice regulations, 42 CFR Part 418, 42 CFR 418.204(b) (respite care); NASW Practice Standards for Serious Illness Care: Hospice and Palliative Social Work, Standard 1: Ethics and Values

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.5 End-of-life practice: hospice, palliative care, continuity, dying stages · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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26. INT-0103 (rev 1) · Best answer: C

A grant proposal includes the statement 'By the end of year one, 75% of enrolled caregivers will show reduced stress on a standardized scale.' In which proposal section does this statement belong?

A. Statement of need
B. Program goals
C. Program objectives
D. Evaluation plan

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The statement of need documents the problem the program addresses, usually with local data and research. It describes current conditions, not the change the program promises to achieve.

B **Incorrect** (trap: Wrong model or concept). Goals describe the broad, long-term result a program works toward, such as better caregiver well-being. A target with a percentage and a deadline is too specific to be a goal.

C **Correct.** Objectives are specific, measurable, time-bound statements of the change a program will produce (Coley, Scheinberg & Levites Strekalova, 2021; Kettner, Moroney & Martin, 2022). This statement names who will change (enrolled caregivers), by how much (75%), how change will be measured (a standardized scale) and by when (end of year one), so it is an outcome objective. It beats B because goals are broad statements of intended direction, such as improving caregiver well-being, and are not written with numeric targets or deadlines.

D **Incorrect** (trap: Wrong model or concept). The evaluation plan explains how progress on the objectives will be measured, such as data sources, instruments and timing. It refers to the objectives rather than stating them.

Takeaway: In proposals, the need statement documents the problem, goals state broad intent, and objectives give measurable, time-bound targets.

Sources: Coley, S. M., Scheinberg, C. A., & Levites Strekalova, Y. A. (2021). Proposal Writing: Effective Grantsmanship for Funding (6th ed.). SAGE, Ch. 7, Program Objectives and Evaluation; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7, program goals and objectives

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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27. ETH-1656 (rev 2) · Best answer: A

In the tenth session of trauma-focused work at a private practice, a client sits quietly for several minutes, then says, 'I think I'm done coming here,' and looks away. Sleep and work have improved. The previous session focused on the most painful memory so far. What should the social worker do **FIRST**?

A. Invite the client to say more about wanting to stop

B. Agree to end and plan a final session to review gains

C. Recommend continuing until the trauma processing is finished

Why each option is right or wrong

A **Correct.** A sudden wish to end deserves to be understood before it is acted on: it may reflect readiness, avoidance after a hard session, or a rupture the client has not named (Hepworth et al.). This remark is abrupt, follows the most painful session so far and comes with withdrawal, so inviting the client to say more comes first. A competent client may still end services (NASW Code 1.02, 1.17(a)); exploring keeps that choice informed rather than overriding it. It beats B because agreeing at once may close the door on an avoidance pattern or rupture that could be worked through.

B **Incorrect** (trap: Right action, wrong time). Honoring a client's decision to end, with a planned closing session, is right once the decision is clear (NASW Code 1.17(a)).

Agreeing on the spot to an abrupt remark made after a painful session skips learning whether the client is ending from strength or retreating from something the work stirred up.

C **Incorrect** (trap: Overrides the client's choice). The worker may share an honest view once the client's reasons are known, but pressing a client to continue until the worker is satisfied overrides the client's judgment about their own care. Better sleep and work may also mean the client is ending from strength.

Takeaway: When a client's wish to end services is sudden or unexplained, explore it before agreeing or persuading; then honor an informed choice.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.17(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, termination: client-initiated endings
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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28. INT-1689 (rev 1) · Best answer: A

A family services agency will begin evening home visits next quarter. The director asks a senior social worker to draft a field safety policy. Over the past 2 years, staff have filed several incident reports about daytime visits, and some workers have described near misses at staff meetings. What should the social worker do **FIRST**?

A. Review past visit problems with frontline staff
B. Adapt a neighboring agency's evening visit policy
C. Require two workers on every evening visit
D. Schedule de-escalation training for field staff

Why each option is right or wrong

A **Correct.** Risk-reducing policy starts with assessing the agency's actual risks. NASW's safety guidelines call for analyzing past incidents to find gaps in protocols and for assessing risk before field visits (NASW, 2013). The agency already has 2 years of incident reports and staff accounts of near misses, the most relevant data for designing evening visit rules. It beats C because a blanket two-worker rule may fit some visits but, set before the risks are known, could halve visit capacity without addressing the real hazards (Reamer, 2015).

B **Incorrect** (trap: Good but not best). Another agency's policy can be a useful model during drafting, but copied now it reflects that agency's clients, neighborhoods and staffing rather than the risks in this agency's own incident history.

C **Incorrect** (trap: Skips assessment). Paired visits are a recognized option for higher-risk visits. Mandated for every visit before reviewing incidents, the rule may miss the actual hazards and cut visit capacity unnecessarily.

D **Incorrect** (trap: Right action, wrong time). Training is a core part of a safety program and belongs in the policy. Planned first, it cannot be matched to the specific risks the incident review would reveal.

Takeaway: Build risk-reducing policies on an assessment of the agency's own incident data and staff experience before choosing rules or training.

Sources: NASW (2013). Guidelines for Social Worker Safety in the Workplace, Standard 2: Prevention; Standard 6: Risk Assessment for Field Visits; Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 1: Professional Risk Management: An Overview

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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29. INT-1095 (rev 1) · Best answer: C

A street outreach social worker has met for months with an adult who has schizophrenia and alcohol use disorder and has lived outdoors for 6 years. The client declines treatment but says, 'I'd take an apartment if I didn't have to quit drinking.' The worker's program follows a Housing First model. Which action is consistent with that model?

A. Refer the client to a sober transitional apartment first

B. Keep building rapport until the client accepts treatment

C. Offer a permanent apartment with services the client may choose

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Transitional programs that require sobriety follow a treatment-first, housing-readiness approach. Housing First removes those preconditions, so making housing depend on abstinence contradicts the model the program follows.

B **Incorrect** (trap: Good but not best). Continued engagement matters, but waiting for treatment acceptance before offering housing is the readiness approach that Housing First replaces. Housing comes first; treatment is offered, not required.

C **Correct.** Housing First offers immediate access to permanent housing without requiring sobriety or treatment, and pairs it with voluntary, client-directed support services (Tsemberis, 2010). The client is ready for housing and declines treatment, the situation the model was built for. Treatment stays available and is often accepted later, once basic stability exists. It beats A because a sober transitional step is a treatment-first approach that makes housing depend on abstinence, the very condition this client rejects.

Takeaway: Housing First provides permanent housing without sobriety or treatment requirements, with voluntary support services offered alongside.

Sources: Tsemberis, S. (2010). Housing First: The Pathways Model to End Homelessness for People with Mental Illness and Addiction. Hazelden, core principles: immediate housing, consumer choice, separation of housing and treatment

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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30. ETH-1103 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

A clinic's waitlist for therapy intake has grown to about 6 weeks. Before a meeting with hospital discharge planners, the clinic director tells the outreach social worker to keep promising 'appointments within the week' so referrals and funding stay steady. Several patients referred last month waited weeks without care. What should the social worker do **FIRST**?

- **A.** Describe the program to planners as the director instructed
- **B.** Tell the director that planners need the actual wait time
- **C.** Report the director's instruction to the clinic's board
- **D.** Give planners the actual wait without telling the director

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Repeating a claim the social worker knows is false makes the worker part of the deception (4.04). An employer's instruction does not excuse misrepresenting services to the public (3.09(d)).

B **Correct.** Social workers make sure their representations to the public about services are accurate, and they do not take part in deception (NASW Code 4.06(c), 4.04; Integrity principle). Promising same-week care when the wait is 6 weeks misleads referral sources, and patients have already gone without timely care. Social workers make employers aware of their ethical obligations (3.09(c)) and do not let administrative orders interfere with ethical practice (3.09(d)), so a direct conversation with the director comes first. B beats D because talking with the director first gives the agency a chance to correct its message and plan for referrals; accurate information to planners follows either way.

C **Incorrect** (trap: Skips assessment). Going to the board may become necessary if the director insists on misleading referral sources. Doing it before any conversation skips the direct step the Code expects first.

D **Incorrect** (trap: Right action, wrong time). Planners must get accurate information, but bypassing the director leaves the instruction in place for other staff and later meetings. Raising it first lets the agency fix its message; accuracy at the meeting is required whatever the director says.

Takeaway: Descriptions of services must be accurate; when told to mislead, raise it with the decision-maker first and never repeat the false claim.

Sources: NASW Code of Ethics (2021), 4.06(c); NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 3.09(c); NASW Code of Ethics (2021), Ethical Principles: Integrity
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

31. ETH-1619 (rev 2) · Best answer: B

A 14-year-old joined a parent in the United States 4 months ago after living 7 years with a grandparent in the home country while the parent worked here. The school social worker learns the teen is withdrawn, cries about the grandparent and refuses to follow the parent's rules. The parent calls the teen ungrateful. Which approach is **BEST**?

A. Help the parent set firmer rules with clear consequences
B. Help both see the conflict as grief and strain from long separation
C. Refer the teen for evaluation of oppositional defiant disorder

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Clear rules matter, and structure can help once the relationship is stronger. Leading with firmer consequences treats the conflict as simple misbehavior and can deepen the teen's sense of loss and resentment.

B **Correct.** Many immigrant youth go through long separations from parents during migration, and reunification often brings grief for the caregiver left behind, distance from a parent who feels like a stranger, and conflict over authority (Suarez-Orozco et al., 2011). The teen's crying for the grandparent, withdrawal and refusal of rules 4 months after arrival fit this pattern. Helping parent and teen understand it and rebuild their bond addresses the cause. It beats A because firmer rules may intensify the power struggle while the teen still grieves and does not yet trust the parent's authority.

C **Incorrect** (trap: Skips assessment). Defiance that began with a major family transition and comes with grief points first to adjustment and loss. A diagnostic referral now risks labeling an expected reaction as a disorder.

Takeaway: After long migration-related separations, reunified families often face grief and authority conflict; address the separation, not just the behavior.

Sources: Suarez-Orozco, C., Bang, H. J., & Kim, H. Y. (2011). I felt like my heart was staying behind: Psychological implications of family separations and reunifications for immigrant youth. Journal of Adolescent Research, 26(2), 222-257, whole article
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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32. INT-1120 (rev 1) · Best answer: A

A client has gone to the emergency department three times for a racing heart and chest tightness. Cardiac testing was normal each time, and the physician diagnosed panic disorder and referred the client to the social worker. The client asks, 'If my heart is fine, why does my body do this?' What should the social worker do **FIRST**?

A. Explain how the body's alarm response produces these sensations
B. Begin exercises that bring on a racing heart in session
C. Ask the physician to order further heart tests for reassurance

Why each option is right or wrong

A **Correct.** Cognitive-behavioral treatment for panic disorder begins with psychoeducation about the nature of anxiety and panic, including how the body's fight-or-flight response creates sensations like a racing heart (Craske & Barlow, 2022). The client's question invites exactly this, and understanding the cycle of sensation and fear reduces catastrophic thinking. It also lays the groundwork for later steps. It beats B because exercises that deliberately bring on body sensations are an effective later step, but starting them before the client understands the alarm response can feel frightening and confusing.

B **Incorrect** (trap: Right action, wrong time). Interoceptive exposure, which brings on feared body sensations on purpose, is a core part of panic treatment. It comes after psychoeducation, when the client understands why the sensations are harmless and agrees to the rationale.

C **Incorrect** (trap: Hands off too soon). Medical causes have been ruled out three times. More testing can reinforce the belief that something is physically wrong and feed reassurance seeking. New or different symptoms would justify another medical look.

Takeaway: In CBT for panic, start with psychoeducation on the body's alarm response before exposure to feared sensations.

Sources: Craske, M. G., & Barlow, D. H. (2022). Mastery of Your Anxiety and Panic: Therapist Guide (5th ed.). Oxford University Press, psychoeducation on the nature of anxiety and panic

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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33. ETH-1155 (rev 1) · Best answer: C

A program director at a family counseling agency tells staff to keep notes for clients in custody disputes 'as thin as possible,' recording only the date and 'attended,' so the notes cannot hurt clients if subpoenaed. A social worker sees several of these clients, including one who recently self-harmed. Which documentation approach is **MOST** appropriate?

A. Follow the director's instruction for these clients

B. Keep thin notes but document risk events in full

C. Write sufficient notes limited to service-relevant facts

D. Keep detailed notes in a separate personal file

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Thin notes may seem protective, but a record that shows only attendance cannot support continuity, risk management or billing. Agency directives should not override ethical practice (3.09(d)); the better course is sufficient notes plus raising the concern with the director.

B **Incorrect** (trap: Good but not best). Recording risk events fully is right, but leaving every other session as 'attended' still fails the sufficiency standard. Progress, interventions and plans also need documenting so another provider could continue care.

C **Correct.** Records must be accurate and sufficient enough to support current care and continuity (NASW Code 3.04(a), 3.04(b)), while including only information relevant to services (3.04(c)). That balance answers both concerns: the client who self-harmed

needs assessment and safety planning documented, yet private details unrelated to treatment can stay out. It beats B because sufficiency applies to every session, not only crises. Subpoenaed records are protected by asserting privilege, not by documenting less than care requires, and a director's order cannot override this (3.09(d)).

D **Incorrect** (trap: Misuses the technique). Separately kept process notes may hold a clinician's impressions, but they cannot replace the clinical record. Moving required content, such as risk assessments, into a private file leaves the official chart incomplete and inaccurate (3.04(a)).

Takeaway: Litigation risk does not justify thin records; document what care requires and leave out what is irrelevant to services.

Sources: NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 3.04(c); NASW Code of Ethics (2021), 3.09(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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34. ETH-1092 (rev 3) · Best answer: B

A social worker staffs a tenant association that is developing a cooperative housing building. At a meeting, members vote to bar anyone with a criminal record from applying. Several members have relatives returning from prison who had hoped to apply. What is the **BEST** response by the social worker?

A. Help members write the vote into the building's rules
B. Raise how the rule excludes people and discuss alternatives
C. Tell members the ban is unjust and ask them to repeal it

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Respecting the group's decision-making is important, but helping put an exclusionary rule into effect without raising its impact sets aside the duty to promote justice and access.

B **Correct.** Group self-determination matters, but social justice ranks above it in the Ethical Principles Screen (Dolgoff et al., 2012), and social workers should work to expand choice and opportunity for all people, especially those who are disadvantaged (NASW Code 6.04(b)). A blanket ban shuts out people, including members' own relatives, regardless of offense or time since release. Naming that effect and exploring alternatives, such as individual review, informs the group's choice without taking it over. B beats C because naming the injustice is right, but pressing for repeal takes the decision from the group instead of informing it.

C **Incorrect** (trap: Too directive). The concern is valid, which makes a firm stand tempting. Pressing members to repeal the ban replaces the group's decision with the social worker's, and a lecture is less likely to change minds than laying out the rule's effects and workable alternatives.

Takeaway: When a client group's choice would exclude vulnerable people, raise its justice impact and alternatives while leaving the decision with the group.

Sources: NASW Code of Ethics (2021), 6.04(b); Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen: social justice; self-determination
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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35. INT-1606 (rev 2) · Best answer: B

A newly hired associate social worker at a community mental health center is drafting goals for the supervision contract. The associate is skilled in individual cognitive behavioral work but has little experience with family sessions, which make up a third of the assigned caseload. The supervisor will also complete the associate's licensure evaluations. Which approach to the goals is **BEST**?

A. Let the supervisor set goals from the first licensure evaluation
B. Make family session work a priority learning goal
C. Ask that the family cases go to more experienced colleagues

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Evaluation results can sharpen goals later, and the supervisor's view matters. Waiting for the first formal evaluation, though, leaves months of family sessions without targeted supervision when the gap is already known; supervisees help set their own learning goals from the start.

B **Correct.** Educational supervision is individualized to the supervisee's learning needs (Kadushin & Harkness, 2014), and the supervision contract should set written goals with responsibilities and time frames (NASW & ASWB supervision standards, 2013). The associate already knows the gap, and family sessions are a third of the caseload. Naming it now directs supervision where clients need it, even though the same supervisor will evaluate the associate. It beats A because the need is already clear, and delay leaves family clients served without focused support.

C **Incorrect** (trap: Hands off too soon). Social workers may take on work they intend to become competent in (NASW Code 4.01(a)). Asking to shed a third of the caseload avoids the learning that supervision is meant to support.

Takeaway: Supervisees self-assess and put known skill gaps into the supervision contract as priority learning goals, even when the supervisor also evaluates them.

Sources: Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 4: Educational Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 2 (Conduct of Supervision): Contracting for Supervision; NASW Code of Ethics (2021), 4.01(a)
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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36. ETH-1626 (rev 2) · Best answer: C

A community mental health agency receives 15 tablets with a year of free data service to lend to clients for video sessions. About 60 clients ask for one. Staff propose a first-come, first-served sign-up form on the agency website. The social worker on the planning group knows that many clients who lack internet at home have been missing sessions. Which way of allocating the tablets is **MOST** fair?

A. Offer them through the website form in sign-up order
B. Hold a random drawing among the clients who ask
C. Give priority to clients without a device or data plan
D. Let each worker choose the clients who seem most deserving

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). First come, first served sounds neutral, but a website form favors clients who are already online, which defeats the program's purpose. Sign-up order can be fair when every applicant has the same means to apply.

B **Incorrect** (trap: Good but not best). A lottery is transparent and gives each applicant an equal chance, so it is fairer than a race to sign up. Equal chances still ignore need, so tablets could go to clients who already attend video sessions without trouble.

C **Correct.** Distributive justice asks that scarce goods go first to those with the greatest need; Rawls's difference principle holds that inequalities are justified only when they benefit the least advantaged (Rawls, 1971). The tablets exist to close a digital gap, and clients without devices or data are the ones missing care. It beats B because a lottery treats applicants equally but would hand some tablets to clients who already have access. The Code also directs attention to differences in clients' access to electronic services (NASW Code 1.05(e)).

D **Incorrect** (trap: Misreads law or ethics). Personal nominations invite bias and inconsistency, and clients cannot see how decisions are made. Worker input is useful for confirming need, but it should apply shared, stated criteria rather than individual judgments about who deserves help.

Takeaway: When allocating scarce resources, use clear criteria that give priority to those with the greatest need, not first access or equal chance.

Sources: Rawls, J. (1971). A Theory of Justice. Harvard University Press, difference principle; NASW Code of Ethics (2021), 1.05(e)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 1 of 3

37. INT-1047 (rev 2) · Best answer: D

PRETEST — unscored on this form; do not count it

On a home visit, a single parent in treatment for depression reports returning to work and feeling better. During the session, the 12-year-old answers questions put to the parent and tells the younger siblings when to sit down. The parent looks to the 12-year-old before speaking. Using a structural approach, which response is **BEST**?

A. Thank the 12-year-old for helping and let them keep guiding the siblings
B. Arrange individual sessions to address the 12-year-old's stress
C. Pause family sessions until the parent's depression fully remits
D. Ask the parent to direct the siblings while the 12-year-old rests

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Praising the 12-year-old for guiding the siblings reinforces the parentified role and the blurred hierarchy. In structural terms, the social worker would be joining the family's problem pattern rather than changing it.

B **Incorrect** (trap: Good but not best). Individual support for the 12-year-old may help later, but it treats a structural problem as the child's issue. The pattern lives in how the family is organized, which is best changed in family sessions.

C **Incorrect** (trap: Right action, wrong time). Waiting for full remission delays help the family needs now. The parent is working and feeling better, which suggests they can begin taking back the parental role with support.

D **Correct.** Structural family therapy sees parentification as a hierarchy problem: a child has taken over the parent's executive role. Boundary making in the session puts the parent back in charge and frees the child to be a sibling (Minuchin, 1974). The parent is in treatment, back at work, and feeling better, so they can take on that role now. It beats B because the 12-year-old's stress is real, but individual sessions leave the family structure that produces it unchanged; restructuring happens with the whole family present.

Takeaway: With a parentified child, structural work restores hierarchy: support the parent in the executive role and release the child from it.

Sources: Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, subsystems, hierarchy, and boundary making

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

38. ASM-1601 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A social worker at a community brain injury program receives a neuropsychological evaluation for a 34-year-old client injured in a fall 8 months ago. The client has signed releases for the program and the evaluator. After reading the report, the social worker understands its summary but not how the processing-speed and memory scores should shape the client's job-training plan. What should the social worker do **NEXT**?

A. Ask the psychologist who did the testing about practical implications
B. Explain to the client what each test score means for work
C. Revise the job-training plan based on the report's summary
D. Request a second neuropsychological evaluation from another provider

Why each option is right or wrong

A **Correct.** Integrating an outside evaluation means applying its findings accurately. Social workers seek colleagues' advice when it serves clients and practice within their competence (NASW Code 2.05(a), 1.04(a)). The social worker grasps the summary but not how specific scores translate into job demands, and the evaluating psychologist can explain that, with releases already signed. It beats C because revising the plan on a partial reading could set training goals that are too hard or too easy; the plan should change once the implications are clear.

B **Incorrect** (trap: Outside the social work role). Interpreting individual neuropsychological scores takes testing expertise that most social workers lack, and a wrong explanation could mislead the client about returning to work. The social worker can share the report's plain-language conclusions and arrange for the evaluator to answer score questions.

C **Incorrect** (trap: Right action, wrong time). Updating the job-training plan is the right later step, once the findings' practical meaning is clear. Changing it now, from a summary the social worker only partly understands, risks goals that do not fit the client's abilities.

D **Incorrect** (trap: Skips assessment). A repeat evaluation duplicates recent testing and costs the client time without any reason to doubt the first report. It fits when a report is outdated or its findings are in serious dispute; here the gap is in understanding, not in the testing.

Takeaway: When an outside evaluation's implications are unclear, ask its author before applying them; don't interpret specialized test scores beyond your training.

Sources: NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.04(a)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.11 Integrating records and evaluations (medical, school, psychological, employment) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

39. ASM-1514 (rev 1) · Best answer: A

At a family service agency, the caregiver of a 6-year-old reports that the child's parent left on a military deployment 3 weeks ago. Since then, the child, who was fully toilet trained by age 3, has begun wetting the bed, asks to be fed with a spoon and talks in a baby voice. Which defense mechanism is the child **MOST** likely using?

A. Regression
B. Fixation
C. Displacement

Why each option is right or wrong

A **Correct.** Regression is a return to behavior from an earlier developmental level under stress, after the person had already moved beyond it (Freud, A., 1966). This child was toilet trained by age 3 and began bedwetting, spoon-feeding requests and a baby voice within weeks of a parent's deployment, a stress-linked return to earlier behavior. It beats B because fixation means development stalled at a stage and the behavior never stopped, while this child had clearly progressed and then went back. A medical check for new bedwetting is still sensible.

B **Incorrect** (trap: Wrong model or concept). Fixation describes getting stuck at an earlier stage so that its behaviors persist without a break. This child had mastered toileting years earlier and lost the skill under stress, which is a return rather than a stall.

C **Incorrect** (trap: Wrong model or concept). Displacement shifts feelings from their true target to a safer one, such as anger at the absent parent aimed at a sibling. The child's behavior is a return to earlier habits, not redirected feelings.

Takeaway: Regression is a stress-driven return to earlier behavior after progress; fixation is development that stalled and never moved on.

Sources: Freud, A. (1966). The Ego and the Mechanisms of Defense (rev. ed.). International Universities Press, regression

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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40. ETH-1125 (rev 2) · Best answer: B

An outpatient social worker has seen an adult client for depression for 2 months. The client's primary care physician calls before a routine visit next week and asks for a summary of therapy progress. The client has not signed a release for this physician and has a session with the social worker in 2 days. What is the **BEST** response to the physician?

A. Share a brief progress summary, since this is for treatment
B. Say that consent is needed before sharing, then follow up
C. Confirm the client is in treatment but share no clinical details
D. Ask the physician to send a written records request

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). HIPAA does permit treatment disclosures without authorization, and coordinating with the physician has real value. But the Code expects consent when it can readily be obtained, and nothing here is urgent. Sharing now would fit an emergency or a client who had already agreed to coordination.

B **Correct.** Confidential information is shared with valid consent unless there is a compelling professional reason (NASW Code 1.07(b), 1.07(c)). HIPAA permits a provider to disclose information for another provider's treatment (45 CFR 164.506(c)(2)), but a permission is not a requirement, and the Code sets the higher bar. Nothing here is urgent: the visit is next week and the client will be seen in 2 days, so asking first costs little and respects the client's control. B beats A because A treats a HIPAA permission as if it settled the ethical question when the client can easily be asked.

C **Incorrect** (trap: Misreads law or ethics). Confirming that the person is in treatment is itself a disclosure, made here without the client's agreement. A neutral reply that sharing needs consent avoids confirming anything, and the client can be asked in 2 days.

D **Incorrect** (trap: Good but not best). A written request adds paperwork but does not solve the core problem: the client has not agreed. Whatever form the request takes, the social worker still needs the client's consent before releasing a summary.

Takeaway: HIPAA permission to share for treatment is not a duty to share; when time allows, get the client's consent first.

Sources: NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.506(c)(2)

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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41. ASM-1546 (rev 2) · Best answer: B

A 52-year-old patient is readmitted to the hospital with high blood sugar for the third time in 4 months. A team note calls the patient 'noncompliant with insulin.' The patient works two part-time jobs without health benefits and takes two buses to reach the nearest pharmacy. What should the social worker do **FIRST**?

A. Teach the patient about complications of skipping insulin

B. Ask the patient what makes getting and using insulin difficult

C. Refer the patient to a diabetes self-management education class

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Explaining complications assumes the patient does not understand the risks and can feel like a lecture. A patient readmitted three times likely knows the danger; without learning the barriers, more information is unlikely to change anything.

B **Correct.** Social determinants of health, including economic stability and access to health care, shape outcomes (ODPHP, Healthy People 2030). Labeling a patient noncompliant can hide barriers such as cost, lack of coverage, transportation and work schedules, all hinted at here. Asking the patient directly what gets in the way identifies the actual barrier before choosing a response. It beats C because diabetes education assumes a knowledge gap; if the problem is paying for insulin or reaching the pharmacy, classes will not prevent the next readmission.

C **Incorrect** (trap: Right action, wrong time). Self-management education is valuable when knowledge or skills are missing. Referring before asking about barriers may send

a patient who cannot afford insulin or reach the pharmacy to a class that addresses neither problem.

Takeaway: Before labeling a patient nonadherent, assess social determinants such as cost, coverage, transportation and work demands that block treatment.

Sources: Office of Disease Prevention and Health Promotion (ODPHP) (2020). Healthy People 2030: Social Determinants of Health, SDOH domains: economic stability; health care access and quality
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.9 Poverty's impact; social determinants of health · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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42. ASM-0009 (rev 1) · Best answer: A

An adult client with bipolar disorder takes lithium. At a home visit, the client reports 2 days of vomiting and diarrhea from a stomach virus. The social worker notices a coarse hand tremor and an unsteady walk that were absent last week. What should the social worker do **FIRST**?

A. Help the client get emergency medical care today
B. Advise the client to skip lithium until the illness passes
C. Recheck the client's symptoms at next week's home visit

Why each option is right or wrong

A **Correct.** Vomiting and diarrhea can dehydrate a person taking lithium and raise the blood level. A new coarse tremor and unsteady gait are warning signs of lithium toxicity, which can progress to confusion, seizures and death (Gitlin, 2016). Suspected toxicity needs same-day medical evaluation, including a blood level. It beats B because the social worker does not direct medication changes. Holding a dose is the prescriber's decision, and it would not treat the toxicity that may already be present.

B **Incorrect** (trap: Outside the social work role). Telling a client to hold a medication is a prescribing decision outside the social work role. It also overlooks that the current symptoms already suggest a dangerous lithium level needing urgent testing.

C **Incorrect** (trap: Ignores risk). Waiting a week ignores signs of possible toxicity, which can worsen quickly. Monitoring suits stable side effects, not new neurological symptoms during dehydration.

Takeaway: Dehydration plus new coarse tremor or unsteadiness in a client on lithium suggests toxicity; get emergency medical care the same day.

Sources: Gitlin, M. (2016). Lithium side effects and toxicity: prevalence and management strategies. International Journal of Bipolar Disorders, 4, 27, toxicity: signs and risk factors
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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43. ASM-1658 (rev 1) · Best answer: A

During a biopsychosocial intake at a primary care clinic, a 47-year-old client reports 2 weeks of low mood, irritability, tearfulness and sleeping about 3 hours a night. The medical history shows the client began high-dose prednisone for a lupus flare 3 weeks ago. The client has no past mood episodes and wants help 'getting back to normal.' What should the social worker do **FIRST**?

A. Ask consent to tell the prescriber about the mood changes
B. Begin weekly CBT sessions targeting the low mood and sleep
C. Use a PHQ-9 to confirm and record a depressive disorder

Why each option is right or wrong

A **Correct.** The biological part of a biopsychosocial assessment includes medications, because some drugs cause psychiatric symptoms. Corticosteroids such as prednisone commonly produce mood changes, irritability and insomnia, especially at high doses (Kaplan & Sadock). Symptoms that began a week after high-dose prednisone started, with no prior mood history, point to a possible medication effect that only the prescriber can weigh. Sharing the timing, with consent, lets the prescriber decide on any change. It beats B because starting therapy first treats a possible drug effect as a primary mood disorder; DSM-5-TR separates medication-induced depressive symptoms from independent episodes.

B **Incorrect** (trap: Right action, wrong time). Support for low mood and sleep may help, and therapy can run alongside medical care. Starting it first assumes a primary mood disorder before the prescriber has weighed a likely medication effect.

C **Incorrect** (trap: Skips assessment). A PHQ-9 can track symptoms, but a screener does not diagnose, and the timing suggests the symptoms may be medication-induced. Recording a depressive disorder now could mislabel a drug effect.

Takeaway: When symptoms begin soon after a new medication starts, share the timing with the prescriber before treating them as a primary psychiatric disorder.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), medication-induced psychiatric symptoms: corticosteroids; DSM-5-TR (APA, 2022), Depressive Disorders: Substance/Medication-Induced Depressive Disorder
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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44. ASM-1654 (rev 1) · Best answer: D

A 34-year-old client with schizophrenia grew up in a middle-income family and was in college when symptoms began at age 20. Since then, the client has lost a series of jobs and now lives on disability benefits in low-cost housing. A new staff member remarks that the client's poverty probably caused the illness. Which account of the link between low income and mental illness does this history **MOST** likely support?

A. Social causation
B. Culture of poverty
C. Intergenerational transmission
D. Social selection

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Social causation holds that the stresses of poverty raise the risk of disorder. It fits when hardship comes first, as research suggests for some depression. Here the client's income fell after the illness began, so the history runs the other way.

B **Incorrect** (trap: Wrong model or concept). The culture of poverty is a contested idea describing values said to keep families poor across generations. It does not explain a person moving from middle income into poverty after a psychiatric illness.

C **Incorrect** (trap: Wrong model or concept). Intergenerational transmission describes poverty passed from parents to children through limited resources and opportunity. This client did not grow up poor, so the concept does not fit the history given.

D **Correct.** The social selection, or drift, explanation holds that a disorder leads to lower socioeconomic status as symptoms disrupt school and work. This client started out middle-income and became poor only after onset, the order selection predicts. Dohrenwend and colleagues found that selection appears more important for schizophrenia, while causation appears more important for depression in women and for antisocial personality and substance use disorders in men (Dohrenwend et al., 1992). It beats A because social causation requires poverty and its stresses to come before the illness, which this history does not show.

Takeaway: Social selection (drift): illness lowers status. Social causation: poverty's stress raises illness risk. Evidence for schizophrenia leans toward selection.

Sources: Dohrenwend, B. P., Levav, I., Shrout, P. E., Schwartz, S., Naveh, G., Link, B. G., Skodol, A. E., & Stueve, A. (1992). Socioeconomic status and psychiatric disorders: The causation-selection issue. Science, 255(5047), 946-952, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.9 Poverty's impact; social determinants of health · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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45. ETH-1071 (rev 2) · Best answer: A

A clinic social worker spends 30 minutes on the phone helping a client complete a housing application. The client's health plan covers psychotherapy but not case management. A colleague notes that the call could be billed as psychotherapy because the client seemed calmer afterward. What is the **BEST** way for the social worker to handle the call?

A. Record it as case management with no therapy claim

B. Bill it as therapy rather than case management

C. Skip documenting the call because the plan won't pay

Why each option is right or wrong

A **Correct.** Billing must accurately reflect the nature and extent of the services provided (NASW Code 3.05). Helping with a housing application is case management, even if the client felt calmer, so labeling it psychotherapy to obtain payment would misrepresent the service (4.04). The call still belongs in the record, because documentation supports continuity of care (3.04(b)). A beats C because the payment question does not change the duty to document a service that took place.

B **Incorrect** (trap: Misreads law or ethics). A calming effect does not turn paperwork help into psychotherapy. Coding a service as something it was not, to fit coverage, is billing fraud (3.05, 4.04).

C **Incorrect** (trap: Misreads law or ethics). Whether a payer covers a service has no bearing on whether it is documented. The call is part of the client's care and needs a timely note (3.04(b)).

Takeaway: Bill services as what they actually were; coverage gaps never justify relabeling a service, and unbilled services are still documented.

Sources: NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.04(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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46. ETH-1095 (rev 2) · Best answer: B

A new client in private practice says that a social worker at another agency, who treated the client last year, borrowed $2,000 during treatment and never repaid it. The client feels embarrassed and asks whether that was allowed. What is the **BEST** response?

A. File a complaint about the former provider with the board
B. Say it was improper and describe complaint options
C. Call the former provider to ask about the loan
D. Suggest setting it aside to focus on current goals

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Exposing unethical conduct matters, but this information came from the client in confidence. A complaint should be filed by the client or with the client's consent, which has not been given.

B **Correct.** Borrowing from a client exploits the professional relationship (NASW Code 1.06(b)), so answering honestly is not unwarranted criticism of a colleague. Social workers act to discourage and correct unethical conduct (2.10(a)), but this account is the client's confidential information. Explaining complaint options and supporting the client's choice addresses the misconduct while leaving the decision with the client. B beats A because filing without the client's consent would disclose confidential information the client has not authorized (1.07(b)).

C **Incorrect** (trap: Misreads law or ethics). Contacting the former provider reveals the client's disclosure without consent and places the social worker in the middle of the client's dispute. It also cannot resolve the ethical issue.

D **Incorrect** (trap: Good but not best). Current goals matter, but the client asked a direct question about possible exploitation. Deflecting leaves the client without accurate information about their rights and options.

Takeaway: When a client reports a prior provider's misconduct, answer honestly and explain complaint options; act on it only with consent.

Sources: NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), 2.10(a); NASW Code of Ethics (2021), 1.07(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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47. INT-0105 (rev 1) · Best answer: B

A client changes the subject each time a sibling's death comes up. The social worker points out the pattern and suggests that avoiding the topic may protect the client from guilt carried since childhood. Which treatment approach is the social worker **MOST** likely using?

A. Cognitive behavioral therapy
B. Psychodynamic therapy
C. Eye movement desensitization and reprocessing

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). CBT might target guilt by examining the thoughts behind it, such as self-blame, and weighing the evidence. It focuses on present thinking, not on interpreting in-session avoidance as a defense with childhood roots.

B **Correct.** Psychodynamic therapy explores how clients avoid distressing feelings and links present patterns to earlier experience (Shedler, 2010). Pointing out the avoidance and suggesting it guards against long-held guilt is an interpretation of a defense, a core psychodynamic technique aimed at insight. It beats A because cognitive behavioral therapy would identify the guilt-related thoughts and test their accuracy in the present, rather than interpret avoidance in session as a defense rooted in childhood.

C **Incorrect** (trap: Wrong model or concept). EMDR processes distressing memories while the client attends to bilateral stimulation, such as guided eye movements. Nothing in this stem describes that kind of structured memory processing.

Takeaway: Interpreting a client's avoidance as a defense linked to past experience is a psychodynamic technique aimed at insight.

Sources: Shedler, J. (2010). The efficacy of psychodynamic psychotherapy. American Psychologist, 65(2), 98-109, distinctive features of psychodynamic technique
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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48. ETH-1669 (rev 2) · Best answer: C

A community agency will close its counseling program in 8 weeks. The director tells staff to wait until the final week to inform clients, so they do not leave early. A social worker meets with the director and explains that clients need time to plan, but the director refuses to change course. The agency's ethics committee meets in 2 weeks. What should the social worker do **NEXT**?

A. Tell clients now rather than wait for the committee
B. Follow the director's timeline and plan referrals quietly
C. Raise the director's timeline with the committee

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Prompt notice is the goal, and if the agency's channels fail in time, the worker may need to inform clients directly. Going around the director now skips a review that could change the policy for all clients in the program.

B **Incorrect** (trap: Misreads law or ethics). Planning referrals helps, but waiting until the final week leaves clients little time to process the ending or arrange care. Following an order that blocks prompt notice puts agency interests ahead of clients' needs.

C **Correct.** Clients facing the end of services should be told promptly so transfers can be arranged (NASW Code 1.17(e)). Social workers should not let an employer's orders interfere with ethical practice and should take reasonable steps to bring agency practice in line with the Code (3.09(d)). The worker has already raised the issue with the director; the ethics committee is the next channel, and a meeting in 2 weeks still leaves about 6 weeks of notice if the policy changes. It beats A because acting alone against a direct order skips an available step that could fix the policy for every client.

Takeaway: When an employer blocks prompt notice of ending services, use the next internal channel before acting alone, keeping clients' needs central.

Sources: NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 3.09(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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49. ETH-1003 (rev 1) · Best answer: C

A social worker at a family services agency receives a court order signed by a judge in a child dependency case. It directs release of a parent's complete treatment record. The record includes the parent's detailed account of childhood sexual abuse, which has no bearing on the current case. What is the **BEST** response?

A. Send the complete record by the court's stated deadline
B. Decline to follow the order to protect the parent
C. Ask the court to limit the order or seal the record
D. Send a brief summary in place of the full record

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Complying is lawful, but sending everything without first asking the court for protection exposes sensitive history unrelated to the case. If the court declines to narrow the order, full compliance is then correct.

B **Incorrect** (trap: Misreads law or ethics). A judge's order is legally binding. Refusing to comply risks contempt and does little to protect the parent; the lawful path is to ask the court itself to limit or seal the disclosure.

C **Correct.** A court order, unlike an attorney's subpoena, carries the court's authority and must be followed. Still, the Code directs social workers to protect confidentiality as far as the law permits: when ordered disclosure could harm the client, the worker asks the court to withdraw or narrow the order or keep the records under seal (NASW Code 1.07(j)). HIPAA likewise limits disclosure to what the order expressly authorizes (45 CFR 164.512(e)(1)(i)). C beats A because full compliance is lawful but exposes unrelated trauma history without first seeking protection.

D **Incorrect** (trap: Misreads law or ethics). Substituting a summary changes what the court ordered without its permission. The court, not the social worker, decides what is produced; a summary is acceptable only if the court agrees to modify the order.

Takeaway: Obey a court order, but when disclosure could harm the client, ask the court to narrow the order or seal the records.

Sources: NASW Code of Ethics (2021), 1.07(j); HIPAA Privacy Rule, 45 CFR 164.512(e)(1)(i)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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50. INT-1100 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A client seeks help because their partner drinks heavily most nights and refuses to discuss treatment. The client feels safe at home and wants the partner to get help. A relative suggests gathering the family for a surprise confrontation. What is the **BEST** approach for the social worker to offer?

A. Help the client plan a family confrontation with clear consequences
B. Coach the client to reward sober behavior and ask at calm moments
C. Refer the client to Al-Anon to focus on their own coping

Why each option is right or wrong

A **Incorrect** (trap: Too directive). A surprise confrontation can feel decisive, but many families never follow through with one, and it engaged far fewer drinkers than CRAFT in head-to-head research. Confrontation also risks anger and withdrawal, so it is a weaker route to the client's goal.

B **Correct.** Community Reinforcement and Family Training (CRAFT) teaches a concerned family member to reward sober behavior, let the natural consequences of drinking occur, communicate positively, and suggest treatment when the drinker is most receptive. The client wants the partner in treatment and feels safe at home, which CRAFT requires. In a randomized trial, CRAFT engaged about two-thirds of drinkers in treatment, far more than a confrontational intervention or Al-Anon facilitation (Miller, Meyers & Tonigan, 1999). It beats C because Al-Anon supports the client's own coping but rarely moves the drinker into care, which is the client's stated aim.

C **Incorrect** (trap: Good but not best). Al-Anon offers real support, and family members' well-being improved in research, but it is not designed to bring the drinker into treatment. It fits a client whose main goal is their own coping rather than the partner's engagement.

Takeaway: To help a family member engage a treatment-refusing drinker, CRAFT's positive reinforcement outperforms confrontation and Al-Anon facilitation.

Sources: Miller, W. R., Meyers, R. J., & Tonigan, J. S. (1999). Engaging the unmotivated in treatment for alcohol problems: A comparison of three strategies for intervention through family members. Journal of Consulting and Clinical Psychology, 67(5), 688-697, whole article
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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51. INT-1594 (rev 1) · Best answer: D

A social worker doing the quarterly internal chart audit at a counseling agency reviews 10 sampled charts for one colleague. Nine document sessions on dates when the electronic check-in system shows the client never arrived, and all nine were billed. Agency policy names a compliance officer to receive billing concerns. What should the social worker do **NEXT**?

A. Ask the colleague to explain the mismatched dates
B. Expand the review before reporting to the compliance officer
C. Correct the billing for the nine sessions
D. Report the findings to the compliance officer

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Talking first with a colleague fits when that is likely to be productive and no formal process applies (NASW Code 2.10(c)). Here the social worker is acting as auditor, the evidence shows a pattern, and a private talk could prompt changes to records before review.

B **Incorrect** (trap: Right action, wrong time). A wider review may well be needed, but the compliance officer should set its scope and method. Expanding it first, on the auditor's own initiative, delays reporting a strong pattern the audit has already found.

C **Incorrect** (trap: Outside the social work role). Correcting claims is a billing and compliance task that follows investigation. Changing them now, before the compliance officer reviews the evidence, could alter the record.

D **Correct.** Billing must reflect services actually provided, and social workers may not take part in or condone fraud (NASW Code 3.05, 4.04). An auditor's findings go through the agency's established channel, which here is the compliance officer (NASW Code 2.10(b); HHS-OIG, 2023). Nine of 10 billed sessions contradicted by check-in data form a pattern, not a stray error. This beats A because informal resolution suits ordinary collegial concerns, while the audit role and policy route billing concerns to compliance.

Takeaway: Audit findings that suggest billing for services not provided go promptly through the designated compliance channel, not informal resolution.

Sources: NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 2.10(b); NASW Code of Ethics (2021), 2.10(c); HHS Office of Inspector General (2023). General Compliance Program Guidance, compliance officer; reporting and responding to potential noncompliance
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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52. ASM-0117 (rev 1) · Best answer: A

A 4-year-old is referred for frequent tantrums and hitting at home and preschool. The caregiver, who benefited from cognitive behavioral therapy for anxiety, asks whether the preschooler could receive the same kind of individual talk therapy. Which modality is **MOST** appropriate for this preschooler?

A. Parent-child interaction therapy
B. Individual cognitive therapy with thought records
C. Weekly social skills group with peers

Why each option is right or wrong

A **Correct.** Modality should match the client's developmental level. A 4-year-old is in Piaget's preoperational stage and lacks the abstract reasoning needed for cognitive restructuring, and disruptive behavior at this age responds best to caregiver-involved behavioral treatment such as parent-child interaction therapy (Kaminski & Claussen, 2017). It beats B because thought records require reading, writing and the ability to examine one's own thoughts, which a preschooler has not developed. The caregiver's success with CBT does not make it fit the child.

B **Incorrect** (trap: Wrong model or concept). Cognitive work with thought records suits older children, adolescents and adults who can reflect on their thinking. A preschooler cannot yet do this kind of abstract, written work, so the approach is mismatched to development.

C **Incorrect** (trap: Good but not best). A social skills group may help some young children, but it leaves out the caregiver, who is central to changing disruptive behavior at this age. Caregiver-involved treatment has the strongest evidence for preschool tantrums and aggression.

Takeaway: Match modality to developmental level: young children with disruptive behavior do best with caregiver-involved behavioral treatment, not cognitive restructuring.

Sources: Kaminski, J. W., & Claussen, A. H. (2017). Evidence base update for psychosocial treatments for disruptive behaviors in children. Journal of Clinical Child & Adolescent Psychology, 46(4), 477-499, whole article; Piaget, J., & Inhelder, B. (1969). The Psychology of the Child. Basic Books, preoperational stage

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.3 Choosing intervention modality by ability, development, culture, life stage · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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53. ETH-1046 (rev 1) · Best answer: B

A social worker makes an honest report of suspected child abuse that is later not confirmed. What legal protection generally applies?

A. Psychotherapist-patient privilege
B. Statutory immunity
C. Sovereign immunity

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Psychotherapist-patient privilege protects confidential communications from compelled disclosure in court. It belongs to the client and limits disclosure; it does not protect a reporter from being sued over a report.

B **Correct.** CAPTA requires states, as a condition of funding, to provide immunity from civil and criminal liability for people who make good-faith reports of suspected child abuse or neglect (42 U.S.C. 5106a(b)(2)(B)(vii)). State reporting laws therefore protect an honest reporter even when the report is not substantiated. B beats C because sovereign immunity protects governments, not individual reporters, and does not turn on good faith.

C **Incorrect** (trap: Wrong model or concept). Sovereign immunity shields governments, and sometimes their agencies, from certain lawsuits. It does not depend on good faith,

and it is not the protection reporting laws give individual reporters, including those in private practice.

Takeaway: Reporting laws give reporters immunity for good-faith reports, even when the report is not substantiated.

Sources: Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(vii)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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54. ASM-1068 (rev 2) · Best answer: B

A school social worker meets with the parent of a 15-year-old who has taken fluoxetine for 3 months. The teen's mood and attendance have improved, and the teen reports no suicidal thoughts at regular check-ins. The parent just read about the antidepressant warning for youth and plans to stop the medicine tonight. What is the social worker's **BEST** response?

A. Support the parent's decision to stop the medicine tonight
B. Encourage the parent to talk with the prescriber before stopping
C. Assure the parent that the risk is limited to the first weeks

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Parents decide about a minor's treatment, and this concern is valid. Endorsing an abrupt stop without the prescriber, though, risks relapse in a teen who is improving and bypasses medical guidance.

B **Correct.** Antidepressants carry an FDA boxed warning about increased suicidal thinking in children, adolescents and young adults, which calls for close monitoring, especially early in treatment and after dose changes, rather than automatic discontinuation (Hammad et al., 2006; Kaplan & Sadock). Stopping a medicine that is working risks relapse, and medication changes belong with the prescriber. Encouraging a prompt talk with the prescriber respects the parent's authority while keeping the decision informed. It beats C because reassurance about medication risk is medical advice outside the social work role.

C **Incorrect** (trap: Outside the social work role). Risk is highest early in treatment and after dose changes, but offering medical reassurance oversteps the social work role. The prescriber should answer the parent's question about this teen's risk.

Takeaway: A youth antidepressant boxed warning calls for close monitoring; families bring stopping decisions to the prescriber first.

Sources: Hammad, T. A., Laughren, T., & Racoosin, J. (2006). Suicidality in pediatric patients treated with antidepressant drugs. Archives of General Psychiatry, 63(3), 332-339, whole article; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Antidepressants: boxed warning on suicidality in youth
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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55. ASM-1153 (rev 1) · Best answer: C

A client with schizophrenia who lives in supportive housing smokes two packs of cigarettes a day. The client tells the social worker they want to quit because cigarettes use up most of their spending money. A staff member says quitting will worsen the client's symptoms and that smoking is the client's only pleasure. The client's symptoms have been stable for a year. What is the social worker's **BEST** response?

A. Suggest waiting until the client's stability is more established
B. Encourage the client to cut down gradually on their own
C. Help the client get cessation treatment through the prescriber

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Waiting reflects a common belief that quitting destabilizes people with schizophrenia. Evidence does not support routine delay, and this client has been stable for a year and wants to act now. Delay keeps money and health going to cigarettes.

B **Incorrect** (trap: Good but not best). Supporting a gradual cutback respects the client's goal, but unaided attempts succeed far less often than treatment. Medication plus counseling works better than either alone, and the prescriber needs to be involved because smoking affects levels of some antipsychotics.

C **Correct.** Smoking is far more common among people with serious mental illness, and it is a major cause of their early deaths. In a large trial, cessation medicines helped smokers with psychiatric disorders quit without a significant rise in serious psychiatric side effects (Anthenelli et al., 2016), and the Public Health Service guideline recommends combining medication with counseling (Fiore et al., 2008). The prescriber also needs to know, since smoking speeds the breakdown of some antipsychotics. It beats B because unaided cutting down works far less often than treatment.

Takeaway: Offer evidence-based tobacco treatment to clients with stable serious mental illness who want to quit, and involve the prescriber.

Sources: Anthenelli, R. M., Benowitz, N. L., West, R., et al. (2016). Neuropsychiatric safety and efficacy of varenicline, bupropion, and nicotine patch in smokers with and without psychiatric disorders (EAGLES): A double-blind, randomised, placebo-controlled clinical trial. Lancet, 387(10037), 2507-2520, whole article; Fiore, M. C., et al. (2008). Treating Tobacco Use and Dependence: 2008 Update. Clinical Practice Guideline. U.S. Department of Health and Human Services, Public Health Service, Recommendations: combining counseling and medication

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

56. ASM-1110 (rev 2) · Best answer: D

In a nursing facility, a resident with major neurocognitive disorder strikes staff during evening showers but calmly accepts morning sponge baths. An adult child shares that the resident was held in a crowded detention camp as a teenager, where guards forced group showers. Staff ask the social worker to request a sedative before showers. What is the **BEST** response?

A. Ask the prescriber to consider a sedative before showers
B. Help the resident talk through the camp memories
C. Record the hitting as aggression from the dementia
D. Adjust the shower routine to remove the camp reminder

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Medication may be considered if distress continues after the routine changes. Requesting it first treats a trauma reaction as a symptom to suppress and adds sedation and fall risk for an older adult.

B **Incorrect** (trap: Ignores risk). Talking through trauma can help in some therapies, but asking a resident with major neurocognitive disorder to revisit camp memories can deepen distress without removing the trigger. The immediate need is a safer bathing routine.

C **Incorrect** (trap: Near-miss diagnosis). Neurocognitive disorder can cause agitation, but this pattern is tied to evening showers and matches a known trauma history. Labeling it dementia-related aggression misses the cause and invites the wrong response.

D **Correct.** Trauma-informed care means recognizing reminders of past trauma and changing practices to avoid re-traumatizing (SAMHSA, 2014), and federal nursing facility rules require trauma-informed care that reduces triggers (42 CFR 483.25(m)). The resident fights only during showers that echo forced camp showers and accepts sponge baths, so the behavior points to a trigger, not random aggression. Changing the routine treats the cause. It beats A because a sedative would mask distress, add sedation and fall risk, and skip a nondrug change the pattern suggests will work.

Takeaway: When an older adult's distress tracks a situation linked to past trauma, change the care routine to remove the trigger.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Four Rs: realize, recognize, respond, resist re-traumatization; Requirements for Long-Term Care Facilities (CMS), 42 CFR 483.25(m)

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

57. ASM-1555 (rev 2) · Best answer: C

A 15-year-old tells the school social worker that an online contact who seemed to be a peer received a private image from the teen and now demands money. The contact threatens to send the image to the teen's followers by tonight. Shaking, the teen says they want to delete everything and pretend it never happened. What should the social worker do **FIRST**?

A. Help the teen delete the chat and close the account
B. Ask the teen what led them to share the image
C. Help the teen save the messages and file a report

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Wanting to erase the problem is understandable, but deleting the messages and account destroys evidence. Blocking the contact is advised only after the profile and messages are saved for a report.

B **Incorrect** (trap: Right action, wrong time). Asking why the teen shared the image can sound blaming and delays action before tonight's deadline. Understanding what happened can come later, in a supportive conversation, after the report is made.

C **Correct.** Sextortion of a minor is a crime. FBI guidance tells young victims to report it to the FBI or NCMEC's CyberTipline, to block the offender without deleting the profile or messages, and not to pay; NCMEC can also help remove images (FBI, 2020; FBI Albuquerque press release). Saved messages help investigators identify the offender, and tonight's deadline makes prompt reporting the priority. It beats A because deleting everything, the teen's instinct, destroys the evidence needed to stop the threat.

Takeaway: In sextortion of a minor, preserve the evidence and report promptly; deleting messages or pausing to explore blame helps the offender.

Sources: FBI (2020). Sextortion, fbi.gov guidance for young victims: report; NCMEC CyberTipline; Take It Down; FBI Albuquerque Field Office (2022). FBI Media Alert: FBI in New Mexico Warns of Significant Increase in Sextortion, Press release, Dec. 19, 2022: block the predator but do not delete the profile or messages
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.21 Technology and social media effects on clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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58. ASM-1594 (rev 1) · Best answer: A

Which tool measures how well an older adult can handle money, medicines, shopping and meals?

A. Lawton Instrumental Activities of Daily Living Scale
B. Katz Index of Independence in Activities of Daily Living
C. Mini-Cog brief cognitive screen

Why each option is right or wrong

A **Correct.** The Lawton scale measures instrumental activities of daily living, the complex tasks needed to live independently, such as managing money, medications, shopping, cooking and transportation (Lawton & Brody, 1969). Money, medications, shopping and meal preparation are exactly these tasks, and losing them often signals the first loss of independence. It beats B because the Katz Index measures basic self-care, such as bathing, dressing and feeding, and would miss these higher-level tasks.

B **Incorrect** (trap: Wrong model or concept). The Katz Index measures basic activities of daily living: bathing, dressing, toileting, transferring, continence and feeding (Katz et al., 1963). Those basic tasks are often kept long after money, medication and shopping skills decline, so the Katz Index would miss the areas named.

C **Incorrect** (trap: Wrong model or concept). The Mini-Cog screens for cognitive impairment with word recall and clock drawing. It may help explain the decline, but it does not measure the daily living tasks the question asks about.

Takeaway: Basic ADLs cover self-care (Katz); instrumental ADLs cover independent-living tasks such as finances, medications and shopping (Lawton).

Sources: Lawton, M. P., & Brody, E. M. (1969). Assessment of older people: Self-maintaining and instrumental activities of daily living. The Gerontologist, 9(3), 179-186, whole article; Katz, S., Ford, A. B., Moskowitz, R. W., Jackson, B. A., & Jaffe, M. W. (1963). Studies of illness in the aged. The index of ADL. JAMA, 185(12), 914-919, whole article

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.6 Tools for assessing strengths, resources, needs, challenges · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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59. ASM-1005 (rev 2) · Best answer: A

On an inpatient unit, a social worker reviews the 4-year history of a 33-year-old patient. The patient has had four manic episodes, each with grandiose delusions and voices, and these episodes have filled most of the illness. Twice, after the mood returned to normal, the patient kept hearing voices that criticized them for about 5 weeks. What is the **MOST** likely diagnosis?

A. Schizoaffective disorder, bipolar type
B. Bipolar I disorder with psychotic features
C. Schizophrenia
D. Delusional disorder

Why each option is right or wrong

A **Correct.** Schizoaffective disorder requires a major mood episode occurring along with schizophrenia's core symptoms (here, delusions and hallucinations), at least 2 weeks of hallucinations or delusions with no mood episode, and mood episodes present for most of the illness (DSM-5-TR). Manic episodes with grandiose delusions and voices, voices lasting about 5 weeks after the mood normalized, and an illness dominated by manic episodes meet all three. The manic episodes make it the bipolar type. It beats B because in bipolar I with psychotic features, psychosis occurs only during mood episodes; here it continued for weeks after the mood returned to normal.

B **Incorrect** (trap: Near-miss diagnosis). Bipolar I with psychotic features fits the manic episodes, but its psychosis happens only during mood episodes. Voices that continue for weeks while mood is normal point to schizoaffective disorder instead.

C **Incorrect** (trap: Near-miss diagnosis). Schizophrenia fits an illness dominated by psychosis, with mood episodes present only briefly. Here manic episodes filled most of the illness, so schizophrenia is not the best fit.

D **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves fixed beliefs without prominent hallucinations or major mood episodes dominating the course. Repeated mania and critical voices do not fit it.

Takeaway: Psychosis for 2+ weeks without a mood episode, in an illness dominated by mood episodes, points to schizoaffective disorder rather than a mood disorder with psychotic features.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizoaffective Disorder; DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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60. INT-1077 (rev 1) · Best answer: B

A high school social worker completes a suicide risk assessment with a 16-year-old who reported fleeting thoughts that 'it would be easier not to wake up.' The thoughts come at night when the student worries about grades, and the student names a parent and a coach as supports. An assistant principal asks the worker to have the student sign a no-harm agreement before returning to class. What is the **MOST** appropriate step?

A. Have the student sign the agreement the administrator requested
B. Build a written safety plan with the student and family
C. Send the student back to class and check in next week

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The request sounds protective, but a signed promise not to self-harm has no evidence of preventing suicide and gives no legal protection. It can also give staff false reassurance. A safety plan gives the student skills and contacts instead.

B **Correct.** A safety plan is a written, collaborative list of warning signs, coping strategies, people and places for distraction, people to ask for help, crisis contacts and steps to make the environment safer (Stanley & Brown, 2012). It gives the student and family concrete actions for the nighttime worries described. It beats A because no-harm agreements lack evidence that they prevent suicide and do not protect the worker legally; critics recommend replacing them with a commitment to treatment and safety planning (Rudd et al., 2006).

C **Incorrect** (trap: Ignores risk). The thoughts are low-level, but the student still needs a plan for the worried nights when they occur. Sending the student back without coping steps or family involvement leaves the risk unaddressed until next week.

Takeaway: After a risk assessment, build a collaborative written safety plan; no-harm contracts lack evidence and are not a substitute.

Sources: Stanley, B., & Brown, G. K. (2012). Safety planning intervention. Cognitive and Behavioral Practice, 19(2), 256-264, safety plan components; Rudd, M. D., Mandrusiak, M., & Joiner, T. E. (2006). The case against no-suicide contracts: The commitment to treatment statement as a practice alternative. Journal of Clinical Psychology, 62(2), 243-251, whole article
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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61. ETH-1686 (rev 2) · Best answer: D

A hospital field instructor has given an MSW intern weekly feedback and a written learning plan for missed notes and late arrivals. At midterm, the intern still falls short on several competencies. The intern then discloses a learning disability, says the school has never been told, and asks the instructor not to give a failing rating. What should the field instructor do **NEXT**?

A. Give a passing midterm rating while the intern seeks help
B. Arrange accommodations with the intern without involving the school
C. End the field placement, since the learning plan has failed
D. Involve the school's field liaison in reviewing the plan

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A passing rating that does not reflect performance misleads the school and the intern and could put patients at risk. The disclosure does not change what the intern has shown so far; it may change supports going forward.

B **Incorrect** (trap: Outside the social work role). Adjusting tasks informally is well meant, but formal accommodations run through the school, which decides what is reasonable within program standards. Private arrangements can be unfair to the intern and leave the evaluation on uncertain footing.

C **Incorrect** (trap: Hands off too soon). Ending the placement at midterm, right after a disability disclosure and before the school is involved, is premature and could look discriminatory. It may be considered later if the intern still falls short with proper supports.

D **Correct.** Field instructors evaluate students fairly and against clearly stated criteria (NASW Code 3.02(b); 3.03). The school runs the field program and its disability accommodation process, so when a student may not meet competencies, the field liaison shares the next decision. The liaison can review the learning plan and help the intern seek formal accommodations; the instructor shares the performance concerns and leaves disclosure of the disability to the intern. It beats B because accommodations arranged privately on site bypass the school's process and may not fit program standards.

Takeaway: When a student is falling short, involve the school's field liaison; accommodations go through the school, and evaluations stay honest and criteria-based.

Sources: NASW Code of Ethics (2021), 3.02(b); NASW Code of Ethics (2021), 3.03
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

Section 2

62. INT-1590 (rev 2) · Best answer: B

A supported employment program's new contract pays the agency only for clients still employed 6 months after enrollment. Within a quarter, the social worker who manages the program sees intake staff steering applicants with criminal records or recent hospitalizations to a waitlist. Placement rates have risen. Intake staff completed eligibility training last month and say they fear losing the contract. What action is **BEST**?

A. Collect another quarter of intake data to confirm the pattern
B. Raise the pattern with leaders and seek fairer performance measures
C. Remind intake staff of the eligibility rules at a staff meeting

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). More data can firm up a pattern, but staff have already said why they are steering applicants, and each added quarter turns away more of the people the program exists to serve. Waiting would make sense only if the pattern were unclear.

B **Correct.** Evaluation and payment measures shape services: rewarding only easy successes can lead a program to enroll the clients most likely to succeed and turn away those with the greatest need. The fix has to reach the incentive, so the manager brings the data to leaders and advocates for measures that reward progress or adjust for client difficulty (NASW Code 3.07(b), 3.09(b)). This beats C because staff were trained on the rules last month and are responding to the contract; a reminder leaves that pressure in place.

C **Incorrect** (trap: Good but not best). A reminder treats the problem as a knowledge gap. Staff were trained on the rules last month and are reacting to the payment structure, so the screening is likely to continue unless the incentive itself changes.

Takeaway: When performance measures push staff to exclude harder-to-serve clients, address the measure itself through agency and funder channels.

Sources: NASW Code of Ethics (2021), 3.07(b); NASW Code of Ethics (2021), 3.09(b)
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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63. ETH-1535 (rev 1) · Best answer: A

A 15-year-old in foster care asks to attend the upcoming meeting where the team will discuss changing the permanency goal from reunification to adoption. The foster parent suggests keeping the youth away because the discussion may be upsetting. What is the **MOST** appropriate response by the child welfare social worker?

A. Include the youth after preparing them for the meeting
B. Hold the meeting without the youth and report back
C. Ask the youth to send written views instead

Why each option is right or wrong

A **Correct.** Law limits minors' self-determination, but social workers still involve young people in decisions about their lives at a developmentally appropriate level (NASW Code 1.02). Federal law requires that case plans for youth 14 and older be developed in consultation with the youth (42 U.S.C. 675(1)(B)). A 15-year-old asking to take part in a decision about adoption should be included, with preparation and support during the meeting. It beats C because written input is a fallback when attendance is not possible; it keeps the youth out of the actual discussion.

B **Incorrect** (trap: Overrides the client's choice). Shielding the youth from distress may seem protective, but it shuts the youth out of a decision central to their future. Preparation and support address the risk of upset without removing the youth's voice.

C **Incorrect** (trap: Good but not best). Written input respects the youth's views more than exclusion does, and it suits a youth who prefers not to attend. This youth asked to be present, so substituting a letter limits participation without good reason.

Takeaway: Involve youth in decisions about their own care at a developmentally appropriate level; prepare and support them rather than excluding them.

Sources: NASW Code of Ethics (2021), 1.02; 42 U.S.C. 675 (Social Security Act, Title IV-E case plan requirements), 675(1)(B)

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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64. ASM-1627 (rev 3) · Best answer: A

A home health social worker meets with an adult child who cares full time for a 78-year-old parent with advanced Parkinson's disease. The caregiver reports back pain, poor sleep and missed work. When the social worker mentions an adult day program, the caregiver shakes their head and says, 'That's not how our family does things.' What should the social worker do **NEXT**?

A. Ask what kinds of help would feel acceptable to the caregiver

B. Arrange in-home aide visits in place of the day program

C. Urge the caregiver to try the day program for a week

Why each option is right or wrong

A **Correct.** The caregiver's remark signals a family or cultural norm about caregiving, but its meaning is still unclear: it may concern strangers, shame, duty or a past bad experience. Asking what help would feel acceptable explores that meaning and invites options the family can accept, such as relatives sharing shifts or help at home. Cultural values shape caregiving largely through how families use support (Knight & Sayegh, 2010; NASW Family Caregivers Standard 4). It beats B because in-home aides still bring outsiders into care, and the social worker does not yet know whether that would be acceptable.

B **Incorrect** (trap: Skips assessment). In-home help suits many caregivers and may fit this family. Arranging it now assumes the objection is only about leaving home; if the concern is outsiders providing care, aides will be refused as well.

C **Incorrect** (trap: Overrides the client's choice). Respite would ease the strain, but pressing a service the caregiver has just declined overrides the caregiver's stated values and may end the conversation. The program can be revisited later if the family's view changes.

Takeaway: When a caregiver declines help on family or cultural grounds, explore what support would fit their values before proposing alternatives.

Sources: Knight, B. G., & Sayegh, P. (2010). Cultural values and caregiving: The updated sociocultural stress and coping model. The Journals of Gerontology, Series B, 65B(1), 5-13, whole article; NASW Standards for Social Work Practice with Family Caregivers of Older Adults (2010), Standard 4: Cultural and Linguistic Competence

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.14 How caregiving affects families · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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65. ASM-0115 (rev 1) · Best answer: C

A parent seeks help for a 9-year-old's constant defiance and says the school complains about the child every day. With the parent's written consent to exchange information with the school, the social worker obtains school records. The records show good grades, no discipline reports and positive comments from staff. What should the social worker do **NEXT**?

A. Diagnose oppositional defiant disorder from the parent's report

B. Refer the parent to a behavioral parent training program

C. Discuss the conflicting accounts with the parent and teacher

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A diagnosis built on one informant, after records contradict part of that informant's account, skips the work of reconciling the data. Defiance limited to home can still matter, but the conflicting reports must be understood first.

B **Incorrect** (trap: Right action, wrong time). Parent training may well help if defiance at home is confirmed, and it is an evidence-based option. Recommending it now acts before the social worker understands why the home and school pictures differ.

C **Correct.** Collateral records are integrated with other information, and differences between informants are explored rather than ignored or used to discard one source (De Los Reyes & Kazdin, 2005). The parent describes daily complaints from school, but the records show none, so the next step is to clarify the conflict with both the parent and the teacher. Behavior that appears only at home is itself useful information. It beats B because recommending treatment before reconciling the accounts acts on an incomplete picture.

Takeaway: When collateral records conflict with a client's account, explore the discrepancy with the informants before drawing conclusions or planning treatment.

Sources: De Los Reyes, A., & Kazdin, A. E. (2005). Informant discrepancies in the assessment of childhood psychopathology: A critical review, theoretical framework, and recommendations for further study. Psychological Bulletin, 131(4), 483-509, whole article; Achenbach, T. M., McConaughy, S. H., & Howell, C. T. (1987). Child/adolescent behavioral and emotional problems: Implications of cross-informant correlations for situational specificity. Psychological Bulletin, 101(2), 213-232, whole article

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.11 Integrating records and evaluations (medical, school, psychological, employment) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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66. ETH-1538 (rev 2) · Best answer: C

Before elective surgery, a nurse tells an adult patient that the hospital requires an advance directive on file. The upset patient tells the hospital social worker they do not want to make one and asks whether the surgery can go ahead without it. What should the social worker do **FIRST**?

A. Help the patient complete a brief directive
B. Clarify the hospital's duties with the nurse
C. Inform the patient that a directive is voluntary

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Helping the patient fill out a form treats the nurse's mistaken claim as true and pressures the patient into a document they declined. An advance directive must reflect the patient's voluntary choice.

B **Incorrect** (trap: Right action, wrong time). Correcting the nurse's understanding is an important follow-up so other patients are not misinformed. Doing it first leaves an upset patient believing surgery depends on a document they do not want.

C **Correct.** Federal rules require hospitals to give adult inpatients written information about advance directives at admission, but hospitals may not condition care on whether a patient has one (42 CFR 489.102). The patient has been misinformed, is upset and has asked directly, so the first step is to answer the patient and affirm the right to decline (NASW Code 1.02). It beats B because the nurse does need accurate information, but that conversation can follow; the patient's informed choice and distress come first.

Takeaway: Hospitals must offer advance directive information but cannot require a directive; patients may decline without any effect on care.

Sources: 42 CFR 489.102 (Patient Self-Determination Act requirements), (a)(1), (a)(3); NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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67. ASM-1054 (rev 1) · Best answer: B

An ACT team social worker visits a 23-year-old client 6 months after a first hospitalization for schizophrenia, during which the voices stopped. The client now speaks of having 'ruined everything,' feels guilty about burdening family, sleeps poorly and no longer enjoys music. The client is tearful and expressive while talking. A team member calls this negative symptoms. What is the **MOST** likely explanation?

A. Negative symptoms of schizophrenia
B. Co-occurring depression
C. Antipsychotic-induced parkinsonism
D. Schizoaffective disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Negative symptoms such as diminished expression and low motivation can resemble depression, but they lack prominent guilt and hopelessness. This client is tearful and expressive, which argues against blunted emotional expression.

B **Correct.** Depression commonly co-occurs with schizophrenia, often after a first episode as insight returns, and it raises suicide risk. Guilt, hopelessness ('ruined everything'), poor sleep and lost enjoyment, with visible tearfulness, point to depression: the client feels sadness and shows it. It beats A because negative symptoms involve reduced expression and drive without this felt sadness and guilt. Recognizing co-occurring depression should prompt a suicide risk assessment and treatment planning (DSM-5-TR; Kaplan & Sadock).

C **Incorrect** (trap: Near-miss diagnosis). Antipsychotic-induced parkinsonism causes a masklike face, slowed movement and stiffness that can mimic depression. It does not explain guilt or hopeless statements, and this client shows expressive emotion.

D **Incorrect** (trap: Near-miss diagnosis). Schizoaffective disorder requires a major mood episode during the active psychotic phase, with mood symptoms present for most of the illness. Here low mood began after the psychosis remitted, which fits depression co-occurring with schizophrenia.

Takeaway: In schizophrenia, guilt, hopelessness and felt sadness point to co-occurring depression rather than negative symptoms and call for suicide risk assessment.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Schizophrenia: depression and suicide; differential of negative symptoms

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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68. ASM-1615 (rev 1) · Best answer: C

A client raised in a rural Latin American community is in outpatient treatment for depression. The client believes the illness began with susto, a loss of spirit caused by a frightening event. The client plans to visit a traditional healer for a cleansing and asks the social worker whether that is 'allowed' alongside counseling. What should the social worker do **FIRST**?

A. Refer the client to a provider who shares the client's culture
B. Contact the healer to coordinate care with counseling
C. Ask how the client hopes the healer's work will help
D. Explain that depression is a treatable medical condition

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). A culture-matched provider is not needed to respect the client's beliefs, and transferring would disrupt care that is underway. The current social worker can bring the client's healing practice into the plan.

B **Incorrect** (trap: Right action, wrong time). Coordinating with the healer may help later, with the client's consent. Doing it first skips learning what the practice involves and what the client wants from it, and any contact outside the treatment team needs the client's permission.

C **Correct.** Culturally responsive planning starts with the client's explanatory model and the help the client is seeking, including from traditional healers (DSM-5-TR Cultural Formulation Interview). Asking how the client hopes the healer will help answers the 'allowed' question collaboratively and shows what the practice involves, so it can be

woven into the plan. Engaging natural helpers such as folk healers is part of culturally competent practice (NASW Cultural Competence Standard 4). It beats B because coordinating with the healer is a reasonable later step that needs the client's understanding and consent first.

D **Incorrect** (trap: Too directive). Leading with a biomedical explanation dismisses the client's understanding of the illness and may close the conversation. The client's model can coexist with counseling; correcting it is not the goal.

Takeaway: Begin culturally responsive planning by eliciting the client's explanatory model and the help sought, then integrate traditional healing with consent.

Sources: DSM-5-TR (APA, 2022), Section III, Culture and Psychiatric Diagnosis: Cultural Formulation Interview; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills

Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.4 Culture in intervention planning · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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69. INT-1059 (rev 1) · Best answer: D

In the Thomas-Kilmann model of conflict, which mode combines high assertiveness with high cooperativeness?

A. Compromising
B. Accommodating
C. Competing
D. Collaborating

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Compromising is intermediate on both assertiveness and cooperativeness. Each side gives up something to reach a quick middle ground, so neither party's concerns are fully met.

B **Incorrect** (trap: Wrong model or concept). Accommodating is cooperative but unassertive: one party sets aside its own concerns to satisfy the other. It can protect a relationship but leaves the accommodating party's needs unmet.

C **Incorrect** (trap: Wrong model or concept). Competing is assertive but uncooperative: one party pursues its own concerns at the other's expense. It can fit an emergency that needs a fast, firm decision.

D **Correct.** Thomas and Kilmann map conflict-handling modes on two axes: assertiveness, pursuing one's own concerns, and cooperativeness, pursuing the other party's concerns. Collaborating is high on both: the parties work together toward a solution that fully meets each side's concerns, often by digging into underlying interests (Thomas & Kilmann, 1974). It beats A because compromising sits in the middle of both axes and gives each side part of what it wants, rather than a solution that satisfies both fully.

Takeaway: Collaborating is high in both assertiveness and cooperativeness; compromising is moderate on both and splits the difference.

Sources: Thomas, K. W., & Kilmann, R. H. (1974). Thomas-Kilmann Conflict Mode Instrument. Xicom, five conflict-handling modes
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.19 Resolving conflict · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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70. ETH-1590 (rev 1) · Best answer: D

At a domestic violence program, a social worker is safety planning with an adult client whose partner has made threats. The client says calling the police is off the table because officers injured a relative during a past call. The client names a cousin, a neighbor and a church member who would help. What is the **MOST** appropriate action by the social worker?

A. Encourage the client to call police in an emergency despite past harm
B. Explore the client's feelings about the relative's injury further
C. Refer the client to a program with police liaison advocates
D. Build the plan around the people and options the client trusts

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Police response is an option to discuss, and the client may choose it. Urging it over the client's stated, experience-based reasons overrides self-determination and makes the plan less likely to be followed.

B **Incorrect** (trap: Good but not best). The client has already explained the reason clearly. More exploration of the past event delays planning while a threat is active; trauma work can come later if the client wants it.

C **Incorrect** (trap: Hands off too soon). Referring out moves the client away from a program already able to plan with them. Police-linked advocacy also centers the option the client has ruled out.

D **Correct.** Anti-oppressive safety planning starts from the client's own analysis of risk, including well-founded distrust of police where communities have been harmed, and builds on supports the client identifies (NASW Code 1.02; 1.05(c)). The client has named three people who would help and a clear reason to avoid police. Building the plan around them yields a plan the client will use, while the worker can still share every emergency option. It beats A because pressing police contact overrides a competent client's informed judgment and may make the plan less likely to be used.

Takeaway: Safety plans build on the supports a client trusts, including community alternatives when police contact feels unsafe to the client.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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71. ETH-1168 (rev 1) · Best answer: B

A social worker is conducting an IRB-approved research interview with an older adult about caring for a spouse who died last year. Partway through, the participant begins crying while describing the final weeks of the spouse's illness. What should the social worker do **FIRST**?

A. Continue with the protocol so the data stay consistent
B. Pause and acknowledge how painful this is to recall
C. End the interview and give a list of grief resources

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Sticking to the protocol puts data ahead of the participant's well-being. The Code requires protecting participants from unwarranted distress, and data consistency does not outweigh responding to visible grief.

B **Correct.** Researchers must protect participants from unwarranted distress and make sure they can reach supportive services (NASW Code 5.02(k), 5.02(j)). The first step when a participant becomes upset is to pause and respond to the person, acknowledging the grief rather than pressing on or shutting down. Then the social worker can offer choices: a break, skipping questions, stopping or continuing, which respects the right to withdraw (5.02(i)). It beats C because ending the interview without checking what the participant wants removes their choice and can feel like dismissing their grief.

C **Incorrect** (trap: Right action, wrong time). Offering grief resources is appropriate, and ending is an option if the participant chooses it. Doing it first, without pausing or asking, decides for the participant and can feel abrupt at a vulnerable moment.

Takeaway: When a research participant becomes distressed, pause and respond to the person first, then offer the choice to continue, skip or stop.

Sources: NASW Code of Ethics (2021), 5.02(k); NASW Code of Ethics (2021), 5.02(j); NASW Code of Ethics (2021), 5.02(i)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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72. INT-1520 (rev 1) · Best answer: D

Which treatment objective for a client with depression is written in measurable terms?

A. Client will feel less depressed and more hopeful about the future
B. Client will increase pleasant activities as often as possible
C. Client will gain insight into the causes of the low mood
D. Client will walk 20 minutes on 5 days each week by week 6

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Hope and mood matter, but 'feel less depressed' names no observable behavior, amount, or deadline. It reads as a broad goal that needs specific objectives under it.

B **Incorrect** (trap: Misuses the technique). This names a behavior, but 'as often as possible' sets no amount or time frame, so there is no way to tell when it has been met.

C **Incorrect** (trap: Good but not best). Insight is an internal state that cannot be observed or counted, and the objective has no time frame. It may be a process aim, but it is not a measurable objective.

D **Correct.** Measurable objectives name an observable behavior, a quantity, and a time frame, so client and worker can tell whether they were met (Hepworth et al.). Walking 20 minutes on 5 days a week by week 6 can be counted and checked. It beats B because B names a behavior, but 'as often as possible' sets no amount or deadline, so progress cannot be judged. Measurable objectives also make ongoing progress monitoring possible (Bloom, Fischer & Orme, 2009).

Takeaway: A measurable client objective states an observable behavior, how much or how often, and by when.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract; Bloom, Fischer & Orme (2009). Evaluating Practice (6th ed.), specifying goals and objectives
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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73. INT-1019 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

A 6-year-old in a new foster home has daily meltdowns. When the child welfare social worker asks how the child felt before a meltdown, the child shrugs or says 'bad.' The foster parent wants help so the child can calm down. What should the social worker do **FIRST** to build the child's regulation skills?

A. Ask the child to tell the story of leaving home
B. Set up a sticker chart that rewards calm days
C. Teach the child to name feelings using a faces chart

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Telling the story of a painful separation belongs to later trauma-processing work, after the child has coping and regulation skills. Asking for it first could raise distress in a child already struggling to stay regulated.

B **Incorrect** (trap: Good but not best). Rewards can reinforce calm behavior and may help alongside skills teaching. Without a way to notice and name feelings, though, the child has no tool for earning the reward, and repeated failure may add frustration.

C **Correct.** Emotion regulation builds on emotional awareness: children first learn to recognize and name feelings, then link them to body cues and calming steps. Trauma-focused CBT, for example, teaches feelings identification among its early coping-skills components, well before any trauma narrative (Cohen, Mannarino & Deblinger, 2017). A child who can only say 'bad' needs that vocabulary first. It beats B because a sticker chart rewards calm days but gives the child no way to notice rising feelings and respond.

Takeaway: A child who cannot name feelings needs feelings identification first; regulation strategies build on it, and trauma processing comes much later.

Sources: Cohen, J. A., Mannarino, A. P., & Deblinger, E. (2017). Treating Trauma and Traumatic Grief in Children and Adolescents (2nd ed.). Guilford Press, affective modulation skills
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.6 Teaching emotion-regulation skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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74. INT-1575 (rev 1) · Best answer: C

A 14-year-old in a residential treatment program has been banging their head against walls several times a week. Staff incident reports from the past 6 weeks record the date and time of each episode. The team is ready to start a new behavior plan and wants to judge whether it works. Which approach is **BEST**?

A. Count episodes for 2 more weeks before starting the plan
B. Start the plan and log episodes in new incident reports
C. Use the past 6 weeks of incident reports as the baseline
D. Plan to pause the behavior plan later to confirm its effect

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Collecting a fresh baseline before intervening is the usual first step when a problem is not urgent. Here self-injury is happening several times a week and dated records already exist, so a 2-week delay adds risk without adding needed information.

B **Incorrect** (trap: Good but not best). Tracking during treatment is useful and is the fallback when no earlier data exist. Ignoring 6 weeks of dated records, though, gives up the before-and-after comparison the team needs to judge whether the plan works.

C **Correct.** When reliable records of the target behavior already exist, the baseline can be reconstructed from them so that treatment starts at once (Di Noia & Tripodi, 2008). Dated incident reports covering 6 weeks give repeated measures of head banging, and self-injury makes any delay hard to justify. This beats B because starting without using the records leaves no pre-plan level to compare against, so the team could not tell whether episodes actually dropped.

D **Incorrect** (trap: Ignores risk). Withdrawing a plan to see whether head banging returns can strengthen causal claims, but it would expose the youth to renewed self-injury. With a behavior this harmful, designs that avoid withdrawal are preferred (Bloom, Fischer & Orme, 2009).

Takeaway: When reliable records of the target problem already exist, use them as a reconstructed baseline instead of delaying urgent treatment.

Sources: Di Noia, J., & Tripodi, T. (2008). Single-Case Design for Clinical Social Workers (2nd ed.). NASW Press, baselines: retrospective reconstruction from archival data; Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: withdrawal designs and ethical limits
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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75. ASM-1009 (rev 2) · Best answer: B

A 76-year-old client is evaluated at a memory clinic. Over the past year, the client's alertness has varied widely from day to day, and the client often sees small children in the living room who are not there. Months after the memory changes began, the client developed a shuffling walk and stiff arms. The client's partner says the client now needs help with bills and medications. What is the **MOST** likely diagnosis?

A. Major neurocognitive disorder due to Alzheimer's disease
B. Major neurocognitive disorder with Lewy bodies
C. Delirium
D. Major neurocognitive disorder due to Parkinson's disease

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Alzheimer's disease typically starts with gradual memory loss and steady decline. Marked day-to-day swings in alertness, early well-formed visual hallucinations and later parkinsonism point to Lewy bodies instead.

B **Correct.** Neurocognitive disorder with Lewy bodies is marked by fluctuating cognition with changes in attention and alertness, recurrent well-formed visual hallucinations, and parkinsonism that begins after the cognitive decline (DSM-5-TR). This client shows all three, and needing help with bills and medications makes it major. It beats D because neurocognitive disorder due to Parkinson's disease arises in the setting of already established Parkinson's disease; here the shuffling walk and stiffness began months after the memory changes. Recognition matters, because people with this condition can react severely to antipsychotic medication.

C **Incorrect** (trap: Near-miss diagnosis). Delirium fluctuates and can cause hallucinations, which makes it tempting. It begins over hours to days and has a medical cause, while this decline has built gradually over a year.

D **Incorrect** (trap: Near-miss diagnosis). This diagnosis also combines cognitive decline, parkinsonism and sometimes hallucinations, which makes it a close neighbor. It applies when cognitive decline develops in someone with established Parkinson's disease. Here the movement signs came months after the memory problems, the sequence that points to Lewy bodies (DSM-5-TR).

Takeaway: Fluctuating alertness, recurrent detailed visual hallucinations and later parkinsonism in a gradual decline suggest neurocognitive disorder with Lewy bodies.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major or Mild Neurocognitive Disorder With Lewy Bodies

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

76. INT-1640 (rev 1) · Best answer: A

In an intensive care unit, a patient's family has heard different predictions about recovery from the cardiology and neurology teams. The family asks for a meeting, and the social worker is asked to arrange it. What should the social worker do **FIRST**?

A. Convene a brief team meeting to align the medical message
B. Ask the family to list the questions they most want answered
C. Schedule the family meeting so each team can present its view

Why each option is right or wrong

A **Correct.** Family conferences go better when clinicians meet beforehand to share what each knows and agree on the information and goals for the meeting (Curtis & White, 2008). This family's distress comes from conflicting messages, so the teams must reconcile their views before meeting the family together. Contributing to team decisions is part of the social worker's role (NASW Code 2.03(a)). It beats B because gathering the family's questions is useful preparation, but it helps most once the team has a shared message to answer them with.

B **Incorrect** (trap: Right action, wrong time). Learning the family's questions is good preparation, but it does not fix the source of the confusion. Without a shared team message, the answers will conflict again in the meeting.

C **Incorrect** (trap: Good but not best). Bringing every team together sounds transparent, but presenting differing predictions side by side in front of the family repeats the confusion. Disagreements should be worked through among clinicians first.

Takeaway: Before a family conference, the team meets to reconcile information and agree on goals, especially when the family has heard conflicting messages.

Sources: Curtis, J. R., & White, D. B. (2008). Practical guidance for evidence-based ICU family conferences. Chest, 134(4), 835-843, Preparing for the family conference; NASW Code of Ethics (2021), 2.03(a)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.3 Inter- and intradisciplinary teamwork (co-therapy, care conferences, IEPs) · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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77. INT-1141 (rev 1) · Best answer: A

During in-home structural family therapy for a 13-year-old's aggression, one parent asks to speak privately. The parent says the other parent controls the money, checks the parent's phone, and pushed the parent into a wall last week. The parent asks the social worker to keep family sessions going, fearing that stopping them would anger the other parent. What is the **BEST** response?

A. Connect the parent with a domestic violence advocate to plan safely
B. Continue family sessions as asked, keeping the focus on the teen
C. Stop family sessions and explain the reason to the family
D. Ask the other parent privately about the pushing

Why each option is right or wrong

- **A** **Correct.** Family and couples therapy may be dangerous for a survivor of partner violence, and a provider who learns of abuse should refer the survivor to a domestic violence program (SAMHSA TIP 25, Ch. 2). The parent describes coercive control and a recent assault and fears retaliation if sessions change, so any change needs a careful safety plan. A domestic violence advocate can plan how and when to adjust the sessions without raising risk. It beats B because continuing joint work as if nothing changed can expose the parent to retaliation and leaves the danger unaddressed.
- **B** **Incorrect** (trap: Ignores risk). Honoring the parent's request and the teen's needs is appealing, and abrupt changes can raise risk. Still, joint sessions with a controlling partner can lead to retaliation for what is said in them. Decisions about the sessions should come from a safety plan, not continue by default.
- **C** **Incorrect** (trap: Misreads law or ethics). Stopping family sessions may be part of the eventual plan, but announcing the reason reveals the parent's private disclosure without consent and could provoke the retaliation the parent fears.
- **D** **Incorrect** (trap: Ignores risk). Raising the pushing with the other parent tells the person using violence that the parent has disclosed it. That can increase danger and breaks the trust the parent placed in the social worker.

Takeaway: When partner abuse surfaces in family therapy, plan safety with a domestic violence specialist before changing or continuing joint sessions.

Sources: SAMHSA TIP 25 (1997), Ch. 2: couples and family therapy with survivors; referral to domestic violence programs

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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78. ASM-1129 (rev 2) · Best answer: A

Group home staff call the social worker about a resident with schizophrenia. For 2 days, the resident has taken only sips of water and almost no food. The resident stands in one spot for hours, holds odd postures, does not answer questions and stiffens when staff try to guide them. What should the social worker do **FIRST**?

- **A.** Get the resident to an emergency department today
- **B.** Rate the resident's negative symptoms with a standard scale
- **C.** Ask the psychiatrist to see the resident at the next clinic visit
- **D.** Have staff push fluids and track intake at the home

Why each option is right or wrong

- **A** **Correct.** Mutism, posturing, rigid resistance to being moved and refusal of food and fluids are signs of catatonia, which DSM-5-TR recognizes in schizophrenia and other conditions. Catatonia with poor intake can lead to dehydration, malnutrition and other life-threatening complications, so it needs urgent medical evaluation today. It beats C because waiting for a routine psychiatry visit leaves a resident who is barely drinking without care, and the cause, including a medical one, must be found quickly (DSM-5-TR).
- **B** **Incorrect** (trap: Near-miss diagnosis). These are not negative symptoms such as low motivation or reduced expression. Mutism, posturing and resistance to movement are

catatonic signs, and rating negative symptoms delays urgent care for a resident who is barely drinking.

C **Incorrect** (trap: Ignores risk). Psychiatric follow-up matters, but 2 days of almost no fluids with catatonic signs cannot wait for a routine visit. A prompt psychiatry appointment fits only after medical stability is assured.

D **Incorrect** (trap: Ignores risk). Encouraging fluids and tracking intake is sensible for a resident who is eating poorly for other reasons. A resident with catatonic signs often cannot respond to prompting, and 2 days of minimal fluids already calls for same-day medical evaluation, so home monitoring delays care.

Takeaway: Catatonic signs with poor food or fluid intake are a medical emergency; arrange same-day evaluation rather than routine follow-up.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Catatonia Associated With Another Mental Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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79. ASM-1590 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

In Lazarus and Folkman's stress and coping model, what is a person's judgment about whether they have the resources to handle a stressor called?

A. Primary appraisal
B. Secondary appraisal
C. Emotion-focused coping

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Primary appraisal is the person's judgment of what a situation means for them: whether it is irrelevant, harmless, or a threat, harm or challenge. It comes before deciding whether one can cope.

B **Correct.** In Lazarus and Folkman's transactional model, secondary appraisal is the person's judgment of what can be done and whether their resources and options are enough to manage a stressor (Lazarus & Folkman, 1984). Assessing it shows how capable a client feels of coping. It beats A because primary appraisal concerns whether the event is a threat, harm or challenge, not whether the person can handle it.

C **Incorrect** (trap: Wrong model or concept). Emotion-focused coping is a strategy for managing distress, such as seeking comfort or reframing. It is something a person does after appraisal, not the judgment about available resources.

Takeaway: Primary appraisal asks whether a stressor threatens me; secondary appraisal asks whether I have the resources to cope.

Sources: Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, cognitive appraisal: primary and secondary appraisal
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.5 Assessing resilience and coping capacity · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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80. ETH-1050 (rev 2) · Best answer: A

At intake in an outpatient clinic, a client discloses a suicide attempt 8 years ago. The client then asks the social worker to keep it out of the record and falls silent, looking at the intake form. What should the social worker do **FIRST**?

A. Ask what worries the client about having it recorded
B. Leave it out of the record and keep a private memo
C. Record the attempt and explain who can access the record
D. Describe the attempt vaguely as past difficulties

Why each option is right or wrong

A **Correct.** A request to keep something out of a record usually rests on a specific fear, such as who might see it, and the right reassurance depends on what that fear is. The client has not said why and has gone quiet, so asking comes first; the social worker can then record what care requires and explain how the record is protected (NASW Code 3.04(a), 1.07(e)). It beats C because recording the attempt and explaining access is the right next step, but doing it before learning the concern may answer the wrong worry.

B **Incorrect** (trap: Misreads law or ethics). A private memo that holds material clinical facts outside the record creates a shadow record. The official record becomes inaccurate, and the memo may still be reachable in legal proceedings. Risk history belongs in the clinical record (3.04(a)).

C **Incorrect** (trap: Right action, wrong time). Records must be accurate and include information directly relevant to service delivery (NASW Code 3.04(a), 3.04(c)), so a past suicide attempt, a major risk factor, belongs in the record, and explaining who can see it often eases the fear (1.07(e)). This is the right next step once the social worker knows what the client is worried about.

D **Incorrect** (trap: Misuses the technique). Leaving out nonessential sensitive detail is right under 3.04(c), but a past suicide attempt is essential risk information. A vague phrase misleads future providers and makes the record inaccurate.

Takeaway: Before answering a request to keep information out of the record, learn the client's concern; then record what care requires and explain access.

Sources: NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(c); NASW Code of Ethics (2021), 1.07(e)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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81. ASM-1651 (rev 1) · Best answer: A

A 9-year-old has lived with a foster family for 4 months and has weekly visits with their birth parent, who is working toward reunification. The foster parent reports that the child refuses outings and recently said, 'If I have fun here, it means I don't love my real family.' The birth parent has been cooperative with the agency. What would be **MOST** helpful?

A. Help the birth parent tell the child it is fine to have fun there
B. Tell the child the birth parent would want them to have fun
C. Refer the child for an evaluation of a possible attachment disorder

Why each option is right or wrong

A **Correct.** Children in care often feel that enjoying a new home betrays their parents. This loyalty conflict can block adjustment, and the message that frees the child carries most weight when it comes from the person the child fears hurting. Because this birth parent is cooperative, helping them tell the child directly that having fun with the foster family is fine can release the bind. Fahlberg notes that attachments transfer more readily with the cooperation of earlier parenting figures. It beats B because a secondhand message from the social worker cannot answer the child's fear about the parent's real feelings.

B **Incorrect** (trap: Good but not best). Passing along what the parent would probably want is well meant, but it puts words in the parent's mouth without checking. The child's worry is about the parent's real feelings, so a secondhand message from the social worker carries little weight.

C **Incorrect** (trap: Skips assessment). The child is describing a common loyalty bind, not a pattern such as indiscriminate friendliness or failure to seek comfort. A referral would pathologize an expected reaction to placement; evaluation fits if broader attachment problems appear.

Takeaway: Loyalty conflicts in placement ease most when birth parents themselves tell the child it is all right to enjoy the new home.

Sources: Fahlberg, V. I. (1991). A Child's Journey Through Placement. Perspectives Press, loyalty conflicts; transferring attachments with the cooperation of prior caregivers
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.13 Effects of out-of-home placement (hospital, foster, residential, justice) · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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82. INT-1053 (rev 1) · Best answer: D

A parent has returned home after a year-long military deployment. In family sessions, the partner and two teens say everyone feels distant, but they struggle to put the feeling into words. The social worker uses Satir's experiential approach. Which intervention is **MOST** consistent with this approach?

A. Prescribe that the family keep its distance this week
B. Ask circular questions about who notices the distance first
C. Build a three-generation genogram of military service
D. Have members place one another to show closeness

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Prescribing the symptom is a strategic, paradoxical move. It does not reflect Satir's warm, growth-focused stance (Satir, 1972), and it offers no experiential method to help the family reach feelings they cannot yet name.

B **Incorrect** (trap: Wrong model or concept). Circular questions come from Milan systemic therapy. They can reveal relationship patterns, but they rely on talk about differences and are not the action-based method Satir used to bring feelings into the room.

C **Incorrect** (trap: Wrong model or concept). A genogram is the signature tool of Bowen theory, used to trace patterns across generations. It is a thinking-centered assessment, not an experiential exercise for feelings the family cannot yet put into words.

D **Correct.** Family sculpting is an experiential technique associated with Satir: members physically arrange one another in space to show closeness, distance and roles. It turns feelings the family cannot yet name into something everyone can see and talk about, which fits a family that feels distant but lacks words for it (Nichols & Davis, 2020). It beats B because circular questions are a Milan tool that works through spoken descriptions of differences, the very thing this family finds hard right now.

Takeaway: Satir's experiential work uses action methods such as sculpting to make unspoken feelings and family roles visible.

Sources: Nichols, M. P., & Davis, S. D. (2020). Family Therapy: Concepts and Methods (12th ed.). Pearson, experiential family therapy: family sculpting; Satir, V. (1972). Peoplemaking. Science and Behavior Books, the therapist's warmth and focus on growth

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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83. ETH-1189 (rev 2) · Best answer: C

After a weekend continuing education workshop, a social worker is eager to try a new brief technique for trauma. The presenter supported it with case stories and client testimonials. The social worker has several clients with trauma histories and a supervisor available for consultation. Which action is **BEST** before the social worker uses the technique with clients?

A. Try it with consenting clients while getting supervision

B. Ask the presenter for more detailed case examples

C. Search for published outcome studies of the method

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Study, training, consultation and supervision belong in adopting any new technique (1.04(b)), and clients' informed consent is needed too (1.03(a)). They address how to use a method, though, not whether it works; with only testimonials, the evidence question comes first.

B **Incorrect** (trap: Misuses the technique). More case examples are the same kind of evidence the workshop already offered. Anecdotes cannot show whether improvements came from the technique, so collecting more of them does not answer the central question.

C **Correct.** Social workers should base practice on recognized knowledge, including empirically based knowledge (4.01(c)). Case stories and testimonials cannot show whether a technique works or is safe, and clients with trauma histories can be harmed by an unproven method. Checking the research is the first question; training and supervision (1.04(b)) matter once the evidence supports using the technique. It beats A because supervision helps apply a sound method well but cannot make up for missing evidence that the method works.

Takeaway: Testimonials and case stories are not evidence of effectiveness; check peer-reviewed outcome research before adopting a technique with clients.

Sources: NASW Code of Ethics (2021), 4.01(c); NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 1.03(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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84. ASM-1016 (rev 1) · Best answer: C

In the fourth session at a rural agency, a client reports losing hours at a time, finding receipts for purchases the client does not remember and being greeted by strangers using another name. The social worker has no training in dissociative disorders, and no one at the agency does. The clinical director can connect the worker with a regional expert in these conditions. What is the **MOST** appropriate action?

A. Record dissociative amnesia based on the reported gaps
B. Ask the client to describe early trauma in detail
C. Consult a specialist about how to assess the client

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Assigning a dissociative diagnosis without training risks a wrong label. Being called by another name suggests more than amnesia, and the differential needs a clinician competent in dissociative disorders.

B **Incorrect** (trap: Misuses the technique). Trauma history matters, but pressing for detailed early memories before safety and competence are in place can destabilize a client with dissociative symptoms. A trained assessor paces this carefully.

C **Correct.** Social workers practice within their competence and, in an area new to them, seek consultation or supervision from someone competent in it (NASW Code 1.04(b); 2.05(a)). Lost time, unremembered purchases and being called by another name suggest a possible dissociative disorder, such as dissociative identity disorder (DSM-5-TR), a differential the worker is not trained to make, with no in-house expertise. Consulting the regional expert supports an accurate assessment and a referral if needed. It beats B because detailed trauma probing by an untrained worker can overwhelm the client and still would not settle the diagnosis.

Takeaway: When a presentation falls outside your training and no in-house expertise exists, consult a competent specialist before diagnosing.

Sources: NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 2.05(a); DSM-5-TR (APA, 2022), Dissociative Disorders: Dissociative Identity Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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85. ETH-0101 (rev 1) · Best answer: A

A transitional housing program requires staff to discharge any resident who misses curfew 3 times. A resident has missed curfew 3 times because a new job ends after curfew. The resident has no other place to live. What should the social worker do **FIRST**?

A. Ask agency leaders whether the rule allows a work exception
B. Begin discharge planning as the written policy requires
C. Encourage the resident to look for a job with earlier hours
D. Request an ethics committee review of the curfew policy

Why each option is right or wrong

A **Correct.** When an agency policy conflicts with a client's welfare, the Code directs a responsible effort to resolve the conflict, starting with clarifying the facts and options (NASW Code, Purpose; 3.09(d)). Finding out whether the rule has an exception or appeal process is that first step, and it may solve the problem through ordinary channels. It beats D because an ethics committee review is a later step, used if leaders cannot or will not resolve the conflict. Reamer's decision-making framework likewise begins by identifying the issues and options before escalating (Reamer, 2018).

B **Incorrect** (trap: Misreads law or ethics). Discharging the resident to homelessness puts the written rule ahead of serious harm to the client without any effort to resolve the conflict. Following policy is right when the policy is ethical as applied, which is the question still open here.

C **Incorrect** (trap: Overrides the client's choice). Steering the resident toward different work asks the client to give up a job they chose in order to fit a rule. The conflict lies in the policy as applied, so the social worker addresses the rule before asking the client to change.

D **Incorrect** (trap: Right action, wrong time). Ethics committee review is a reasonable escalation if program leaders refuse an exception or the conflict cannot be resolved. Going there first skips a simpler step that may settle the issue quickly through ordinary channels.

Takeaway: When policy and client welfare conflict, first seek resolution through the agency's channels, then consult or escalate if that fails.

Sources: NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 3.09(d); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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86. ASM-1111 (rev 1) · Best answer: B

At a veterans clinic, a client describes following an order during deployment that led to civilian deaths. The client reports deep shame, says, 'I'm not a good person anymore,' and has pulled away from a faith community they once valued. Which concept **MOST** likely describes this reaction?

A. Survivor guilt
B. Moral injury
C. Secondary traumatic stress
D. Prolonged grief disorder

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Survivor guilt centers on having lived when comrades or others died. This client's shame is about their own action that caused deaths, which points elsewhere.

B **Correct.** Moral injury is the lasting shame, guilt and loss of meaning that can follow taking part in, failing to prevent, or witnessing acts that violate deeply held moral beliefs (Litz et al., 2009). The client's distress centers on their own role in civilian deaths, a damaged sense of being a good person, and withdrawal from a faith community. It beats A because survivor guilt concerns living when others died, while this client's anguish concerns what they did. Moral injury is not a DSM-5-TR diagnosis and can occur with or without PTSD.

C **Incorrect** (trap: Wrong model or concept). Secondary traumatic stress arises from hearing about or caring for others' trauma, as in helping professionals. The client was directly involved in the events.

D **Incorrect** (trap: Near-miss diagnosis). Prolonged grief disorder involves intense longing for, or preoccupation with, a person who died, lasting at least a year in adults. The client's distress is shame over a moral violation, not yearning for a loved one.

Takeaway: Shame and loss of meaning after acts that violate one's moral code suggest moral injury, distinct from fear-based trauma reactions.

Sources: Litz, B. T., Stein, N., Delaney, E., Lebowitz, L., Nash, W. P., Silva, C., & Maguen, S. (2009). Moral injury and moral repair in war veterans: A preliminary model and intervention strategy. Clinical Psychology Review, 29(8), 695-706, Definition of moral injury

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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87. ASM-1185 (rev 1) · Best answer: D

A 76-year-old client in a senior center counseling program pays part of an adult grandchild's rent each month. The client manages their own bills, which are current, and explains, 'I want to help while I'm alive to see it.' The client describes no pressure from the grandchild. The client's adult child calls the social worker, angry, insisting the payments are elder abuse that must be reported. What is the **BEST** response?

A. File an adult protective services report as the child asks

B. Ask the client to pause the payments until the family agrees

C. Share the client's finances with the adult child to ease worry

D. Support the client's choice and decline to file a report

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A report fits reasonable suspicion of exploitation, such as unpaid bills, pressure, deception or declining capacity. None is present here, so a report would act on a relative's disagreement and override a capable client's choice.

B **Incorrect** (trap: Overrides the client's choice). Asking a capable adult to stop gifts until relatives approve puts the family's wishes above the client's. It would be reasonable to revisit the payments if the client's needs began to go unmet.

C **Incorrect** (trap: Misreads law or ethics). Sharing the client's financial details with a relative needs the client's consent (NASW Code 1.07(b)). Reassuring the adult child this way would breach confidentiality, however well meant.

D **Correct.** Financial exploitation is improper or unauthorized use of an older person's resources for another's gain (42 U.S.C. 1397j(8)). A capable client who pays their own bills, gives freely and reports no pressure is making a gift, not being exploited, so self-determination governs (NASW Code 1.02). It beats A because a report needs reasonable suspicion, and nothing here suggests deception, pressure, incapacity or unmet needs. The social worker can listen to the adult child without sharing the client's information.

Takeaway: A capable older adult's voluntary gifts to family are not exploitation; report when there are signs of pressure, deception, incapacity or unmet needs.

Sources: Elder Justice Act, 42 U.S.C. 1397j(8); NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.07(b)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.15 Exploitation across the lifespan: financial, immigration-related, trafficking · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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88. ASM-1676 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

Which instrument is designed to measure how ready a client is to change their drinking?

A. SOCRATES
B. AUDIT
C. CAGE
D. CIWA-Ar

Why each option is right or wrong

A **Correct.** SOCRATES (Stages of Change Readiness and Treatment Eagerness Scale) measures motivation to change drinking. Its scales cover recognizing a problem, ambivalence about it, and steps already taken toward change (Miller & Tonigan, 1996). It tells the worker where the client stands on change, not how much they drink. It beats B, the strongest competitor, because the AUDIT measures hazardous or harmful drinking and likely dependence; a high AUDIT score says nothing about whether the client is ready to act.

B **Incorrect** (trap: Wrong model or concept). The AUDIT is a 10-item World Health Organization screen for hazardous and harmful drinking and likely dependence (Babor et al., 2001). It shows how risky the drinking is, not how ready the client is to change it.

C **Incorrect** (trap: Wrong model or concept). The CAGE is a four-question screen for problem drinking that asks about cutting down, annoyance, guilt and morning drinking (Ewing, 1984). A positive screen prompts further assessment but does not measure readiness for change.

D **Incorrect** (trap: Wrong model or concept). The CIWA-Ar rates the severity of alcohol withdrawal so medical staff can guide withdrawal treatment (Sullivan et al., 1989). It is used during withdrawal management, not to gauge motivation.

Takeaway: Screens such as the AUDIT and CAGE measure drinking risk; readiness instruments such as SOCRATES measure motivation to change.

Sources: Miller, W. R., & Tonigan, J. S. (1996). Assessing drinkers' motivation for change: The Stages of Change Readiness and Treatment Eagerness Scale (SOCRATES). Psychology of Addictive Behaviors, 10, 81-89, scales: recognition, ambivalence, taking steps; Babor, Higgins-Biddle, Saunders & Monteiro (2001). AUDIT: The Alcohol Use Disorders Identification Test, Guidelines for Use in Primary Care (2nd ed.). World Health Organization, purpose of the AUDIT; Ewing, J. A. (1984). Detecting alcoholism: The CAGE questionnaire. JAMA, 252(14), 1905-1907, whole article; Sullivan, J. T., et al. (1989). Assessment of alcohol withdrawal: The revised Clinical Institute Withdrawal Assessment for Alcohol scale (CIWA-Ar). British Journal of Addiction, 84(11), 1353-1357, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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89. ETH-1555 (rev 1) · Best answer: C

A client in an intensive outpatient program for alcohol use disorder lost Medicaid coverage 3 weeks ago. The state mailed the renewal form to an old address, and the client never received it. The client is doing well and wants to continue. Several other clients have lost coverage the same way. What should the social worker do **FIRST**?

- **A.** Propose a fix for renewal mailings to state officials
- **B.** Ask the client why the address was left outdated
- **C.** Help the client submit the renewal paperwork this week
- **D.** Discharge the client with referrals to free treatment programs

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). The pattern across several clients justifies systems advocacy, which can follow soon (NASW Code 6.04(a)). Starting there leaves this client without coverage while a slow policy process unfolds.

B **Incorrect** (trap: Good but not best). The stem already explains how coverage was lost. Asking why the address was not updated adds blame and delays help; correcting contact details is simply part of filing the renewal.

C **Correct.** When a policy or procedure cuts off care, the first response secures the client's own access, since the social worker's primary duty is the client's well-being (NASW Code 1.01). Medicaid rules require states to reconsider a person dropped for not returning a renewal form, without a new application, if the form comes in within 90 days (42 CFR 435.916). Filing now can restore coverage and keep treatment going. It beats A because the pattern across clients warrants advocacy, but a proposal to officials will not restore this client's coverage in time.

D **Incorrect** (trap: Hands off too soon). Ending treatment for a client who is doing well and wants to continue, when coverage can likely be restored, risks a needless break in care. Referral to free programs fits only if coverage cannot be restored or other payment arranged.

Takeaway: When a procedural policy cuts off a client's coverage, restore the client's access first; systems advocacy for the wider pattern follows.

Sources: NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 6.04(a); Medicaid eligibility renewal regulations, 42 CFR 435.916
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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90. ETH-1062 (rev 1) · Best answer: A

A hospital social worker's work phone breaks, and a replacement will take a week. Several discharged patients need follow-up calls about appointments. A coworker suggests using a personal cell phone in the meantime. What is the **BEST** way to handle patient contact this week?

A. Use the office landline and secure messaging system
B. Text patients from the personal phone without names
C. Use the personal phone and delete messages each day

Why each option is right or wrong

A **Correct.** Using personal devices for client contact can compromise boundaries and confidentiality, so workers rely on employer-provided systems where available (Tech Standards 2.11) and protect electronic communication with safeguards such as encryption (NASW Code 1.07(m)). The hospital already has an office line and secure messaging, so a personal phone is unnecessary. A beats B because leaving names out of texts does not stop replies, numbers and message histories from sitting on a personal device outside the hospital's safeguards.

B **Incorrect** (trap: Misuses the technique). Omitting names reduces exposure, but texts still sit on an unsecured personal device, patients gain the worker's personal number, and replies arrive outside work systems. A personal phone is a last resort, not the first choice.

C **Incorrect** (trap: Boundary slip or collusion). Giving patients a personal number blurs the professional boundary, and daily deletion does not undo that. It also erases communications that may need to be documented.

Takeaway: Use employer-provided, secure channels for client contact; personal devices blur boundaries and put confidential information at risk.

Sources: NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.11; NASW Code of Ethics (2021), 1.07(m)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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91. INT-1018 (rev 2) · Best answer: B

Through an employee assistance program, a client meets with a social worker by video. The client says they handle anger in staff meetings by clamping down and showing nothing, but later get headaches and snap at their children. Which emotion-regulation skill is **BEST** for the social worker to teach?

A. Vent the anger fully at home to release the pressure
B. Reframe triggering events before anger builds
C. Practice muscle relaxation each evening after work

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Venting seems to release pressure, but research on catharsis shows that expressing anger this way tends to keep it going (Bushman, 2002). It would also land on the children, which is already part of the problem.

B **Correct.** Emotion regulation research separates strategies used early, such as cognitive reappraisal, from strategies used once an emotion is under way, such as suppression (Gross, 1998). Reappraisal reduces felt distress, while habitual suppression hides expression but leaves inner arousal and is linked to poorer well-being and relationships (Gross & John, 2003). The client's headaches and irritability at home fit the costs of suppression. It beats C because evening relaxation eases tension afterward but leaves the suppression pattern in place.

C **Incorrect** (trap: Good but not best). Progressive muscle relaxation is a sound skill and may ease the headaches. It works after the fact, though, and leaves in place the suppression that builds the arousal and spills over at home.

Takeaway: Cognitive reappraisal changes an emotion's impact early; habitual suppression hides expression but keeps arousal and carries physical and relational costs.

Sources: Gross, J. J. (1998). The emerging field of emotion regulation: An integrative review. Review of General Psychology, 2(3), 271-299, antecedent-focused versus response-focused regulation; Gross, J. J., & John, O. P. (2003). Individual differences in two emotion regulation processes: Implications for affect, relationships, and well-being. Journal of Personality and Social Psychology, 85(2), 348-362, whole article; Bushman, B. J. (2002). Does venting anger feed or extinguish the flame? Catharsis, rumination, distraction, anger, and aggressive responding. Personality and Social Psychology Bulletin, 28(6), 724-731, whole article

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.6 Teaching emotion-regulation skills · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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92. INT-1616 (rev 1) · Best answer: B

In a large hospital, each social worker answers to two people. The head of social work oversees professional standards. A unit nurse manager sets daily assignments. Which organizational structure does this describe?

A. Flat structure
B. Matrix structure
C. Functional structure
D. Divisional structure

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A flat structure has few management layers and wide spans of control. It describes how many levels an organization has, not dual reporting lines.

B **Correct.** A matrix structure gives staff two lines of authority, typically a functional or professional department and a program or project unit (Galbraith, 2009). Here the department director oversees professional standards while the unit manager directs daily work, so each social worker answers to both. It beats C because a functional structure groups staff by discipline under a single chain of command; the second reporting line to the unit manager is what makes this a matrix.

C **Incorrect** (trap: Wrong model or concept). A functional structure groups staff by profession or function, such as a social work department, under one chain of command. The added reporting line to the unit manager goes beyond that.

D **Incorrect** (trap: Wrong model or concept). A divisional structure organizes staff into self-contained units, such as service lines, each with its own chain of command. Here professional oversight stays with a separate department.

Takeaway: A matrix structure gives staff two lines of authority, usually a professional department and a program or unit.

Sources: Galbraith, J. R. (2009). Designing Matrix Organizations That Actually Work. Jossey-Bass, Matrix structure defined: dual reporting relationships

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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93. ETH-1143 (rev 2) · Best answer: A

At a community after-school program, a 10-year-old shows the social worker a bruise on their side and says a parent's partner punched them last night. The social worker calls the child abuse hotline, and the intake worker accepts the report. What should the social worker do **NEXT**?

A. Document when, to whom and what was reported
B. Ask the child more questions to add detail for investigators
C. Call the parent to explain that a report has been made
D. Wait to document until child protective services shares findings

Why each option is right or wrong

A **Correct.** Records should be accurate, timely and sufficient for continuity of services (NASW Code 3.04(a), 3.04(b)). After a report, the worker documents when it was made, to whom, and what was reported, including the child's own words. Investigating and contacting the family belong to child protective services once a report is accepted (DePanfilis, 2018). A beats C because calling the parent could alert the partner, who is the alleged abuser, and put the child at risk before investigators act.

B **Incorrect** (trap: Skips assessment). The child's statement already supports the report, which has been made. More questioning risks leading the child and can complicate the investigation; detailed interviews belong to trained investigators.

C **Incorrect** (trap: Ignores risk). Telling a family about a report can be appropriate in some cases, but here the alleged abuser is the parent's partner. Contact with the family is for investigators to plan, so the child is not exposed to retaliation or pressure.

D **Incorrect** (trap: Right action, wrong time). Documentation should be timely, made while the details are fresh. Waiting for the agency's findings, which may take weeks or never come back to the reporter, leaves the record incomplete.

Takeaway: After making an abuse report, document when, to whom and what was reported; leave investigation and family contact to CPS.

Sources: NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b); DePanfilis, D. (2018). Child Protective Services: A Guide for Caseworkers. U.S. Department of Health and Human Services, Children's Bureau, Ch. 6: Initial Assessment or Investigation

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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94. ETH-1670 (rev 3) · Best answer: B

A group home for adults with serious mental illness supports a resident's wish to ride the city bus alone to a new job. A staff member asks what happens if the resident gets lost. The team leader says an occasional wrong turn is part of ordinary adult life. The team arranges travel training and lets the resident ride alone. Which principle is the team **MOST** likely applying?

A. Least restrictive alternative
B. Dignity of risk
C. Harm reduction
D. Beneficence

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The least restrictive alternative favors the least confining setting or intervention that meets a need, such as community care over hospitalization. It fits partly, but the team leader's stated reason is that occasional mishaps are part of adult life, not the choice of a setting.

B **Correct.** Dignity of risk holds that people with disabilities have the right to take the ordinary risks of daily life, and that shielding them from all risk undermines dignity and growth (Perske, 1972). The team weighs the resident's goal against staff worry and supports the attempt with preparation, which promotes self-determination (NASW Code 1.02). It beats A because the least restrictive alternative concerns choosing the least confining setting or intervention, while this team's stated reason is that occasional mishaps belong to an ordinary adult life.

C **Incorrect** (trap: Wrong model or concept). Harm reduction lowers the harms of a behavior a person will continue, such as drug use, without requiring them to stop. Riding a bus to work is not a harmful behavior to be reduced.

D **Incorrect** (trap: Wrong model or concept). Beneficence is the duty to act for another's good. It can justify protective limits as easily as freedom, so it does not capture the specific idea that facing everyday dangers is part of a full life.

Takeaway: Dignity of risk: people with disabilities have the right to face ordinary risks; prepare them rather than prohibit.

Sources: Perske, R. (1972). The dignity of risk and the mentally retarded. Mental Retardation, 10(1), whole article; NASW Code of Ethics (2021), 1.02
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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95. INT-1011 (rev 1) · Best answer: D

A group program was first built at a medical center for patients with chronic pain and illness. It uses 8 weekly classes of body scan, sitting meditation, and gentle yoga. What is it called?

A. Mindfulness-based cognitive therapy
B. DBT skills training group
C. Acceptance and commitment therapy
D. Mindfulness-based stress reduction

Why each option is right or wrong

- **A** **Incorrect** (trap: Wrong model or concept). Mindfulness-based cognitive therapy uses an 8-week group much like MBSR, but it was designed for people with recurrent depression and adds cognitive therapy elements aimed at preventing relapse (Segal, Williams & Teasdale, 2013).
- **B** **Incorrect** (trap: Wrong model or concept). DBT skills groups teach mindfulness as one of four modules, alongside distress tolerance, emotion regulation, and interpersonal effectiveness. They were developed for chronic self-harm, not chronic pain.
- **C** **Incorrect** (trap: Wrong model or concept). ACT builds psychological flexibility through acceptance, defusion, and values-based action. It is usually delivered as individual therapy and is not a fixed 8-week meditation and yoga curriculum.
- **D** **Correct.** Mindfulness-based stress reduction (MBSR) was developed by Jon Kabat-Zinn at the University of Massachusetts Medical Center for medical patients, including people with chronic pain (Kabat-Zinn, 1982). The 8-week group teaches the body scan, sitting meditation, and mindful movement. It beats A because mindfulness-based cognitive therapy borrows MBSR's 8-week format but was built later to prevent depressive relapse and adds cognitive therapy elements.

Takeaway: MBSR is Kabat-Zinn's 8-week group for medical patients; MBCT adapts it, adding cognitive elements, to prevent depressive relapse.

Sources: Kabat-Zinn, J. (1982). An outpatient program in behavioral medicine for chronic pain patients based on the practice of mindfulness meditation. General Hospital Psychiatry, 4(1), 33-47, whole article; Segal, Z. V., Williams, J. M. G., & Teasdale, J. D. (2013). Mindfulness-Based Cognitive Therapy for Depression (2nd ed.). Guilford Press, development of MBCT from MBSR

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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96. INT-1506 (rev 1) · Best answer: B

During the first week of an intensive outpatient program for alcohol use, a client wipes away tears while saying, 'Honestly, I'm doing great. This place is just a formality for my job.' Which response by the social worker is **MOST** helpful at this point?

- **A.** Point out the mismatch between the tears and the words
- **B.** Gently reflect the sadness that shows as the client speaks
- **C.** Accept the statement and continue with the intake questions
- **D.** Suggest that the tears may come from grief the client avoids

Why each option is right or wrong

- **A** **Incorrect** (trap: Right action, wrong time). Pointing out a discrepancy is a valid technique, but it belongs later, once trust is established. In the first week, it is likely to be heard as calling the client dishonest and to increase discord.
- **B** **Correct.** Early in a relationship, a gap between words and feelings is met with empathic reflection, not confrontation. Naming the sadness gently shows the client that the worker notices and accepts the feeling without challenging the words. It beats A because confronting a discrepancy works only after rapport exists; done in the first week, it can feel like an accusation and raise defensiveness. Reflection builds the alliance that later confrontation needs (Hepworth et al.; Miller & Rollnick, 2023).

C **Incorrect** (trap: Good but not best). Moving on respects the client's words but ignores the visible feeling and misses a chance to show understanding. Noticing and reflecting the emotion builds rapport more effectively.

D **Incorrect** (trap: Too directive). Offering a hypothesis about hidden grief is an interpretation, the riskiest of the interviewing techniques, and it needs a strong alliance. Offered in the first week, it is likely to feel presumptuous.

Takeaway: Early on, meet a gap between a client's words and feelings with gentle reflection; save confrontation and interpretation until rapport forms.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Empathic communication; confrontation; Miller & Rollnick (2023). Motivational Interviewing (4th ed.), Engaging: reflective listening
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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97. ASM-0008 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

Which part of the mental status examination records answers that wander off and never reach the point?

A. Thought content
B. Perception
C. Thought process

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Thought content records the themes of thinking, such as delusions, preoccupations, obsessions or suicidal ideation. The question describes how ideas connect, not what they are about.

B **Incorrect** (trap: Wrong model or concept). Perception covers sensory experiences such as hallucinations and illusions. Wandering, loosely connected answers are a feature of thinking rather than perception.

C **Correct.** Thought process describes how thoughts are organized and connected: goal-directed, circumstantial, tangential, or loosely associated (Kaplan & Sadock's Synopsis). Answers that wander through loosely related details and never reach the point describe tangentiality, a process finding. It beats A because thought content covers what the client thinks about, such as delusions, obsessions or suicidal ideas, rather than how ideas link together.

Takeaway: MSE: thought process is how ideas connect (tangential, loose); thought content is what the client thinks about.

Sources: Boland, R., Verduin, M. L., & Ruiz, P. (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.). Wolters Kluwer, mental status examination: thought process
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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98. ASM-1550 (rev 2) · Best answer: D

A client with bipolar disorder attends a supported employment program and receives Social Security disability benefits and Medicaid. The employer offers the client more weekly hours. The client says, 'I want the hours, but if I earn more I could lose my check and my medication coverage.' Unsure how earnings affect either benefit, the client asks the social worker what to do. What is the **BEST** response?

A. Encourage the client to accept the hours as a recovery step
B. Explain that the added earnings will end the client's benefits
C. Explore what the extra hours would mean to the client
D. Refer the client to a work incentives benefits planner

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Work can support recovery, but urging the client to accept brushes past a real financial risk the client raised. Encouragement fits once the client has accurate information about how earnings affect benefits and chooses to go ahead.

B **Incorrect** (trap: Outside the social work role). Work incentive rules often let beneficiaries earn some income while keeping benefits or health coverage, so this statement may be wrong. Predicting benefit outcomes requires specialized knowledge the social worker does not have.

C **Incorrect** (trap: Good but not best). Exploring meaning helps when a client is unsure what they want. This client already wants the hours and has named a specific, factual barrier, so exploration alone leaves the real obstacle in place.

D **Correct.** Fear of losing cash benefits and health coverage is a common poverty trap that shapes work decisions. How earnings affect Social Security benefits and Medicaid is complex and individual, and free work incentives counseling is available to Social Security disability beneficiaries ages 14 to full retirement age (SSA, Ticket to Work). Referring when another professional's specialized knowledge is needed follows NASW Code 1.16. It beats C because the client has already stated both the wish and the fear; what is missing is accurate benefits information, not more exploration.

Takeaway: When benefit rules tied to poverty shape a client's work decisions, connect the client to specialized benefits counseling before the decision is made.

Sources: Social Security Administration (SSA), Ticket to Work (2025). Benefits Counseling and the Path to Employment, Work Incentives Planning and Assistance (WIPA) fact sheet; NASW Code of Ethics (2021), 1.16(a) Referral for Services

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.9 Poverty's impact; social determinants of health · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

99. ASM-1103 (rev 1) · Best answer: A

At intake for a court-ordered partner violence program, a participant says, 'I lose my temper and can't stop myself.' The participant describes staying calm when a manager yells at them in front of coworkers. Twice, the participant smashed the partner's phone after the partner went out with friends. Which explanation is **MOST** likely supported by this pattern?

A. Purposeful control of the partner
B. Poor impulse control across settings
C. Reaction to the partner's provocations
D. Job stress displaced onto home

Why each option is right or wrong

A **Correct.** The Duluth model holds that partner violence is mainly a tactic of power and control, not a loss of control (Pence & Paymar, 1993). The decisive detail is selectivity: the participant stays calm when a manager humiliates them in public but destroys the partner's phone after the partner sees friends, an act that limits the partner's contacts. It beats B because a true impulse-control problem would show up across settings, including under workplace provocation. Claims of 'losing it' are a common way people who abuse partners minimize choice.

B **Incorrect** (trap: Wrong model or concept). Some people do have broad anger problems. This participant, though, holds back with a manager who yells in public and targets only the partner, which points away from a general impulse-control deficit.

C **Incorrect** (trap: Wrong model or concept). Blaming the partner's behavior is a common account given by people who abuse, not an explanation the facts support. Going out with friends is ordinary; smashing the phone in response is an effort to restrict the partner.

D **Incorrect** (trap: Wrong model or concept). Displacement would predict anger spilling onto whoever is near after hard days at work. Here the violence follows the partner's independence, which fits control of the partner rather than displaced job stress.

Takeaway: Violence aimed selectively at a partner, with restraint elsewhere, signals purposeful control rather than an anger or impulse problem.

Sources: Pence, E., & Paymar, M. (1993). Education Groups for Men Who Batter: The Duluth Model. Springer, Power and control wheel

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.12 Traits of perpetrators of abuse, neglect, exploitation · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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100. INT-1186 (rev 1) · Best answer: C

On an inpatient psychiatric unit, policy requires a body check for injuries and contraband at admission. A newly admitted adult with a history of childhood sexual abuse starts shaking and crying when told about the check and backs into a corner. The nurse asks the social worker for help. What is the **MOST** appropriate action?

A. Ask the nurse to skip the check for this patient
B. Explain the safety reasons for the check, then proceed
C. Plan with the patient and nurse how the check will be done

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Waiving the check may calm the moment but leaves possible injuries or dangerous items undetected on a unit where other patients and staff could be harmed. Trauma-informed care adapts how safety procedures happen; it does not drop them.

B **Incorrect** (trap: Good but not best). Transparency is a trauma-informed principle and belongs in the response. On its own, though, it still imposes the procedure on a frightened survivor without any say, which risks re-traumatization.

C **Correct.** A trauma-informed approach keeps needed safety procedures while giving the person as much voice and choice as possible, in line with SAMHSA's principles of safety, collaboration and empowerment (SAMHSA, 2014). For a survivor of sexual abuse, a body check can echo the original violation, so involving the patient in how it is done, such as who conducts it, the order of steps, staying partly clothed and pausing when needed, lowers the risk of re-traumatization. It beats B because explanation alone is transparent but leaves the patient powerless in a procedure that resembles past abuse.

Takeaway: Trauma-informed care adapts required safety procedures with the person's input instead of dropping them or imposing them unchanged.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles; resisting re-traumatization
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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101. INT-1526 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

In a telehealth employee assistance session, a client says the main goal is for their partner to stop drinking. The assessment shows the client is losing sleep and phoning the partner's employer to excuse missed shifts. The social worker and client are now setting goals. Which goal is **MOST** appropriate?

A. Client will let the partner handle their own work absences for 4 weeks
B. Partner will attend a mutual-help meeting each week by the second month
C. Client will get the partner to agree to an assessment by week 4
D. Client will accept that the partner's drinking is outside their control

Why each option is right or wrong

A **Correct.** Goals should target changes the client can make, be stated in observable terms and include a time frame (Hepworth et al.). The client cannot control the partner's drinking but can stop managing its consequences. Letting the partner handle their own work absences for 4 weeks is observable, feasible and within the client's power. It beats C, which sounds like the client's action but succeeds only if the partner agrees, so the client could do everything right and still fail the goal.

B **Incorrect** (trap: Misuses the technique). This is measurable, but it describes the partner's behavior. The partner is not the client and has agreed to no goal, so the client has no way to make it happen.

C **Incorrect** (trap: Misuses the technique). This is phrased as the client's action, but success depends on the partner's choice. A goal the client cannot reach alone sets up failure. Inviting an assessment could still be one step within a plan.

D **Incorrect** (trap: Good but not best). Accepting the limits of control is a worthy aim, but it names an inner state with no observable behavior or time frame, so progress cannot be tracked. It works better as a theme behind a measurable goal.

Takeaway: Set goals on changes within the client's own control; a goal that depends on another person's behavior is not feasible.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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102. ETH-1705 (rev 1) · Best answer: C

At a county behavioral health clinic, a staff member made a mocking comment about a colleague's tribal ceremony during a team meeting. The staff member has admitted it and wants to make amends, and the colleague has agreed to a restorative circle. The clinic asks the social worker, a trained facilitator, to lead it. What should the social worker do **FIRST**?

A. Convene the circle and open with shared ground rules

B. Ask the staff member to write an apology to the colleague

C. Meet separately with each person to prepare

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Shared agreements are a standard opening for a circle. Setting them before meeting each party alone skips the step that tests readiness and lets the harmed colleague shape the process.

B **Incorrect** (trap: Right action, wrong time). A written apology may become part of the repair, but deciding its form before hearing from the colleague puts the outcome ahead of the harmed person's needs. Agreements should come out of the restorative process.

C **Correct.** Restorative processes depend on preparation: the facilitator meets each party separately before any joint meeting to hear their experience, explain the process, check readiness and confirm that participation is voluntary (Office for Victims of Crime, 2000; Zehr, 2015). The colleague harmed by a remark about a sacred practice needs a chance to say what repair would mean before facing the staff member. It beats A because ground rules matter once people are in the room, but opening a circle without preparation risks re-harming the colleague.

Takeaway: Before any restorative circle, meet separately with the harmed person and the person responsible to prepare, check readiness and confirm voluntary participation.

Sources: Office for Victims of Crime (2000). The Restorative Justice and Mediation Collection: Executive Summary (OVC Bulletin), Preparation: separate in-person meetings with each party; Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Principles: harms and needs; voluntary participation

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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103. INT-1545 (rev 2) · Best answer: C

A school social worker learns that the front office turned away 3 families staying in a motel this semester because they could not show proof of a home address. Each family enrolled only after the social worker stepped in. Which action is **BEST**?

A. Keep helping each family enroll as new cases come up
B. Give front-office staff a written copy of the enrollment rules
C. Raise the enrollment practice with the district's homeless liaison

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Case advocacy helped each family, but a repeated pattern signals a practice problem. Continuing one rescue at a time leaves future families facing the same barrier and delay.

B **Incorrect** (trap: Good but not best). Sharing the rules with front-office staff is a reasonable instinct and may help. It relies on an informal correction, though; the district liaison is the person federal law charges with ensuring immediate enrollment, and can change the practice and train staff across schools.

C **Correct.** When the same barrier harms several clients, the worker adds cause advocacy, seeking a change in policy or practice, to case-by-case help (Hepworth et al.). Federal law requires schools to enroll children experiencing homelessness immediately, even without records normally required, and each district must name a liaison to ensure this (McKinney-Vento Act, 42 U.S.C. 11432(g)). Three turn-aways show a front-office practice problem, so raising it with the liaison targets the cause. It beats A because one rescue at a time leaves the practice unchanged for the next family.

Takeaway: When one barrier keeps harming several clients, add cause advocacy through the proper channel to case-by-case help.

Sources: McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11432, 42 U.S.C. 11432(g)(3)(C) and (g)(1)(J)(ii); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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104. ASM-1641 (rev 1) · Best answer: A

Parents who have decided to divorce after years of frequent shouting matches ask the social worker how to protect their children, ages 7 and 11. Which step would be **MOST** helpful to the children's long-term adjustment?

A. Help the parents keep the children out of their disputes
B. Help the parents keep the children in one home and school
C. Arrange individual therapy for each child right away

Why each option is right or wrong

A **Correct.** Children's adjustment after divorce depends heavily on how much conflict between parents they are exposed to. In one long-term study, children from high-conflict marriages that ended did nearly as well as peers from intact families, since the divorce ended their exposure. After years of shouting, the most protective step is a plan that keeps the children out of the parents' disputes. It beats B because a stable home and school help, but continued exposure to conflict would undercut those benefits (Amato, Loomis & Booth, 1995).

B **Incorrect** (trap: Good but not best). Keeping the same home and school offers continuity and is worth doing when possible. It protects less than reducing exposure to parental conflict, which keeps harming children whether or not their surroundings stay the same.

C **Incorrect** (trap: Hands off too soon). Individual therapy may help a child who shows distress after the divorce. Starting it for both children before any sign of need does less to protect them than changing the conflict they are exposed to.

Takeaway: After divorce, limiting children's exposure to conflict between parents protects their adjustment more than any single change in their surroundings.

Sources: Amato, P. R., Loomis, L. S., & Booth, A. (1995). Parental divorce, marital conflict, and offspring well-being during early adulthood. Social Forces, 73(3), 895-915, whole article

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.2 Family functioning and dynamics and their effects · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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105. INT-1184 (rev 2) · Best answer: A

A client in outpatient counseling uses methamphetamine a few times a month and says they are not interested in stopping. The client carries condoms but says they rarely use them during sex while high, then asks, 'Is there anything that can lower my chances of getting HIV?' What is the **BEST** response?

A. Link the client to a clinic offering preventive HIV medication

B. Explore the client's readiness to cut back on methamphetamine

C. Provide condoms and go over safer-sex steps that lower HIV risk

Why each option is right or wrong

A **Correct.** Harm reduction meets people where they are and responds to the risk they name (SAMHSA Harm Reduction Framework). This client has declined to stop using, carries condoms but rarely uses them during sex while high, and asks directly about HIV prevention. CDC guidance supports offering pre-exposure prophylaxis (PrEP) to sexually active adults who have condomless sex, and linking the client to a prescriber is the most effective step within the social work role (CDC, 2021). It beats C because the client already has condoms and says they rarely get used while high, so relying on them alone leaves the main risk in place.

B **Incorrect** (trap: Good but not best). Exploring readiness for change fits when a client is weighing their use. This client has stated no plan to stop and asked a specific prevention question; steering back to the drug use sidesteps that request and can weaken trust.

C **Incorrect** (trap: Good but not best). Condoms and safer-sex teaching are reasonable additions, and the worker can offer them. Used alone, they depend on the very behavior the client says breaks down while high, so they protect less than linkage to medication that works without it.

Takeaway: Harm reduction answers the risk a client names; link clients with ongoing sexual HIV risk to PrEP care, whether or not they stop using.

Sources: CDC (2021). Preexposure Prophylaxis for the Prevention of HIV Infection in the United States: 2021 Update. A Clinical Practice Guideline, Identifying indications for PrEP: sexually active adults; SAMHSA (2023) Harm Reduction Framework, Pillar 6: any positive change, as defined by the person; principle: respect autonomy (meet people where they are)

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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106. ETH-1700 (rev 1) · Best answer: C

A 19-year-old college student tells a campus social worker, 'I'm probably gay, though part of me wishes I weren't.' The student has started going to a campus LGBTQ meeting, mainly to feel less alone, and finds some members welcoming and others distant. The student has told no one else. Using Cass's model, which stage **MOST** likely fits this student?

A. Identity comparison
B. Identity acceptance
C. Identity tolerance
D. Identity pride

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). In identity comparison the person first considers that they might be gay, often feels alienated, and may keep a heterosexual image or treat the feelings as temporary. This student has moved past 'might' to 'probably' and is already seeking contact with other LGBTQ people.

B **Incorrect** (trap: Wrong model or concept). Identity acceptance brings a clear 'I am gay,' more positive feelings, growing contact with the community and selective disclosure. This student still voices ambivalence and has told no one, so full acceptance has not yet arrived.

C **Correct.** Cass (1979) described identity tolerance as the stage when a person concludes they are probably gay and seeks out other gay people, largely to ease isolation, while still holding mixed feelings about the identity. The student's 'probably,' the wish that it were otherwise, joining a group mainly to feel less alone and telling no one else all fit. It beats B because acceptance involves a settled 'I am' and some disclosure, which this student has not reached. Stage models describe common patterns, and individuals may not move through them in order.

D **Incorrect** (trap: Wrong model or concept). Identity pride involves strong immersion in LGBTQ community, wide disclosure and often anger toward a heterosexual world. This student is still uncertain and private, the opposite of that stage's open, us-versus-them stance.

Takeaway: In Cass's model, identity tolerance means concluding one is probably gay and seeking LGBTQ contact to ease isolation, while ambivalence remains.

Sources: Cass, V. C. (1979). Homosexual identity formation: A theoretical model. Journal of Homosexuality, 4(3), 219-235, stages 2-5: comparison, tolerance, acceptance, pride
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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107. INT-1156 (rev 2) · Best answer: B

On the morning their home was condemned, a client arrives at a community crisis center too shaken to speak. An hour later, the client is calm, has listed what must happen today, and mentions usually handling the family's bills and appointments. The client then asks the social worker to 'just make all the calls.' What is the **BEST** response?

A. Make the calls for the client while the client rests
B. Plan which calls the client will make, with help as needed
C. Refer the client to a housing agency for case management

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Taking over tasks fits a client who cannot function, as this client could not an hour ago. Now that the client is calm and capable, doing everything undermines the coping that crisis work aims to restore.

B **Correct.** In crisis work, directiveness matches functioning: directive while a client is immobilized, more collaborative as coping returns (James & Gilliland, 2017). Building and carrying out an action plan with the client restores a sense of mastery (Roberts & Ottens, 2005). This client was shaken an hour ago but is now calm, has listed the day's tasks, and usually manages the family's affairs. Planning calls the client makes, with the social worker stepping in where needed, builds on those strengths. It beats A because taking over now treats a recovering client as helpless and can deepen dependence.

C **Incorrect** (trap: Hands off too soon). Housing case management may help later. Handing off now, when the client needs a plan for today and is ready to act, moves the work away from the crisis contact without addressing the immediate tasks.

Takeaway: As a person in crisis regains composure, hand tasks back through a shared action plan instead of doing things for them.

Sources: Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 6: developing and formulating an action plan; James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage, assessing client mobility; directive, collaborative and nondirective action continuum
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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108. ASM-1177 (rev 1) · Best answer: A

In an afternoon session, a 58-year-old client with depression tells the social worker they have no thoughts of suicide. That evening, the client's partner, who is named on a signed release, calls to say they found a large supply of sleeping pills hidden in a garage toolbox. The partner adds that the client has been unusually quiet since losing a job last month. What should the social worker do **NEXT**?

A. Contact the client promptly about the partner's call and reassess
B. Ask police to bring the client in for an emergency evaluation
C. Add a session later this week to discuss the partner's call

Why each option is right or wrong

A **Correct.** Collateral information is part of suicide inquiry, and hidden pills suggest preparatory behavior that the earlier denial did not address (SAMHSA SAFE-T, 2009; Posner et al., 2011). The social worker should contact the client promptly, share the partner's report, ask directly about intent and plans for the pills, and set the response by the risk found. It beats B because the client has not yet been asked about the pills; an emergency evaluation becomes the step if the client confirms intent, cannot be reached, or cannot keep safe.

B **Incorrect** (trap: Skips assessment). An emergency evaluation is right when assessment shows intent or imminent danger, or when an at-risk client cannot be reached. Here the client has not yet been asked about the pills, so sending police first skips the reassessment that would show whether this level of response is needed.

C **Incorrect** (trap: Ignores risk). Stockpiled pills after a recent job loss are a warning sign that calls for same-day follow-up. Waiting several days leaves possible preparatory behavior unassessed while the client still has access to the means.

Takeaway: Collateral evidence of preparatory behavior overrides a prior denial: reassess the client promptly before choosing the level of response.

Sources: SAMHSA (2009). SAFE-T: Suicide Assessment Five-step Evaluation and Triage (SMA09-4432), Step 3: Suicide inquiry; Posner, K., et al. (2011). The Columbia-Suicide Severity Rating Scale. American Journal of Psychiatry, 168(12), 1266-1277, suicidal behavior: preparatory acts
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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109. INT-1628 (rev 2) · Best answer: C

On an inpatient psychiatric unit, a newly admitted client with schizophrenia stands near the door, refuses to sit, and says staff record thoughts through the ceiling lights. The social worker is meeting the client for the first time to begin a working relationship. What should the social worker do **FIRST**?

A. Explain calmly that the lights cannot record thoughts
B. Ask what the client has noticed about the ceiling lights
C. Introduce self briefly and let the client choose the distance

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Disputing a fixed belief at the first meeting usually deepens mistrust. Gentle, collaborative reality testing can come later, once the client feels safe with the social worker.

B **Incorrect** (trap: Right action, wrong time). Curious, non-judgmental questions about a belief are part of later collaborative work, so this is a tempting move. At first contact with a frightened client standing by the door, though, opening with the feared topic adds pressure; a brief introduction and control over distance come first.

C **Correct.** With a frightened, suspicious client, engagement starts with predictability and control: a short, clear introduction, respect for physical distance, and no pressure to discuss the belief. Kingdon and Turkington (2005) stress building trust before examining delusional beliefs, and standing by the door signals fear. It beats A because arguing against the belief at first contact, however calmly, tends to harden it and casts the social worker as one more person who does not believe the client. Respectful contact can begin now, without waiting.

Takeaway: With a suspicious client, begin with brief, predictable contact that gives the client control, and avoid disputing the belief before trust exists.

Sources: Kingdon, D. G., & Turkington, D. (2005). Cognitive Therapy of Schizophrenia. Guilford Press, Engagement and the therapeutic relationship

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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110. INT-1619 (rev 2) · Best answer: C

Which budgeting system calculates cost per outcome achieved, such as the cost for each client who stays housed for a full year?

A. Line-item budgeting
B. Functional budgeting
C. Program budgeting

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A line-item budget lists expenses such as salaries, rent and supplies. It controls spending but does not link costs to services delivered or outcomes achieved.

B **Incorrect** (trap: Wrong model or concept). In Kettner, Moroney and Martin's framework, functional budgeting yields cost per unit of service or per output, such as cost per counseling hour or per client completing a program, not cost per outcome such as stable housing. In nonprofit accounting, 'functional' instead means sorting expenses into program, management and fundraising; neither use gives cost per outcome.

C **Correct.** Kettner, Moroney and Martin (2022) describe three linked systems. Line-item budgeting lists expenses by category for financial control. Functional budgeting adds total program cost and cost per unit of service or output for management. Program budgeting goes further, to cost per outcome, such as each client who remains housed, for planning. It beats B because functional budgeting stops at outputs, such as cost per case management hour, rather than results achieved.

Takeaway: Line-item budgets list expenses; functional budgets give cost per unit of service or output; program budgets give cost per outcome.

Sources: Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 12-13: line-item, functional and program budgeting
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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111. ETH-1607 (rev 3) · Best answer: A

A Deaf adult who grew up in another country begins services at a community mental health center. A qualified ASL interpreter attends the intake but reports that the client uses many home signs and signs from another country, and much is being missed. Twenty minutes of slower pacing and simpler questions have not closed the gap. What should the social worker do **NEXT**?

A. Add a Certified Deaf Interpreter to the team
B. Replace the interpreter with written notes in English
C. Invite a family member who knows the client's signs
D. Refer the client to a provider who signs fluently

Why each option is right or wrong

A **Correct.** A Certified Deaf Interpreter is a Deaf interpreter who teams with a hearing ASL interpreter when a client uses home signs, a foreign sign language or limited language, bringing native sign fluency and cultural knowledge (RID, 1997). The hearing interpreter has reported exactly these barriers, and simpler questions have not closed the gap. Effective communication is required (28 CFR 36.303(c)(1)). It beats D because a provider fluent in ASL would face the same nonstandard signs, and the client would lose an otherwise suitable service.

B **Incorrect** (trap: Misuses the technique). Written English depends on literacy in a second language, and many Deaf people, especially those schooled abroad, do not read English well enough for a clinical interview. Notes may suit a brief exchange, not an assessment.

C **Incorrect** (trap: Misreads law or ethics). A relative may understand the home signs, but family interpreters can filter content and compromise privacy, and the rules bar relying on an accompanying adult except in an emergency or at the client's own request (28 CFR 36.303(c)(3)). A qualified team can be arranged.

D **Incorrect** (trap: Hands off too soon). A signing provider can be ideal for many Deaf clients, but fluency in ASL does not solve home signs or a foreign sign language. Adding the right interpreter keeps the client in the chosen service.

Takeaway: When a Deaf client's signing is nonstandard or limited, team a Certified Deaf Interpreter with the ASL interpreter.

Sources: Registry of Interpreters for the Deaf (1997). Use of a Certified Deaf Interpreter (Standard Practice Paper), whole paper; ADA Title III regulations, 28 CFR 36.303(c)(1); ADA Title III regulations, 28 CFR 36.303(c)(3)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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112. ETH-1709 (rev 1) · Best answer: C

A residential substance use program serves several Native clients. A non-Native staff member began leading smudging and a 'spirit animal' exercise for all residents after watching online videos. Native residents tell the director the activities mock their ceremonies, though two of them want traditional healing as part of treatment. The director stops the activities and asks the social worker for advice. What should the social worker recommend **FIRST**?

A. Ban Native cultural practices from the program
B. Ask the Native residents to lead smudging for the whole group
C. Seek guidance from local tribal leaders on these practices
D. Have staff complete training on Native spiritual traditions

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). A ban ends the harmful activity but also denies the residents who asked for traditional healing. Respect for culture means offering these practices properly, not removing them.

B **Incorrect** (trap: Boundary slip or collusion). Native residents are experts on their own experience, but leading ceremonies for the program shifts staff work onto clients and may not match who is authorized to lead a ceremony. The program owes the community this effort, not the residents.

C **Correct.** Cultural humility treats a community as the authority on its own practices and holds institutions accountable for culturally responsive services (NASW Code 1.05(c)). Ceremonies taken up by outsiders without permission repeat a long pattern of appropriation that the residents named as harmful. Asking local tribal leaders how, and by whom, traditional healing can be offered respects that authority and lets the program meet the two residents' requests through community resources (NASW Cultural Competence Standard 5). It beats D because staff training is a useful later step, but its content and trainers should come from the community.

D **Incorrect** (trap: Right action, wrong time). Training can help staff understand Native traditions and avoid repeating the harm. Choosing a training before speaking with tribal leaders risks more outsider-designed content; it fits after consultation, ideally led or approved by the community.

Takeaway: When a program misuses another culture's ceremonies, consult that community's cultural authorities before offering or redesigning culturally based practices.

Sources: NASW Code of Ethics (2021), 1.05(c); NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 5: Service Delivery
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

113. ETH-1102 (rev 2) · Best answer: A

A social worker learns that a case manager at the same agency, who has a bachelor's degree in psychology and no license, sees private psychotherapy clients on weekends and advertises as a 'clinical social worker.' When the social worker raises the concern privately, the case manager says the title is harmless and declines to change the ads. What should the social worker do **NEXT**?

A. Report the case manager to the state licensing board
B. File a complaint about the case manager with NASW
C. Tell the agency director about the case manager's ads

Why each option is right or wrong

A **Correct.** Social workers should act to prevent unauthorized and unqualified practice of social work (NASW Code 5.01(e)). When a direct conversation does not resolve a colleague's unethical conduct, the Code points to formal channels such as a state licensing board (2.10(c), 2.10(d)). Licensing boards enforce practice acts, including who may use protected titles and offer licensed services, so the board has authority over this conduct. A beats C because the weekend practice happens outside the agency, and the director cannot enforce the practice act there; the board can.

B **Incorrect** (trap: Misreads law or ethics). NASW adjudicates ethics complaints against its members, and a person who is not a social worker falls outside that process. Enforcement of protected titles and licensed practice belongs to the state.

C **Incorrect** (trap: Good but not best). The director would need to know if agency clients or resources were involved, but the stem describes private weekend practice. The agency has no authority over practice outside the job, while the licensing board enforces the practice act.

Takeaway: Unlicensed use of a protected social work title goes to the state licensing board, which enforces practice acts.

Sources: NASW Code of Ethics (2021), 5.01(e); NASW Code of Ethics (2021), 2.10(c); NASW Code of Ethics (2021), 2.10(d)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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114. ASM-1503 (rev 1) · Best answer: C

In a school-based substance use group, a 16-year-old says two classmates crashed after drinking because 'they're bad drivers.' The teen adds that driving after a few beers is fine for them because 'nothing like that is going to happen to me.' Which developmental concept **MOST** likely explains this belief?

A. Imaginary audience
B. Identity diffusion
C. Personal fable

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The imaginary audience is the adolescent belief that others are always watching and evaluating, which drives self-consciousness and concern about appearance. This teen's statement is about being exempt from danger, not about being observed.

B **Incorrect** (trap: Wrong model or concept). Identity diffusion is the identity status of a young person who has neither explored options nor made commitments. It describes a lack of direction in identity, not a belief that harmful consequences will skip oneself.

C **Correct.** Elkind's personal fable is the adolescent belief that one's experiences are unique and that risks which harm others will pass oneself by (Elkind, 1967). This teen accepts that drinking and driving hurt classmates but insists the same outcome will not happen to them, the core of the fable and a known driver of adolescent risk-taking. It beats A because the imaginary audience, also from Elkind, is the sense of being constantly watched and judged, which explains self-consciousness rather than feeling exempt from danger.

Takeaway: The personal fable is adolescents' sense of being unique and exempt from risks that harm others; the imaginary audience is feeling constantly watched.

Sources: Elkind, D. (1967). Egocentrism in adolescence. Child Development, 38(4), 1025-1034, personal fable and imaginary audience

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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115. ASM-1077 (rev 1) · Best answer: A

A 29-year-old client with bipolar I disorder has been stable for 2 years on valproate. At a community clinic visit, the client happily shares a plan with their partner to start trying for a baby next month. The client's next psychiatry appointment is in 4 months. What is the social worker's **BEST** response?

A. Encourage seeing the prescriber before trying to conceive

B. Suggest tapering off valproate before trying to conceive

C. Explore the client's hopes and worries about parenting

Why each option is right or wrong

A **Correct.** Valproate carries a high risk of birth defects, including neural tube defects, and of later cognitive problems in exposed children; ACOG advises avoiding it in pregnancy and recommends planning medication decisions before conception (ACOG, 2023). Exposure happens early, often before pregnancy is known, so the prescriber needs to be involved now rather than in 4 months. Encouraging an earlier visit supports the client's goal instead of questioning it. It beats C because exploring parenting hopes is worthwhile but leaves the medication risk unaddressed while conception is a month away.

B **Incorrect** (trap: Outside the social work role). Stopping or tapering a mood stabilizer is a prescriber's decision. Abrupt changes can trigger relapse of bipolar disorder, and the safest alternative depends on the client's history. The social worker can raise the concern but should not advise a medication change.

C **Incorrect** (trap: Good but not best). Talking about parenting hopes respects the client and can continue. Alone, it misses the urgent issue: a teratogenic medication and a conception plan that comes before the next psychiatry visit.

Takeaway: When a client on a teratogenic medication plans pregnancy, support the goal and link them to the prescriber before conception.

Sources: American College of Obstetricians and Gynecologists (2023). Clinical Practice Guideline No. 5: Treatment and Management of Mental Health Conditions During Pregnancy and Postpartum, Bipolar disorder: valproate; prepregnancy consultation
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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116. INT-1587 (rev 1) · Best answer: A

A workforce program for young adults is writing objectives for a new grant. The team needs one objective that states the change participants should achieve. Which objective is **BEST**?

- **A.** By month 12, 60% of participants will keep a job for 90 days
- **B.** Staff will hold 40 job-readiness workshops during the grant year
- **C.** Participants will gain the confidence they need to succeed at work
- **D.** The program will help young adults become self-sufficient

Why each option is right or wrong

A **Correct.** An outcome objective states a measurable change in participants, with a target and a deadline, following the SMART guide: specific, measurable, achievable, relevant and time-bound (Doran, 1981; Kettner, Moroney & Martin, 2022). This option names who, what change, how much and by when. It beats B because B is a process objective: it counts staff activity, an output, rather than a change in participants.

B **Incorrect** (trap: Wrong model or concept). This is a well-written process objective, specific and time-bound, but it counts workshops delivered by staff. It describes program activity, not the change participants should achieve.

C **Incorrect** (trap: Misuses the technique). Confidence matters, but this statement names no measure, target or deadline, so no one could tell whether it was met. It could be rewritten with a confidence scale and a target date.

D **Incorrect** (trap: Wrong model or concept). This is a goal: a broad statement of intended direction. Goals guide a program, but objectives must state measurable results that show progress toward the goal.

Takeaway: A well-written outcome objective names the participant change, how much, how it is measured and by when.

Sources: Doran, G. T. (1981). There's a S.M.A.R.T. way to write management's goals and objectives. Management Review, 70(11), 35-36, whole article; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7: Organizational Mission and Program Goals, Objectives, and Activities
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.2 Setting program objectives and outcomes · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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117. ETH-1008 (rev 1) · Best answer: D

At the start of couples work by video, a social worker explained a policy that information shared privately by one partner may be raised in joint sessions. Before the fourth session, one partner emails the social worker about a recent affair and asks that it stay secret. What is the **BEST** response?

A. Keep the information private to protect the alliance
B. Disclose the affair to the other partner in the next session
C. End joint work and refer each partner for individual help
D. Remind the partner of the agreed rule and urge disclosure

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Keeping the secret breaks the policy both partners accepted and makes the social worker an ally of one partner. It would be appropriate only if a different policy, such as keeping individual disclosures private, had been agreed at the start.

B **Incorrect** (trap: Right action, wrong time). The policy allows the information to come out, but announcing it without warning skips the step of letting the partner disclose it. The worker should first remind the partner of the policy and encourage self-disclosure.

C **Incorrect** (trap: Hands off too soon). Ending couples work hands off a situation the policy was designed to handle. Referral might be considered later if the couple's goals change, but the agreed policy already gives a path forward.

D **Correct.** When work involves more than one person, the social worker tells clients at the outset how information shared among participants will be handled (NASW Code 1.07(g)). Having set a policy that private disclosures may be raised jointly, the worker applies it. The fair first move is to remind the partner of the policy and encourage them to disclose in session. D beats B because disclosure by the worker removes the partner's chance to share it themselves; the worker raises it only if the partner declines.

Takeaway: Apply the information-sharing policy set at the start of couples or family work, giving the disclosing person a chance to share first.

Sources: NASW Code of Ethics (2021), 1.07(g)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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118. ASM-1125 (rev 1) · Best answer: C

A 58-year-old client in an employee assistance program has been meeting with a social worker about work stress. Midway through a session, the client becomes sweaty and short of breath and rubs their left arm, saying the chest pressure is 'just my nerves.' The client has never had a panic attack before. What should the social worker do **FIRST**?

A. Guide the client through slow, paced breathing
B. Ask what thoughts came just before the pressure began
C. Get an ambulance to the office immediately

Why each option is right or wrong

- **A** **Incorrect** (trap: Ignores risk). Paced breathing helps during a panic attack once a medical cause has been excluded. Using it now treats new chest pressure with arm pain as anxiety and loses time if the heart is involved.
- **B** **Incorrect** (trap: Right action, wrong time). Exploring the thoughts behind an attack is a useful cognitive step in treating panic. It belongs after a medical emergency is ruled out, and asking now delays care for possible cardiac symptoms.
- **C** **Correct.** Possible medical emergencies come before any psychological approach. DSM-5-TR notes that panic-like symptoms that first appear after about age 45, or that look atypical, raise the chance of a medical cause. New chest pressure with left arm pain, sweating and breathlessness in a 58-year-old with no panic history can signal a heart attack, so the social worker gets an ambulance now. It beats A because breathing coaching assumes the episode is panic and wastes minutes that matter if it is cardiac (DSM-5-TR).

Takeaway: When panic-like symptoms first appear in midlife with cardiac red flags, treat them as a medical emergency before considering anxiety.

Sources: DSM-5-TR (APA, 2022), Anxiety Disorders: Panic Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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119. ETH-1107 (rev 1) · Best answer: A

Residents of a public housing complex tell a community social worker that the city plans to close the neighborhood's only public health clinic. Several residents want to speak at next month's city council hearing but have never addressed public officials. What is the **MOST** appropriate role for the social worker?

- **A.** Help residents prepare and deliver their own testimony
- **B.** Testify at the hearing on the residents' behalf
- **C.** Stay neutral because the closure is a political matter

Why each option is right or wrong

- **A** **Correct.** Social workers help the public take informed part in shaping social policies and institutions (NASW Code 6.02) and take political action for access to needed services (6.04(a)). Residents have said they want to speak, so helping them prepare and present testimony builds their capacity and puts the people affected in front of decision-makers, in keeping with the NASW Social Justice principle. A beats B because speaking for residents who want to speak for themselves replaces their voice; the worker might add testimony too, but not instead of theirs.
- **B** **Incorrect** (trap: Good but not best). Professional testimony can support a community's case, but these residents want to speak. Speaking in their place limits their participation and the weight that comes from people directly affected.
- **C** **Incorrect** (trap: Misreads law or ethics). The Code calls for political action and public participation to protect access to services (6.04(a)). Staying neutral on a closure that cuts a neighborhood's health access would ignore that responsibility.

Takeaway: Support people affected by a policy to take part in shaping it themselves, rather than speaking for them.

Sources: NASW Code of Ethics (2021), 6.02; NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), Ethical Principles: Social Justice
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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120. ETH-1170 (rev 2) · Best answer: A

A social worker at a nonprofit counseling agency meets with a client who works nights and can attend only evening sessions, which the agency does not offer. A free clinic nearby runs evening groups for the client's concern. The agency director has told staff not to mention that clinic because funding depends on client numbers. Which action is **MOST** consistent with the social worker's ethical duties?

A. Tell the client about the clinic's evening services
B. Help the client ask the employer for a daytime shift
C. Ask the director to approve mentioning the evening clinic

Why each option is right or wrong

A **Correct.** Clients' interests are primary (NASW Code 1.01), and informed consent includes telling clients about reasonable alternatives (1.03(a)). An employer's policy should not interfere with ethical practice (3.09(d)). This client cannot attend any session the agency offers, so withholding a free clinic that fits the schedule would leave the client without care to protect agency numbers. It beats C because seeking permission delays information the client needs now, and access cannot hinge on approval; the social worker can raise the policy with the director afterward (3.09(b)).

B **Incorrect** (trap: Good but not best). Solving a scheduling barrier can help, but asking the client to rearrange their job to fit the agency's hours puts the burden on the client while an option that already fits goes unmentioned. The client should hear about the alternative first.

C **Incorrect** (trap: Right action, wrong time). Raising the policy with the director is worthwhile, and the social worker should do it. Making the client wait for approval, though, puts the agency's funding concern ahead of the client's access to timely care.

Takeaway: When an employer's instruction conflicts with a client's interests, inform the client of fitting alternatives and address the policy separately.

Sources: NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 3.09(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 1 of 3

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121. ETH-1179 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

At a first meeting, an older adult tells the social worker, a newly assigned case manager at a senior center, that the previous case manager 'never returned a single call' and 'made me feel like a bother.' The client then asks whether that worker was 'just lazy.' What is the **BEST** response by the social worker?

A. Agree that the previous worker handled the case poorly
B. Ask what the client needs to feel supported this time
C. Explain that heavy caseloads can delay returned calls
D. Offer to help file a complaint about the previous worker

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Agreeing passes judgment on a colleague's conduct without any facts, which is the unwarranted criticism the Code warns against. It may feel validating, but it shifts the focus to blame rather than the client's current needs.

B **Correct.** The Code asks social workers to avoid unwarranted negative criticism of colleagues in communications with clients (2.01(b)) while keeping the client's well-being primary (1.01). The new case manager has no facts about what happened, so judging the previous worker either way would be unfounded. Inviting the client to describe what support should look like now honors the frustration and builds the new relationship. It beats C because explaining caseload pressures defends the colleague at the cost of brushing aside the client's experience.

C **Incorrect** (trap: Good but not best). Offering an excuse for the previous worker can sound like taking the colleague's side. The client hears that the complaint is being explained away, which weakens trust at a first meeting.

D **Incorrect** (trap: Skips assessment). Complaint options matter if the client wants to pursue one, and the social worker can share them later. Proposing a complaint now assumes misconduct the social worker cannot confirm and steers the client toward a step the client did not request.

Takeaway: When clients criticize a former provider, avoid judging the colleague either way; focus on what the client needs from the current relationship.

Sources: NASW Code of Ethics (2021), 2.01(b); NASW Code of Ethics (2021), 1.01
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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122. ETH-0010 (rev 1) · Best answer: B

A client with limited English proficiency arrives for a mental health intake with their 15-year-old child, who offers to interpret. The client says they would prefer that the child interpret. What is the **MOST** appropriate action?

A. Proceed with the child interpreting, as the client prefers
B. Use a qualified telephone or video interpreter for the intake
C. Ask a bilingual receptionist to interpret during the intake session

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A child interpreting a parent's mental health intake may be exposed to adult content, role reversal and pressure to filter what is said. Even with the client's preference, a minor is an inappropriate interpreter outside an emergency.

B **Correct.** A clinical intake requires accurate, confidential communication. Social workers use qualified interpreters for clients with language barriers (NASW Code 1.03(b); Cultural Competence Standard 9). Federal health-care rules bar relying on a minor child to interpret except briefly in an emergency (45 CFR 92.201(e)). A qualified remote interpreter is available now, so the intake can go ahead properly. It beats C because a bilingual staff member without interpreter training may miss clinical meaning, and the client never agreed to share this information with front-desk staff.

C **Incorrect** (trap: Good but not best). A bilingual receptionist is closer to adequate than a child, but staff without interpreter training may misinterpret clinical terms. It also widens who hears confidential information. A qualified interpreter is the standard.

Takeaway: Use qualified interpreters for clinical communication; never rely on a client's minor child except briefly in an emergency.

Sources: NASW Code of Ethics (2021), 1.03(b); NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication; Nondiscrimination in Health Programs and Activities (Section 1557 regulations), 45 CFR 92.201(e)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3